Uniform Medical Plan

# 2027 UMP Consumer-Directed Health Plan (PEBB)
# Certificate of Coverage



**Self-insured by the State of Washington · Effective January 1, 2027**

**Printed under the direction of the Washington State Health Care Authority Public Employees Benefits Board (PEBB)**

HCA 54-0021 (8/26)

# Directory

## Directory: medical services

| Contact type and description | Contact information |
|---|---|
| **UMP Customer Service**<br>Contact UMP Customer Service for questions about your medical benefits, including information on behavioral health support services, the expert second opinion program, your care management benefit, and more. | **Call:** 1-888-849-3681 (TRS: 711)<br>Monday–Friday: 5 a.m. to 8 p.m.; Saturday: 8 a.m. to 4:30 p.m. (Pacific)<br>**Chat now:** Sign in to your Regence account at ump.regence.com/ump/signin to chat now<br>Monday–Friday: 5 a.m. to 8 p.m.; Saturday: 8 a.m. to 4:30 p.m.<br>**Email:** Send secure email via your Regence account at ump.regence.com/ump/signin<br>**Visit:** UMP website at ump.regence.com/pebb<br>**Outside the U.S.:**<br>If you are outside the U.S. and you have questions about your benefits and coverage, you can use email or chat now to contact UMP Customer Service. You may request a customer service representative to contact you at a scheduled time during normal business hours. You can also use an app (for example, WhatsApp or Google Voice) to call the UMP Customer Service toll-free number listed above.<br>If you are outside the U.S. and need to find a local provider, make an appointment, or be hospitalized, call Blue Cross Blue Shield Global® Core at 1-800-810-2583 or call collect at 1-804-673-1177, 24/7. You can also use the online provider search tool on the Blue Cross Blue Shield Global® Core website at bcbsglobalcore.com. |
| **Network provider directory** | **Call:** 1-888-849-3681 (TRS: 711)<br>Monday–Friday: 5 a.m. to 8 p.m.; Saturday: 8 a.m. to 4:30 p.m. (Pacific)<br>**Chat now:** Sign in to your Regence account at ump.regence.com/ump/signin to chat now<br>Monday–Friday: 5 a.m. to 8 p.m.; Saturday: 8 a.m. to 4:30 p.m. (Pacific)<br>**Provider search:** ump.regence.com/go/pebb/UMP-CDHP |
| **Medical appeals and grievances (aka complaints)** | **Call:** 1-888-849-3681 (TRS: 711)<br>Monday–Friday: 5 a.m. to 8 p.m.; Saturday: 8 a.m. to 4:30 p.m. (Pacific)<br>**Chat now:** Sign in to your Regence account at ump.regence.com/ump/signin to chat now<br>Monday–Friday: 5 a.m. to 8 p.m.; Saturday: 8 a.m. to 4:30 p.m. (Pacific)<br>**Fax:** 1-877-663-7526<br>**Online:** Sign in to your secure Regence account at ump.regence.com/ump/signin. Go to Appeals to appeal online.<br>**Email:** UMPmemberappeals@regence.com<br>**Mail:** |

| Contact type and description | Contact information |
|---|---|
| | UMP Appeals and Grievances<br>Regence BlueShield<br>PO Box 1106<br>Lewiston, ID 83501-1106 |
| **Prior authorization**<br>For providers submitting medical service prior authorization requests | **Call:** 1-888-849-3682 (TRS: 711)<br>Monday–Friday: 7 a.m. to 5 p.m. (Pacific)<br>**Visit:** availity.com |
| **Access to medical claims** | **Visit:** Sign in to your Regence account at ump.regence.com/ump/signin |
| **Claims**<br>For members submitting medical service claims | **Fax:** 1-877-357-3418<br>**Online:** Sign in to your secure Regence account at ump.regence.com/ump/signin. Go to Claims to submit a claim online.<br>**Mail:**<br>Regence BlueShield<br>Attn: UMP Claims<br>PO Box 1106<br>Lewiston, ID 83501-1106 |
| **Coordination of benefits**<br>Contact UMP if you or your dependents have other insurance to make sure your claims are processed correctly. You may fax or mail the "Multiple Coverage Inquiry" form to UMP. | **Call:** 1-888-849-3681 (TRS: 711)<br>**Visit:** UMP commonly used forms available online at ump.regence.com/pebb/forms/common-forms and under Medical forms choose the "Multiple Coverage Inquiry" form<br>**Fax:** 1-877-357-3418<br>**Mail:**<br>Regence BlueShield<br>Attn: UMP Claims<br>PO Box 1106<br>Lewiston, ID 83501-1106 |
| **HealthEquity**<br>Health savings account (HSA) trustee | **Call:** 1-844-351-6853 (TRS: 711), 24/7<br>**Visit:** HealthEquity website at learn.healthequity.com/pebb |
| **Medicare** | **Call:** 1-800-MEDICARE (1-800-633-4227) (TTY: 1-877-486-2048) 24/7<br>**Visit:** medicare.gov |
| **Eligibility, enrollment, and address changes** | **Employees:** Contact your payroll or benefits office<br>**Continuation Coverage and retired employees of a former employer group:** Call the PEBB Program: 1-800-200-1004 (TRS: 711)<br>**Retirees:** Call the PEBB Program: 1-800-200-1004 (TRS: 711)<br>Monday–Friday: 8 a.m. to 4:30 p.m. (Pacific)<br>**Visit:** hca.wa.gov/erb<br>**Address changes for Retirees, Continuation Coverage, and retired employees of a former employer group:** |

| Contact type and description | Contact information |
|---|---|
| | You can also fax or mail a letter to update your address. Please include the subscriber's name and new address.<br>**Fax:** 1-360-725-0771<br>**Mail:**<br>Health Care Authority<br>PEBB Program<br>PO Box 42684<br>Olympia, WA 98504-2684 |
| **Medical policies that affect coverage or care**<br>Including prior authorization, Health Technology Clinical Committee (HTCC) information, clinical policies, and prescription drugs covered under medical benefits. | **Visit:** Policies that affect your care webpage at ump.regence.com/pebb/benefits/policies |

# Directory: prescription drug services

| Contact type and description | Contact information |
|---|---|
| **Prescription drugs**<br>Contact customer service, locate network pharmacies, ask prescription drug questions, access your ArrayRx account, and check prescription drug prices. | ArrayRx<br>**Call:** 1-888-361-1611 (TRS: 711)<br>Monday–Friday: 7:30 a.m. to 5:30 p.m. (Pacific)<br>Available outside these hours with limited services.<br>**Visit:** Prescription drug coverage webpage at ump.regence.com/pebb/benefits/prescriptions |
| **Network mail-order pharmacies**<br>Order new prescriptions and refills, check order status, manage prescriptions, track your package | Costco Mail-Order Pharmacy<br>**UMP members do not need to be Costco members to use their mail-order service.**<br>**Call:** 1-800-607-6861 (TRS: 711)<br>Monday–Friday: 5 a.m. to 7 p.m.; Saturday: 9:30 a.m. to 2 p.m. (Pacific)<br>**Visit:** Sign in to your Costco account at rx.costco.com<br><br>Postal Prescription Services (PPS)<br>**Call:** 1-800-552-6694 (TRS: 711)<br>Monday–Friday: 6 a.m. to 6 p.m.; Saturday: 9 a.m. to 2 p.m. (Pacific)<br>**Providers fax:** 1-800-723-9023<br>**Mailing a prescription order:**<br>Postal Prescription Services<br>PO Box 2718<br>Portland, OR 97208-2718<br>**Visit:** Sign in to your PPS account at ppsrx.com |
| **Network specialty drug pharmacy**<br>Order new prescriptions and refills for specialty drugs | Ardon Health<br>**Call:** 1-855-425-4085 (TRS: 711)<br>Monday–Friday: 8 a.m. to 7 p.m.; Saturday: 8 a.m. to 12 p.m. (Pacific)<br>Closed Sundays and all major holidays<br>For urgent health questions that require the assistance of a clinician after-hours, you can reach an Ardon Health pharmacist 24/7, by calling 1-855-425-4085 and following the prompts.<br>**Providers fax:** 1-855-425-4096<br>**Visit:** ardonhealth.com<br>**Email:** info@ardonhealth.com (This email is not secure) |
| **Prescription drug appeals and complaints** | ArrayRx<br>**Call:** 1-888-361-1611 (TRS: 711)<br>Monday–Friday: 7:30 a.m. to 5:30 p.m. (Pacific)<br>**Fax appeals to:** 1-866-923-0412<br>**Mail:**<br>ArrayRx |

| Contact type and description | Contact information |
|---|---|
| | Attn: Appeal Unit<br>PO Box 40168<br>Portland, OR 97240-0168 |
| **Prescription drug prior authorization**<br>For providers and pharmacists submitting prescription drug prior authorization requests, including requests for an exception for noncovered prescription drugs or changing quantity limits. | ArrayRx<br>**Call:** 1-888-361-1611 (TRS: 711)<br>Monday–Friday: 7:30 a.m. to 5:30 p.m. (Pacific)<br>**Fax:** 1-800-207-8235<br>**Visit:** covermymeds.com |
| **Prescription drug claims** | ArrayRx<br>**Call:** 1-888-361-1611 (TRS: 711)<br>Monday–Friday: 7:30 a.m. to 5:30 p.m. (Pacific)<br>**Fax:** 1-855-668-8550<br>**Mail:**<br>Pharmacy Manual Claims<br>PO Box 999<br>Appleton, WI 54912-0999<br>**Visit:** Find claim forms by visiting forms and publications at hca.wa.gov/ump-forms-pubs |
| **Coordination of benefits**<br>Contact ArrayRx if you or your dependents have other insurance to make sure your claims are processed correctly. You may fax, email, or mail the "ArrayRx Multiple Prescription Drug Coverage Inquiry" form to ArrayRx. | ArrayRx<br>**Call:** 1-888-361-1611 (TRS: 711) to request a form.<br>**Visit:** UMP commonly used forms available online at ump.regence.com/pebb/forms/common-forms and under Prescription drug forms choose the "ArrayRx Multiple Prescription Drug Coverage Inquiry" form<br>**Fax:** 1-855-668-8550<br>**Email:** Send email through your ArrayRx account at ump.regence.com/pebb/benefits/prescriptions<br>(This email is not secure)<br>**Mail:**<br>Pharmacy Manual Claims<br>PO Box 999<br>Appleton, WI 54912-0999 |

# Table of Contents


Directory ..... 1

Directory: medical services ..... 1

Directory: prescription drug services ..... 4

Online services ..... 11

How to use this certificate of coverage ..... 12

About UMP CDHP ..... 12

Features of UMP CDHP ..... 12
Accumulators ..... 13
Finding a health care provider ..... 13
Sample payments to different provider network status ..... 15
How to find a preferred provider ..... 16
Covered and noncovered provider types ..... 16
Primary care providers ..... 17
When you do not have access to a preferred provider: network waiver ..... 17
When and how to request a network waiver ..... 17
How an HSA works ..... 19
Out-of-area services ..... 19
Services received outside the United States ..... 20
What you pay for services ..... 22
Deductible ..... 22
Coinsurance ..... 24
When you pay ..... 25
Health savings account (HSA) ..... 25
Out-of-pocket limit ..... 26
Summary of services and payments ..... 27
Deductible and limits ..... 27
Types of services ..... 28
What else you need to know ..... 31
Benefits: what the plan covers ..... 31
Guidelines for coverage ..... 31
Health Technology Clinical Committee (HTCC) ..... 32
Summary of benefits ..... 32
List of benefits ..... 35
Acupuncture ..... 35
Ambulance ..... 35
Applied Behavior Analysis (ABA) Therapy ..... 36
Autism services ..... 36
Bariatric surgery ..... 37
Behavioral health ..... 37
Breast health screening tests ..... 40
Care Gap Closure Program ..... 40
Care management ..... 40
Chiropractic physician services ..... 41

**Dental services** ..... 41
**Diabetes care supplies** ..... 42
**Diabetes education** ..... 42
**Diagnostic tests, laboratory, and x-rays** ..... 43
**Dialysis** ..... 43
**Durable medical equipment (DME), supplies, and prostheses** ..... 43
**Emergency room** ..... 45
**End-of-life counseling** ..... 45
**Family planning services** ..... 46
**Foot care, maintenance** ..... 47
**Gender affirming care** ..... 47
**Genetic services** ..... 47
**Headaches, chronic migraine or chronic tension type** ..... 48
**Hearing care (diseases and disorders of the ear)** ..... 48
**Hearing exam and hearing aids (instruments)** ..... 48
**Home health care** ..... 49
**Hospice care** ..... 50
**Hospital services** ..... 51
**Infusion drug site of care program** ..... 52
**Knee arthroplasty, total** ..... 53
**Mammogram and Digital Breast Tomosynthesis (DBT)** ..... 53
**Massage therapy** ..... 53
**Mastectomy and breast reconstruction** ..... 53
**Mental health** ..... 54
**Naturopathic physician services** ..... 54
**Nurse line** ..... 54
**Nutrition counseling and therapy** ..... 54
**Obstetric and newborn care** ..... 54
**Office visits** ..... 57
**Omada for Diabetes** ..... 57
**Omada for Prevention and Weight Health** ..... 57
**Orthognathic and Telegnathic surgery** ..... 58
**Pain and joint management, interventional** ..... 58
**Prescription drugs** ..... 58
**Preventive care** ..... 58
**Radiology** ..... 61
**Second opinions** ..... 62

Skilled nursing facility ... 62
Sleep therapy ... 62
Spinal and extremity manipulations ... 63
Spinal injections ... 63
Spinal surgery ... 63
Substance use disorder ... 64
Surgery ... 64
Temporomandibular joint (TMJ) disorder treatment ... 64
Therapy: Habilitative and Rehabilitative ... 65
Tobacco cessation services ... 66
Transplants ... 67
Urgent care ... 68
Virtual care ... 68
Vision care (diseases and disorders of the eye) ... 70
Your prescription drug benefit ... 70
The UMP Preferred Drug List ... 71
How UMP decides which prescription drugs are preferred ... 73
Guidelines for prescription drugs UMP covers ... 73
What you pay for prescription drugs ... 76
Where to buy your prescription drugs ... 78
Limits on your prescription drug coverage ... 82
What to do if the plan denies coverage ... 87
Prescription drugs and products UMP does not cover ... 87
Limits on plan coverage ... 88
Prior authorization of medical services ... 88
General information from UMP Customer Service ... 91
What the plan does not cover ... 91
If you have other HDHP coverage ... 102
Coordination of benefits ... 102
Contact UMP and ArrayRx ... 102
Who pays first ... 102
Billing and payment: submitting a claim ... 106
Submitting a claim for medical services ... 106
Submitting a claim for prescription drugs ... 108
False claims or statements ... 109
Complaint and appeal procedures ... 110
What is a complaint (aka: grievance)? ... 110

**How to submit a complaint (aka: grievance)** ..... 110
**What is an appeal?** ..... 110
**The appeals process** ..... 111
**Complaints about quality of care** ..... 115
**Appeals related to eligibility** ..... 115
**Where to send complaints or appeals** ..... 115
**When another party is responsible for injury or illness** ..... 115
**Occupational injury or illness (workers' compensation) claims** ..... 115
**Legal rights and responsibilities** ..... 116
**Fees and expenses** ..... 117
**Services covered by other insurance** ..... 117
**Motor vehicle coverage** ..... 118
**Future medical expenses** ..... 118
**General provisions** ..... 118
**What you need to know: your rights and responsibilities** ..... 118
**Relationship to Blue Cross and Blue Shield Association** ..... 122
**Right to receive and release needed information** ..... 122
**Right of recovery** ..... 122
**Limitations on liability** ..... 122
**Governing law** ..... 123
**Anti-assignment** ..... 123
**No waiver** ..... 123
**Acronyms** ..... 124
**Eligibility and enrollment** ..... 125
**Eligibility for subscribers and dependents** ..... 125
**Enrollment for subscribers and dependents** ..... 126
**Medicare eligibility and enrollment** ..... 129
**When medical coverage begins** ..... 129
**Making changes** ..... 131
**When medical coverage ends** ..... 136
**General provisions for eligibility and enrollment** ..... 139
**Eligibility and enrollment for a retiree or survivor** ..... 140
**Eligibility for subscribers and dependents** ..... 140
**Enrollment for subscribers and dependents** ..... 142
**Medicare eligibility and enrollment** ..... 145
**When medical coverage begins** ..... 145
**Making changes** ..... 146

**When medical coverage ends** ..... 152
**General provisions for eligibility and enrollment** ..... 153
**Definitions** ..... 155
**Disclosures** ..... 179

# Online services

See the Directory pages at the beginning of this booklet for links and contact information.

Download the Regence app and register for a Regence account. Scan the code below or find it in the Apple App Store or on Google Play.

Use your Regence account to get the following personalized information:

- Access the certificate of coverage (this booklet) and the Summary of Benefits and Coverage (SBC) for your plan, as well as the Glossary of Health Coverage and Medical Terms.
- Access your online pharmacy account to view prescription claims.
- Chat with customer service using the chat now feature.
- Find providers in your plan's network.
- Submit medical claims online.
- View or order your UMP member ID card.
- View your Explanations of Benefits (EOBs).
- View letters UMP sent you.

Visit the UMP website to:

- Access information on BlueCard® or Blue Cross Blue Shield Global® Core.
- Access resources and programs.
- Access the certificates of coverage (this booklet) and the Summaries of Benefits and Coverage (SBCs) for all plans.
- Access UMP medical policies.
- Access wellness tools.
- Download or print documents and forms.
- Find providers in any plan network.
- Get cost estimates for treatment of common medical conditions.
- Learn about submitting medical claims and how to submit them online.
- Review complaints and appeals procedures.

Visit the Policies that affect your care webpage to:

- View Regence medical policies.

Visit the UMP Prescription drug coverage webpage to:

- Find a link to the UMP Preferred Drug List and the Drug Price Estimator.
- Find a link to your online pharmacy account.
- Find information on mail-order and specialty drugs.
- Learn about submitting prescription drug claims.
- Locate network pharmacies.
- Review prescription drug policies and programs.

Visit the HealthEquity website to:

- Sign in to your personal member portal.
- Check your health savings account (HSA) balance.
- View and pay a provider's claim that has already been processed by UMP.
- Make deposits directly from your bank account using electronic funds transfer.
- Manage your investments (for accounts with more than $1,500).
- Check your transaction history.
- Find forms or submit an online request to get reimbursed for expenses you paid out of pocket.
- View and print monthly account statements and tax forms.

# How to use this certificate of coverage

For general topics, check the table of contents.

For an overview of the most common benefits, see the "Summary of benefits" section. The summary also shows:

- How much you will pay.
- The page numbers where you may learn more about a benefit.

To look up unfamiliar terms, see the "Definitions" section.

# About UMP CDHP

Uniform Medical Plan Consumer-Directed Health Plan (UMP CDHP) is a self-insured Preferred Provider Organization (PPO) health plan. UMP is offered through the Washington State Health Care Authority's (HCA's) Public Employees Benefits Board (PEBB) Program. UMP is administered by Regence BlueShield and ArrayRx in partnership with HealthEquity, the trustee (manager) of your health savings account (HSA). Moda Health administers the UMP prescription drug benefits for ArrayRx. All prescription drugs, services, and other benefits covered by this plan may require approval by the PEB Board. Changes to covered benefits that require Board approval take place when benefits are procured for the next calendar year.

This plan is available only to people eligible for coverage through the PEBB Program. See the "Eligibility and enrollment" section for more information.

This plan is designed to keep you and your enrolled dependents healthy and provide benefits in case of injury or illness. Review this certificate of coverage (COC) carefully so you may get the most from your health care benefits.

## Features of UMP CDHP

UMP CDHP is a health plan that covers the same services as the UMP Classic plan. However, this plan has a different payment structure and a higher deductible for covered services for a lower premium.

This plan has one combined deductible for medical services and prescription drugs. If you have one or more dependents enrolled in your plan, you must pay the total combined family deductible before the plan pays for services, including prescription drugs. Certain covered preventive care services and covered insulins are not subject to your deductible, which means the plan will pay for some services before you meet your deductible. See the "Deductible" section under "What you pay for services" for more information.

A major feature of this plan is a tax-free Health Savings Account (HSA). You and the State of Washington may contribute tax-free money into this savings account to pay for IRS-qualified medical expenses (such as the deductible and coinsurance), including certain services and certain drugs the plan may not cover. Contributions remain in your HSA until you use them; the balance rolls over from year-to-year.

If you or your dependents are enrolled in UMP CDHP and want to enroll in another health plan, it must be another high-deductible health plan, but without an HSA.

When enrolled in UMP CDHP you may not be claimed as a dependent on someone else's tax return, and you may not also be enrolled in any of the following plans:

- Medicare Part A or Part B.
- Medicaid (called "Apple Health" in Washington).
- Another comprehensive medical plan, such as a spouse or state-registered domestic partner's plan.
- A Voluntary Employees' Beneficiary Association Medical Expense Plan (VEBA MEP) account, unless you convert it to a limited VEBA MEP.
- TRICARE.
- The Civilian Health and Medical Program of the Department of Veterans Affairs (CHAMPVA).
- A flexible spending arrangement (FSA) or health reimbursement arrangement (HRA). This also applies if your spouse has an FSA, even if you are not covering your spouse on UMP CDHP. This does not apply to a Limited Purpose FSA.

# Accumulators

Insurance accumulators may transfer when a subscriber changes their enrollment from one UMP plan to another UMP plan mid-year, including during a special open enrollment (SOE).

When a subscriber enrolled in a PEBB Program UMP plan changes their own enrollment to another PEBB Program UMP plan (meaning the subscriber continues to be the subscriber on the new PEBB Program UMP Plan) during the plan year, the amounts already accrued toward the medical and pharmacy deductibles, out-of-pocket limits, and benefit limits (see definition of "Limited benefit") will transfer to the new PEBB Program UMP plan. These accumulators will also transfer for any member on the subscriber's account who changes UMP plans with the subscriber.

When a subscriber enrolled in a SEBB Program UMP plan changes their own enrollment to a PEBB Program UMP plan (meaning the subscriber continues to be the subscriber on the new PEBB Program UMP Plan) during the plan year, the amounts already accrued toward the medical and pharmacy deductibles and out-of-pocket limits will transfer to the new PEBB Program UMP plan. These accumulators will also transfer for any member on the subscriber's account who changes UMP plans with the subscriber.

If you have questions, contact UMP Customer Service.

# Finding a health care provider

As a UMP member, you may see preferred, participating, or out-of-network providers for medical services. The amount you pay for services depends on the network status of the provider. Seeing preferred providers will save you money.

Visit the UMP provider search to find UMP CDHP providers. You can search for preferred or participating providers by signing in to your Regence account. See the Directory pages at the beginning of this booklet for links and contact information.

If you use Find a doctor by searching as a guest, you will only see preferred providers. You can confirm a provider's network status before your visit by using the provider search or contacting UMP Customer Service.

**Preferred providers** are in the Preferred Network that applies to UMP CDHP members.

> **ALERT!** Some providers are preferred at one practice location but not another (example: urgent care clinics). Contact UMP Customer Service if you have any questions about the network status of a provider at a specific location.

- You pay 15 percent of the allowed amount after you meet your deductible. The plan pays 85 percent of the allowed amount.
- You pay $0 for covered preventive care services, including covered immunizations. The plan pays 100 percent of the allowed amount.
- The provider cannot bill you for charges above the allowed amount.
- If you see a preferred provider, you will not have to file a claim if the plan is your primary coverage.
- When you receive nonemergency services at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider. For nonemergency services performed at one of these facilities outside of Washington State, you still pay the network rate, but in some states, an out-of-network provider may be allowed to ask you to waive some of your balance billing protections.
- When you receive covered ground or air ambulance services in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

> **ALERT!** Some services and supplies are not covered by the plan (see the "What the plan does not cover" section) or have benefit limits. If you receive services or supplies that are not covered by the plan or you exceed your benefit limit, you will pay for those services or supplies, even if you see preferred or participating providers. Contact UMP Customer Service to find out if a service or supply is covered.

**Participating providers** contract with Regence BlueShield or another BlueCard network as a participating provider.

- You pay 40 percent of the allowed amount after you meet your deductible. The plan pays most covered services at 60 percent of the allowed amount.
- You pay $0 for covered preventive care services, including covered immunizations. The plan pays 100 percent of the allowed amount.
- The provider cannot bill you for charges above the allowed amount.
- If you see a participating provider, you will not have to file a claim if the plan is your primary coverage.

**Out-of-network providers** are not contracted with Regence BlueShield or another BlueCard network.

- You pay 40 percent of the allowed amount after you meet your deductible. The plan pays most covered services at 60 percent of the allowed amount.

**Note:** The provider may bill you for charges above the allowed amount, which is known as balance billing. You pay all charges billed to you above the allowed amount. Any balance billed amounts do not apply to your deductible or out-of-pocket limit.

At an out-of-network facility, when you receive emergency services, you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

At certain out-of-network hospitals in Washington State, when you receive nonemergency inpatient or outpatient services you pay the out-of-network cost-sharing amount and cannot be balance billed. Contact UMP Customer Service if you have any questions about the network status of a hospital in Washington State.

- You pay 40 percent of the allowed amount for covered preventive care services, including covered preventive immunizations. You will pay all charges above the allowed amount (balance billing). The plan pays 60 percent of the allowed amount.
- You pay $0 for flu shots and COVID-19 vaccines.
- The plan pays 100 percent of the allowed amount for covered preventive childhood immunizations.
- The 40 percent coinsurance you pay to out-of-network providers will **not** apply to your out-of-pocket limit.
- Any amount you pay above the allowed amount does **not** apply to your deductible or out-of-pocket limit.
- You may have to pay all charges at the time of service and then fill-out and send a claim form to the plan for reimbursement.
- The provider may choose not to request prior authorization for services that require it. As a result, the plan may delay or deny payment.

**Note:** The plan may send payment for covered out-of-network services to you or the provider.

## Sample payments to different provider network status

The table below shows how much you pay for professional services from preferred, participating, and out-of-network providers when UMP is your primary medical insurance. For these examples, assume you have met your deductible and have not reached your out-of-pocket limit. See descriptions of these provider types beginning on page 13. These are examples only and may not reflect your specific situation.

| Provider type | Must provider accept allowed amount? | Balance billing allowed? | Itemized payments | | You owe provider |
|---|---|---|---|---|---|
| **Preferred provider** | Yes. You pay 15% of the allowed amount (coinsurance). | No | Billed charge:<br>Allowed amount:<br>Plan pays 85%:<br>You pay 15%: | $1,000<br>$900<br>-$765<br>$135 | $135 |
| **Participating provider** | Yes. You pay 40% of the allowed amount (coinsurance). | No | Billed charge:<br>Allowed amount:<br>Plan pays 60%:<br>You pay 40%: | $1,000<br>$900<br>-$540<br>$360 | $360 |

| Provider type | Must provider accept allowed amount? | Balance billing allowed? | Itemized payments | | You owe provider |
|---|---|---|---|---|---|
| **Out-of-network provider*** | No. You pay 40% of the allowed amount (coinsurance), plus all charges above the allowed amount. | Yes | Billed charge:<br>Allowed amount:<br>Plan pays 60%:<br>You pay 40% plus $100** over allowed amount: | $1,000<br>$900<br>-$540<br><br>$460* | $460* |

* This amount does not apply to your out-of-pocket limit.

** For information about when out-of-network providers can balance bill, see the "Finding a healthcare provider" section and the definition of "Balance billing."

# How to find a preferred provider

As a UMP member, you have access to Regence BlueShield preferred providers and Blue Cross and Blue Shield plan providers worldwide through the Blue Cross Blue Shield Global® Core program (see the "Services received outside the United States" section). This means your health coverage is with you wherever you are. Your access to care includes many acute care hospitals, urgent care and ambulatory surgery centers, physicians, and other health care professionals.

To find a preferred provider, choose one of the following:

- Use the UMP provider search.
- Contact UMP Customer Service.
- Sign in to your Regence account, where you have access to more information about providers, as well as other tools.
- Use the Regence mobile application to find providers in your network.
- Call Blue Cross Blue Shield Global® Core Service Center or use their online provider search tool to find providers outside the U.S.
- Visit the Prescription drug coverage webpage to locate network pharmacies.

See the Directory pages at the beginning of this booklet for links and contact information.

# Covered and noncovered provider types

## Covered provider types

The plan pays the allowed amount for covered services only when performed by covered provider types within the scope of their license(s). When a facility charges facility fees, the plan pays the allowed amount if they are covered services and are within the scope of the facility's license. All preferred and participating providers are covered provider types.

See the list of covered provider types on the UMP website at **ump.regence.com/pebb/benefits/providers/covered-providers**.

## Noncovered provider types

If you see a provider who is not a covered provider type, such as a Licensed Athletic Trainer, the plan will not pay for any of the services received, and you will pay for all charges. As with all noncovered services,

any payments you make to a noncovered provider type will not apply to your deductible or out-of-pocket limit. If you have questions about noncovered providers contact UMP Customer Service.

## Primary care providers

A **primary care provider (PCP)** is a physician, nurse practitioner, or physician assistant who provides, coordinates, and helps you access a range of health care services, such as covered immunizations. A PCP may also help coordinate care for you when you need to see specialists.

You are not required to choose a PCP. However, a PCP may help prevent and treat health care conditions early, promoting your health and well-being. Patients who have a PCP have better health outcomes and a better care experience. To be designated as a PCP, a provider must be one of the provider types and practice under one of the specialties listed below.

**Provider type:**

- Doctor of Osteopathic Medicine (D.O.)
- Medical Doctor (M.D.)
- Naturopathic Physician (N.D.)
- Nurse Practitioner (A.R.N.P.)
- Physician Assistant (P.A.)

**Specialties:**

- Adult Medicine
- Family Practice
- General Practice
- Geriatrics
- Internal Medicine
- Obstetrics and gynecology (OB/GYN)
- Pediatrics (for members under age 18)
- Preventive Medicine

## When you do not have access to a preferred provider: network waiver

An approved network waiver allows the plan to pay for covered services provided by an out-of-network provider at the network rate. You may request a network waiver **only** when you do not have access to a preferred provider able to provide medically necessary covered services within 30 miles of your residence. **A service or supply prescribed, ordered, recommended, approved, or given by a provider does not make it a medically necessary covered service or supply**.

## When and how to request a network waiver

### Before your visit

When services require prior authorization, you may request a network waiver before services are provided. For a list of services requiring prior authorization, visit the Policies that affect your care webpage (see the Directory for the webpage link). Your network waiver request should be included with the prior authorization request. See the "Information needed to submit a network waiver request" section to learn what to include in your request.

When the plan approves the network waiver **before** you receive medical services from an out-of-network provider:

- You pay your cost-share for medical services the plan has approved through this waiver as though the provider is preferred.
- You pay $0 for covered preventive services, including covered immunizations. The plan pays 100 percent of the allowed amount.

## After your visit

When you receive any service, except those that require prior authorization, you may request a network waiver **after** the claims have been processed.

Network waiver requests not approved in advance are considered an appeal and must be submitted within 180 days of receiving an Explanation of Benefits. See the "Complaint and appeal procedures" section for information about your appeal rights.

## Information needed to submit a network waiver request

You should include all the following information in your request:

- A letter of explanation from you or your provider stating the need to see the out-of-network provider.
- Details of the research conducted by you or your provider to locate a preferred provider (e.g., dates network status was checked, names and phone numbers of preferred providers that were researched and may have been contacted before receiving services from the out-of-network provider).

## More information needed for prior authorization requests

When submitting a request for prior authorization that includes a network waiver, all the following additional information should also be included:

- Performing provider's name, address, phone number, and National Provider Identifier (NPI) or Tax ID number (TIN)
- Diagnosis code(s)
- Procedure code(s)
- Length of treatment requested or required for service(s)
- Estimated charge(s)

See the "Prior authorization of medical services" section for more information about requesting prior authorization from the plan for medical services.

## Where to send your network waiver request

UMP Member Appeals
Regence BlueShield
PO Box 1106
Lewiston, ID 83501-1106

If you have questions about the network waiver process, contact UMP Customer Service.

**ALERT!** If a network waiver is approved, you must still pay your cost-share for most medical services. Services provided under an approved network waiver apply to your deductible and out-of-pocket limit. Network waivers for ongoing services may require periodic review.

## How an HSA works

**ALERT!** If you enroll in a Consumer-Directed Health Plan with an HSA, you can also choose to enroll in a Limited Purpose Flexible Spending Arrangement (FSA). Limited Purpose FSA funds can be used only for dental and vision expenses, which allows you to save your HSA funds for medical expenses. A Limited Purpose FSA is different from an FSA. While you are enrolled in a Consumer-Directed Health Plan, with an HSA, you may not also enroll in an FSA at the same time. This is because you cannot contribute to an FSA and an HSA at the same time, as you may be subject to tax penalties.

The subscriber is the owner of the HSA. They get the tax advantages associated with an HSA and pay any taxes or penalties that result if the HSA does not comply with Internal Revenue Service (IRS) rules. The subscriber may contribute funds to the HSA within certain limits. Money from the HSA may be used to pay for qualified medical expenses of the subscriber, spouse or tax dependents (claimed as a tax dependent), even if they are not enrolled in this plan. See page 25 for details of how this works with UMP CDHP.

The trustee (manager) of your HSA, HealthEquity, features a member portal on their website (see Directory for link). When you sign into your Regence account, you may also find a link to the HealthEquity portal. Find out which services are available in the portal by reading the "Online services" section.

**ALERT!** You and your dependents may only enroll in other high-deductible health plans that do not include an HSA.

## Out-of-area services

Regence BlueShield has a variety of relationships with other Blue Cross and/or Blue Shield Licensees. Generally, these relationships are called "Inter-Plan Arrangements." These Inter-Plan Arrangements work based on rules and procedures issued by the Blue Cross Blue Shield Association. Whenever you access health care services outside of the geographic area Regence BlueShield serves, the claim for those services may be processed through one of these Inter-Plan Arrangements.

When you receive care outside of Regence BlueShield's service area, you may receive it from providers as described below. Providers contracted with the local Blue Cross and/or Blue Shield Licensee in that geographic area ("Host Blue") as a preferred provider are paid at the preferred provider level and will not bill you for balances beyond any deductible, copay and/or coinsurance for covered services. Providers that contract with the Host Blue as a participating provider are paid at the participating provider level and will not bill you for balances beyond any deductible, copay and/or coinsurance for covered services. Some providers (out-of-network providers) don't contract with the Host Blue.

### BlueCard Program

The BlueCard Program gives you access to covered health care services anywhere in the U.S. and its territories. Under the BlueCard Program, when covered services are received within the geographic area served by a Host Blue, Regence BlueShield will remain responsible for doing what was agreed to in this COC. However, the Host Blue is responsible for contracting with and generally handling all interactions with its providers that participate in the BlueCard Program.

When covered services are received outside of Regence BlueShield's service area and the claim is processed through the BlueCard Program, the amount you pay for covered services is calculated based on the lower of:

- The billed charges for your covered services; or
- The negotiated price that the Host Blue makes available to Regence BlueShield.

Often, this "negotiated price" is a simple discount that reflects an actual price that the Host Blue pays to your provider. Sometimes, it is an estimated price that takes into account special arrangements with a provider or group of providers. In other cases, it may be an average price, based on a discount that results in expected average savings for services from similar types of providers. Host Blues may use several factors to set an estimated or average price, including types of settlements, incentive payments, and/or other credits or charges. Estimated and average pricing may also take into account adjustments to correct Host Blue estimates of past prices. However, such adjustments will not affect the price used for your claim because those adjustments will not be applied after a claim has already been paid.

## Value-Based Programs

You may receive covered services under a Value-Based Program ("VBP") inside a Host Blue's service area. A Host Blue may pay providers that participate in a VBP for reaching agreed-upon cost and quality goals, meeting outcome measures, and coordinating care between its providers. You will not be responsible for paying fees associated with a VBP, except when a Host Blue passes these fees to Regence BlueShield through average pricing or fee schedule adjustments.

## Inter-Plan Programs: Federal or state taxes, surcharges, or fees

Federal or state law may require a surcharge, tax or other fee. If applicable, Regence BlueShield will include any such surcharge, tax or other fee as part of the claim charge passed on to you.

## Out-of-network providers outside Regence BlueShield's service area

When covered services are provided outside of Regence BlueShield's service area by out-of-network providers, the amount you pay will be based on either the Host Blue's out-of-network provider local payment or the pricing arrangements required by applicable state law. Other payment methods may be used in certain situations, such as billed charges for covered services, the payment that would have been made if the health care services had been obtained within Regence BlueShield's service area, or a special negotiated payment to determine the amount that will be paid for services provided by out-of-network providers. In any of these situations, you may be responsible for the difference between the out-of-network provider's billed amount and Regence BlueShield's payment for covered services. Federal or state law, as applicable, will govern payments for out-of-network emergency services.

# Services received outside the United States

**ALERT!** The plan does not cover prescription drugs ordered through mail-order pharmacies located outside the U.S., and there are no network pharmacies located outside the U.S. See "Prescription drugs purchased outside the U.S." to learn more.

# Blue Cross Blue Shield Global® Core

**ALERT!** Claims for services received outside the U.S. may take longer to process. UMP Customer Service is available to assist members with the Blue Cross Blue Shield Global® Core claim process.

If you are outside the U.S., you may be able to take advantage of Blue Cross Blue Shield Global® Core ("BCBS Global® Core") when accessing covered services. BCBS Global® Core is unlike the BlueCard Program available in the U.S. in certain ways. For instance, although BCBS Global® Core helps you access a provider network, the network is not served by a Host Blue.

You may have to pay the provider upfront and submit any claims to obtain reimbursement for those services. When you pay for covered services outside the U.S., you should complete and submit a BCBS Global® Core claim form with the provider's itemized bill(s) to the service center (address is on the form) to initiate claims processing. If you contact the BCBS Global® Core service center for assistance, in most cases, hospitals will not require you to pay for covered inpatient services (except for any applicable deductible, copay and/or coinsurance). In such cases, the hospital will also submit your claims to the service center.

If you need medical assistance services or help locating a provider outside the U.S., or if you need assistance with a claim submission:

- Call the BCBS Global® Core Service Center at 1-800-810-BLUE (2583), or call collect at 1-804-673-1177 (available 24 hours a day, seven days a week). An assistance coordinator, working with a medical professional, will arrange a physician appointment or hospitalization, if necessary.
- Register and sign in on the BCBS Global® Core website at **www.bcbsglobalcore.com** to use the online provider search tool.
- Visit the BCBS Global® Core website to get an international claim form and submit claims electronically.

Covered services received from providers outside the U.S. may not be subject to state or federal protections from surprise or balance billing, and therefore you may be billed for balances beyond any deductible, copay and/or coinsurance for covered services.

## When services received outside the U.S. are covered

Services received outside the U.S. are covered when they:

- Are appropriate for the condition being treated;
- Are covered by the plan;
- Are medically necessary;
- Are not considered to be experimental or investigational by U.S. standards; and
- Have met all medical policy criteria.

**Note:** The plan covers only the services and conditions specifically identified in this COC. Services that are not covered when received in the U.S. are also not covered when received outside the U.S.

## Important tips for receiving care outside the U.S.

- Always carry your UMP member ID card.

- If you need emergency medical care, go to the nearest hospital.
- If you need urgent medical care, contact the BCBS Global® Core Service Center for help finding a network provider.
- If you are admitted to the hospital, call the BCBS Global® Core Service Center to notify the plan of your admission.

# What you pay for services

## Deductible

A deductible is a fixed dollar amount you pay each calendar year before the plan begins paying for covered services. The deductible is combined for medical services and prescription drugs.

Your deductible amount depends on the number of people enrolled in your plan:

- For subscriber only (one member) it is $1,750;
- For subscriber plus one or more dependents (two or more members), it is $3,500.

Your deductible applies to most covered services, including prescription drugs paid by the member directly or paid on behalf of the member by another person including payments made through a manufacturer drug coupon or other manufacturer discount, unless otherwise stated in this COC. See below for services that are not subject to your deductible. Services apply to your UMP deductible in the order claims are received and processed, not necessarily in the order the member receives the services.

**ALERT!** If you receive services with a benefit limit (such as nutritional counseling) before meeting your deductible, those visits still apply to the benefit limit. For example, if you pay out of pocket for a nutritional counseling visit because you have not met your deductible, that visit will apply to the maximum of 12 visits per lifetime. See definition of "Limited benefit" for more information.

## What does not count toward your deductible

The following out-of-pocket expenses do not count toward your deductible:

- Charges for visits that exceed a benefit visit limit.
- Charges that exceed the maximum dollar limit for a limited benefit.
- Out-of-network provider charges above the allowed amount (see the "Sample payments to different provider network status" section).
- Services that are not subject to your deductible, even if you had out-of-pocket costs (except when you pay for covered insulins). For example, covered preventive care services received from an out-of-network provider.
- Services you pay for that are not covered by the plan (see the "What the plan does not cover" section).

**TIP:** You may use money from your HSA to pay for noncovered services if they are qualified medical expenses.

## Services not subject to your deductible

The plan pays the allowed amount for the services listed below, subject to cost-share, even if you have not met your deductible. When you see a preferred or participating provider (or use a network pharmacy) you do not have to meet your deductible before the plan pays for these services:

- Blood pressure monitor if you have a diagnosis of high blood pressure.
- Certain medications covered through the prescription drug benefit as outlined on the UMP Preferred Drug List.
- Continuous glucose monitors.
- Covered contraceptive supplies and services, except as described under the "Sterilization" section.
- Covered preventive care, including covered immunizations.
- Covered screening mammograms and medically necessary diagnostic and supplemental breast exams (see the "Mammogram and Digital Breast Tomosynthesis (DBT)" benefit).
- Flu shots and COVID-19 vaccines.
- Glucometers when diagnosed with diabetes.
- Hemoglobin A1c testing, and retinopathy screening if you have a diagnosis of diabetes.
- International Normalized Ratio (INR) testing if you have a diagnosis of liver disease and/or bleeding disorders.
- Low-Density Lipoprotein (LDL) testing if you have a diagnosis of heart disease.
- Omada for Diabetes services.
- Omada for Prevention and Weight Health services.
- Peak flow meter if you have a diagnosis of asthma.
- Second opinions required by the plan.
- Tobacco cessation services.

## How your deductible works

**If you cover yourself only,** your deductible is $1,750. You must pay this amount for covered services subject to the deductible (including covered prescription drugs) before the plan begins to pay for covered services.

**If you cover yourself and at least one other dependent,** your deductible is $3,500. You and your dependent(s) combined must pay this amount for covered services subject to the deductible (including covered prescription drugs) before the plan begins to pay for covered services. You must pay this amount for covered services for all covered persons combined before the plan pays for any services, including prescription drugs (other than those not subject to the deductible). Once the deductible is met, the plan begins paying for covered services.

**Note:** Only services that are covered and subject to your deductible count toward the deductible. See the "What does not count toward your deductible" section for a list of services that do not count toward your deductible.

# Coinsurance

**TIP:** Allowed amount is the most the plan pays for a specific covered service or supply. Out-of-network providers may charge more than this amount, and you are responsible for paying the difference between the billed amount and the allowed amount. This is called balance billing.

Coinsurance is the percentage of the allowed amount you pay for most medical services and for prescription drugs when the plan pays less than 100 percent. After you meet your deductible, you pay the percentages described below for most covered medical services. See the "What you pay for prescription drugs" section for how much you pay for prescription drugs.

- **For preferred providers:** You pay 15 percent of the allowed amount. The plan pays 85 percent of the allowed amount.
- **For participating providers:** You pay 40 percent of the allowed amount. The plan pays most covered services at 60 percent of the allowed amount.
- **For out-of-network providers:** You pay 40 percent of the allowed amount, and the provider may balance bill you. The plan pays most covered services at 60 percent of the allowed amount.

Professional charges, such as for physician services while you are in the hospital or lab work, may be billed separately.

**In Washington State:**

When you receive nonemergency services at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

At certain out-of-network hospitals in Washington State, when you receive nonemergency inpatient or outpatient services you pay the out-of-network cost-sharing amount and cannot be balance billed. Contact UMP Customer Service if you have any questions about the status of a hospital in Washington State.

At an out-of-network facility, when you receive emergency services you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

When you receive covered ground or air ambulance services in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

**Outside of Washington State:**

For nonemergency services performed at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center outside of Washington State, you still pay the network rate, but in some states, an out-of-network provider may be allowed to ask you to waive some of your balance billing protections.

At an out-of-network facility, when you receive emergency services you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

# Copay

A copay is a set dollar amount you pay, after you meet your deductible, when you receive treatments, supplies, or services including, but not limited to:

- Covered chiropractic, acupuncture, and massage services when you see a preferred provider will have a $15 copay per visit, after you meet your deductible. See the "Spinal and extremity manipulations" benefit, "Acupuncture" benefit, and "Massage therapy" benefit for more details.

Copays for the above services will apply toward the annual out-of-pocket limit.

## When you pay

Most of the time, you pay **after** your claim is processed.

- You will receive an Explanation of Benefits (EOB) from the plan that explains how much the plan paid the provider. The Member Responsibility section of your EOB tells you how much you owe the provider.
- The provider sends you a bill.
- You pay the provider.

**Note:** A provider may ask you to pay your deductible, copay and/or coinsurance, when applicable, at the time of service. When this happens, check your EOB to make sure the amount you paid is accurately reflected in the Member Responsibility section. Contact UMP Customer Service with questions.

## Health savings account (HSA)

The State of Washington will contribute $700.08 for one person, and $1,400.04 for more than one person enrolled in the plan, deposited in equal amounts over the calendar year. These contributions are made only during those months you are enrolled in UMP CDHP. You may also make contributions to your HSA account. To confirm the maximum annual contribution limits, visit the HealthEquity website at **learn.healthequity.com/pebb/hsa/**. The State of Washington's contributions apply to the limits, as does the SmartHealth wellness incentive (see page 25).

You may use your HSA to pay for member cost-sharing and other qualified medical expenses as described in Internal Revenue Code (IRC) 223(d)(2), including expenses not normally covered by the plan. You may use HSA funds to pay for expenses for your spouse or tax dependents, even if they are not covered by the plan. The IRS determines which services are eligible for reimbursement through an HSA, and who may pay for services using HSA funds. For details on how to use your HSA, including a list of items and services that you may pay for with your HSA funds, visit the HealthEquity website or contact HealthEquity (see Directory for link and contact information).

# If you earn the SmartHealth wellness incentive

Eligible subscribers can qualify for a one-time $125 contribution into their HSA (if enrolled in a Consumer-Directed Health Plan in 2027). If you participate in the wellness incentive program and qualified in 2026, the $125 incentive is distributed in your HSA for plan year 2027. The SmartHealth wellness incentive program will be ending at the end of 2027. If you earn the incentive by the cutoff date in 2027 and you are enrolled in a Consumer-Directed Health Plan in 2028, the $125 incentive is still distributed in your HSA for plan year 2028. However, the SmartHealth wellness incentive will no longer be available to earn in 2028. More details on eligibility and program requirements are listed in the "SmartHealth" definition or on HCA's SmartHealth webpage at **hca.wa.gov/pebb-smarthealth**.

# How to pay using your HSA

After a claim is processed, you may sign in to your HealthEquity HSA to view your medical claims. However, you will not be able to view your pharmacy claims.

You may pay for qualified medical expenses (see the definition of "Qualified medical expense") and qualified prescription drug expenses by:

- Using your HealthEquity debit card at the time of service.

- Logging in to your HSA and designating payment to be sent by HealthEquity directly to the provider. However, this option is not available for pharmacy claims. **Note:** You may make a partial payment using this method.
- Logging in to your HSA and paying yourself back for a qualified medical expense and a qualified prescription drug expense you paid using non-HSA funds. For example, if you paid cash at your provider's office for qualified medical expenses, you may reimburse yourself from your HSA. Reminder: Pharmacy claims will not be displayed.

# Out-of-pocket limit

The out-of-pocket limit is the most you pay during a calendar year for covered services from preferred providers and covered prescription drugs from network pharmacies. The out-of-pocket limit is combined for medical and prescription drugs. After you meet your out-of-pocket limit for the year, the plan pays for covered services by preferred providers and covered prescription drugs from network pharmacies at 100 percent of the allowed amount. The plan will not pay more than the allowed amount. **Expenses are counted from January 1, 2027, or your first day of enrollment (whichever is later) through December 31, 2027, or your last day of enrollment (whichever is earlier).**

Your out-of-pocket limit depends on the number of people enrolled in your plan:

- For subscriber only (one member) it is $4,200;
- For subscriber plus one or more dependents (two or more members) it is $8,400. Once a member meets $7,000 in covered out-of-pocket expenses annually, the plan will pay for covered services at 100 percent for that member.

## What counts toward this limit

- Coinsurance for covered prescription drugs paid by the member directly or paid on behalf of the member by another person including payments made through a manufacturer drug coupon or other manufacturer discount.
- Your coinsurance paid to preferred and participating providers and network pharmacies.
- Your coinsurance paid to out-of-network providers for emergency room services, air or ground ambulance, and nonemergency services furnished during a visit or stay at a preferred and participating hospital, hospital outpatient department, critical access hospital, or ambulatory surgical center.
- Your deductible paid to preferred and participating providers.
- Your copay for chiropractic, acupuncture, and massage therapy visits after you meet your deductible.

## What does not count toward this limit

A. Amounts paid by the plan, including services covered in full.

B. Your monthly premiums.

C. Your coinsurance paid to out-of-network providers and non-network pharmacies (except those listed above in "What counts toward this limit").

D. Balance billed amounts.

E. Amounts paid for services the plan does not cover (see the "What the plan does not cover" section).

F. Amounts that are more than the maximum dollar limit for a limited benefit. Any amount you pay over the maximum dollar limit does not count toward the out-of-pocket limit.

G. Amounts paid for visits over a benefit limit. For example, the benefit limit for acupuncture is 24 visits. If you have more than 24 acupuncture visits in one year, you will pay in full for those visits, and what you pay will not count toward this limit.

## What you pay after reaching this limit

After you meet your out-of-pocket limit for the year, you pay:

- $0 of the allowed amount for covered medical services from preferred providers and for covered prescription drugs at network pharmacies.
- $0 of the allowed amount for covered emergency room services and covered air or ground ambulance services.
- B through G (above) in the "What does not count toward this limit" section.
- 40 percent of the allowed amount for covered medical services for participating providers.
- 40 percent of the allowed amount for covered medical services for out-of-network providers. You may be balance billed.

# Summary of services and payments

**ALERT!** Even if a provider orders a test or prescribes a treatment, the plan may not cover it. Review this COC or contact UMP Customer Service if you have questions about benefits or limitations.

On the next several pages, you will find a summary of types of services and what you will pay for them. For a complete understanding of how a benefit works, read the pages listed in the "For more information" column.

All services must be medically necessary to be covered. **If you see an unfamiliar term, see the alphabetical list of definitions in the "Definitions" section.**

This COC applies only to dates of service between the day your coverage begins (no earlier than January 1, 2027) and the day your coverage ends (no later than December 31, 2027).

**ALERT!** If you have coverage under another health plan, see the "If you have other HDHP coverage" section.

## Deductible and limits

| Deductible and limits | Dollar amounts | What else you need to know | For more information, see page(s) |
|---|---|---|---|
| **Deductible** | $1,750 per member (maximum of $3,500 for a family of two or more) Combined deductible for medical services and prescription drugs. | You must meet your deductible before the plan pays for covered medical services and covered prescription drugs. Not all services count toward this | 22–24 |

| Deductible and limits | Dollar amounts | What else you need to know | For more information, see page(s) |
|---|---|---|---|
| | See page 25 if you earned the SmartHealth wellness incentive in 2026 for plan year 2027. | deductible. There is no deductible for covered insulins. | |
| **Out-of-pocket limit** | $4,200 per member (maximum of $8,400 for a family of two or more) Combined out-of-pocket limit for medical services and prescription drugs.<br>Once a member meets $7,000 in covered out-of-pocket expenses annually, the plan will pay for covered services at 100 percent for that member. | Your deductible, copay and/or coinsurance for covered medical services and covered prescription drugs paid to preferred or participating providers, or to network pharmacies count toward this limit. Once you meet your out-of-pocket limit, covered services paid to preferred providers or network pharmacies only are paid at 100% of the allowed amount. | 26–27 |
| **Health savings account (HSA)** | To confirm the maximum annual contribution, visit the HealthEquity website at learn.healthequity.com/pebb/hsa/ | You may pay for any qualified medical expenses from your HSA, including:<br>• Services that apply to your deductible.<br>• Services that are not covered by the plan but are still qualified medical expenses. | 25 |
| **Annual plan payment limit** | None | No limit to how much the plan pays per calendar year. | Not applicable |
| **Lifetime plan payment limit** | None | No limit to how much the plan pays over a lifetime. | Not applicable |

# Types of services

The table in this section describes how much you and the plan will pay for covered services. Unless otherwise noted, all payments are based on the allowed amount, and all services are subject to the deductible. See the "What you pay for services" section for more information about deductibles.

| Type of service | How much you pay for covered services | How much the plan pays for covered services |
|---|---|---|
| **Standard** | How much you pay (your coinsurance) depends on the provider's network status:<br>• **Preferred providers:** You pay 15% of the allowed amount. The provider cannot balance bill you.<br>• **Participating providers:** You pay 40% of the allowed amount. The provider cannot balance bill you. | • **Preferred providers:** The plan pays 85% of the allowed amount.<br>• **Participating providers:** The plan pays 60% of the allowed amount.<br>• **Out-of-network providers:** The plan pays 60% of the allowed amount. |

| Type of service | How much you pay for covered services | How much the plan pays for covered services |
|---|---|---|
| | • **Out-of-network providers:** You pay 40% of the allowed amount. The provider may balance bill you. | |
| **Preventive** | Covered preventive services are not subject to your deductible. How much you pay (your coinsurance) depends on the provider's network status:<br>• **Preferred providers:** You pay $0. The provider cannot balance bill you.<br>• **Participating providers:** You pay $0. The provider cannot balance bill you.<br>• **Out-of-network providers:** You pay 40% of the allowed amount. The provider may balance bill you. | • **Preferred providers:** The plan pays 100% of the allowed amount.<br>• **Participating providers:** The plan pays 100% of the allowed amount.<br>• **Out-of-network providers:** The plan pays 60% of the allowed amount. |
| **Inpatient** | Most inpatient services require both prior authorization (see page 88) and notice (your provider must notify the plan as soon as possible after you are admitted to a facility, but not later than 24 hours after you are admitted; see page 89).<br>Services are considered inpatient only when you are admitted to a facility. See definition of "Inpatient stay."<br>How much you pay when you are admitted to a facility depends on the facility's network status:<br>• **Preferred facilities:** You pay 15% of the allowed amount. The facility cannot balance bill you.<br>• **Participating facilities:** You pay 40% of the allowed amount. The facility cannot balance bill you.<br>• **Out-of-network facilities:** You pay 40% of the allowed amount. The facility may balance bill you.<br>How much you pay for professional services, such as provider consultations or lab tests depends on the provider's network status:<br>• **Preferred providers:** You pay 15% of the allowed amount. The provider cannot balance bill you. | • **Preferred facility:** The plan pays 85% of the allowed amount.<br>• **Participating facility:** The plan pays 60% of the allowed amount.<br>• **Out-of-network facility:** The plan pays 60% of the allowed amount.<br>The plan pays for professional services, such as provider consultations or lab tests, based on the provider's network status:<br>• **Preferred providers:** The plan pays 85% of the allowed amount.<br>• **Participating providers:** The plan pays 60% of the allowed amount.<br>• **Out-of-network providers:** The plan pays 60% of the allowed amount. |

| Type of service | How much you pay for covered services | How much the plan pays for covered services |
|---|---|---|
| | • **Participating providers:** You pay 40% of the allowed amount. The provider cannot balance bill you.<br>• **Out-of-network providers:** You pay 40% of the allowed amount. The provider may balance bill you. | |
| **Outpatient** | If you receive services at a facility that offers inpatient services (like a hospital), but you are not admitted, the services are covered as outpatient. See the specific benefit (e.g., emergency room or diagnostic tests) for how much you pay. You may be billed separately for facility fees in addition to provider fees.<br>• **Preferred providers:** You pay 15% of the allowed amount. The provider cannot balance bill you.<br>• **Participating providers:** You pay 40% of the allowed amount. The provider cannot balance bill you.<br>• **Out-of-network providers:** You pay 40% of the allowed amount. The provider may balance bill you. | • **Preferred providers:** The plan pays 85% of the allowed amount.<br>• **Participating providers:** The plan pays 60% of the allowed amount.<br>• **Out-of-network providers:** The plan pays 60% of the allowed amount. |
| **Facility** | You may be charged facility fees in addition to provider fees when accessing clinics, ambulatory surgery centers, and other facilities. A facility may be referred to as a "provider" on the Explanations of Benefits or other documents. How much you pay depends on the provider's network status:<br>• **Preferred facility:** You pay 15% of the allowed amount; the provider cannot balance bill you.<br>• **Participating facility:** You pay 40% of the allowed amount; the provider cannot balance bill you.<br>• **Out-of-network facility:** You pay 40% of the allowed amount; the provider may balance bill you. | • **Preferred facility:** The plan pays 85% of the allowed amount.<br>• **Participating facility:** The plan pays 60% of the allowed amount.<br>• **Out-of-network facility:** The plan pays 60% of the allowed amount. |
| **Special** | These services have unique payment rules, which are described in the "How much you will pay" column in the | |

| Type of service | How much you pay for covered services | How much the plan pays for covered services |
|---|---|---|
| | Summary of benefits table located in the "Summary of benefits" section. | |

## What else you need to know

- Some services are not covered (see the "What the plan does not cover" section).
- There is no waiting period for preexisting conditions.
- You will save money by seeing preferred providers (see the "Finding a health care provider" section).
- You must be enrolled in this plan for the plan to pay for medically necessary covered services.

# Benefits: what the plan covers

## Guidelines for coverage

> **ALERT!** A service or supply prescribed, ordered, recommended, approved, or given by a provider does not make it a medically necessary covered service or supply.

This plan will cover a service or supply, if it meets all the following conditions. The service or supply must:

- Be listed as covered; and
- Be medically necessary; and
- Be received by a member on a day between the date coverage begins (but no sooner than January 1, 2027) and the date coverage ends (but no later than December 31, 2027); and
- Have been determined to be a covered benefit by the Health Technology Clinical Committee (HTCC), if reviewed by the HTCC, and, if determined to be covered with conditions, meet the conditions of coverage established by the HTCC; and
- Meet the plan's coverage policies and prior authorization requirements.

Limits and exclusions may apply to plan benefits. See both the benefit description and the "What the plan does not cover" section.

Some services require prior authorization and/or notice before you receive treatment. For a list of services requiring prior authorization or plan notice, visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information.

The following sections describe the benefits provided by this plan. Be sure to read them carefully for important information that may help you get the most from your health coverage. If you do not understand the benefits, it is your responsibility to ask for help before receiving services by contacting UMP Customer Service.

UMP CDHP is a self-insured PPO health plan. UMP is offered through HCA's Public Employees Benefits Board (PEBB) Program. UMP is administered by Regence BlueShield, and ArrayRx. All prescription drugs, services, and other benefits covered by this plan may require approval by the PEB Board. Changes to covered benefits that require Board approval take place when benefits are procured for the next calendar year. Prescription drugs newly approved by the U.S. Food and Drug Administration (FDA) may require approval by the PEB Board before the plan will cover them.

# Health Technology Clinical Committee (HTCC)

**ALERT!** HTCC determinations may be implemented by the plan at any time during the calendar year, but are often implemented the January following the HTCC's decision. HTCC decisions are posted on the HCA website at **hca.wa.gov/hta**. Contact UMP Customer Service if you have questions about specific services that the HTCC has reviewed.

Created by chapter 70.14 of the Revised Code of Washington (RCW), the HTCC is a group of 11 independent health care professionals that reviews selected health technologies (services) to determine appropriate coverage, if any, for the services. These may include medical or surgical devices and procedures, medical equipment, and diagnostic tests.

In public meetings, the HTCC considers public comments and scientific evidence regarding the safety, medical effectiveness, and cost-effectiveness of the services when making its determination.

## How HTCC decisions affect UMP benefits

Under state law, the plan must comply with HTCC decisions, RCW 70.14.120 (1)(a), unless such determination conflicts with federal or state law. Services reviewed by the HTCC are either covered, covered with conditions, or not covered. The HTCC determines the conditions, if any, under which the service will be included as a covered benefit and, if covered, the criteria the plan must use to decide whether the service is medically necessary. Criteria established by the HTCC take precedence over Regence's medical policies. When the HTCC determines that a service is not covered, then the service is not covered by the plan. Some HTCC decisions include a requirement to follow FDA or Centers for Medicare and Medicaid Services (CMS) guidelines. You may review these guidelines on the FDA website at **fda.gov** or CMS website at **cms.gov**.

## Where to find HTCC decisions

You may view the list of services that the HTCC has reviewed or currently has under review on the HCA website at **hca.wa.gov/hta**. The website includes:

- Evidence reports
- Instructions on providing public comments on pending reviews or re-reviews
- Public comments
- The decisions and criteria for coverage
- The public meeting schedule

You may also contact UMP Customer Service with questions about coverage of conditions for HTCC technologies.

# Summary of benefits

**ALERT!** Not all covered services and limitations are listed in the table below. See the alphabetical list of all covered services in the "List of benefits" section. If you have questions, contact UMP Customer Service for more information.

Read the pages listed in the "For information" column for detailed information about each benefit. Not all details are included in the table. Also read:

- Services for which your provider must notify the plan or require prior authorization (see the "Limits on plan coverage" section)
- Services that are not covered (exclusions) (see the "What the plan does not cover" section)

If you have questions about your benefits, benefit limitations, services that require prior authorization or notice, or services not covered by the plan, contact UMP Customer Service.

For a description of the types of services listed in the "How much you will pay" column, see the "Types of services" section. For definitions of the rates, see the definitions of "Inpatient rate," "Preventive rate," "Special rate," and "Standard rate."

| Benefit/service | How much you will pay | For information, see page(s): |
|---|---|---|
| **Ambulance** | Special rate:<br>• 20% of the allowed amount for any provider<br>• Applies to your out-of-pocket limit | 35 |
| **Applied Behavior Analysis (ABA) Therapy** | Standard rate | 36 |
| **Behavioral health** | Mental health:<br>• Inpatient rate<br>• Outpatient/professional services: Standard rate<br>Substance use disorder:<br>• Inpatient rate<br>• Outpatient/professional services: Standard rate | 37 |
| **Breast health screening tests** | See the "Mammogram and Digital Breast Tomosynthesis (DBT)" benefit | 53 |
| **Chiropractic physician services** | See the "Spinal and extremity manipulations" benefit | 63 |
| **Diagnostic tests, laboratory, and x-rays** | Standard rate | 43, 53 |
| **Dialysis** | Standard rate | 43 |
| **Durable medical equipment (DME), supplies, and prostheses** | Standard rate | 43, 159 |

| Benefit/service | How much you will pay | For information, see page(s): |
|---|---|---|
| **Emergency room (ER)** | Special rate:<br>ER services are paid at the network rate at preferred, participating and out-of-network hospitals.<br>You pay 15% of the allowed amount. You are usually billed separately for:<br>• Facility charges<br>• Professional (physician) services<br>• Lab tests, x-rays, and other imaging tests | 45, 159 |
| **Hearing aids** | Special rate:<br>• Subject to the deductible<br>• You pay $0 of the allowed amount for one prescribed hearing aid per ear every 36 months. | 48 |
| **Home health care** | Standard rate | 49, 163, 165 |
| **Hospice care** | Special rate:<br>• You pay $0 for medical services after meeting your deductible<br>• You pay $0 for prescription drugs after meeting your deductible<br>• You pay $0 for end-of-life counseling while in hospice after meeting your deductible | 49, 163 |
| **Hospital services** | Inpatient rate<br>Outpatient/professional services: Standard rate | 51, 54, 95 |
| **Mammograms** | See the "Mammogram and Digital Breast Tomosynthesis (DBT)" benefit | 53 |
| **Mental health** | See the "Behavioral health" benefit | 38 |
| **Naturopathic physician services** | Standard rate | 16, 54, 87 |
| **Obstetric and newborn care** | Inpatient rate<br>Outpatient/professional services: Standard rate | 54 |
| **Office visits** | Standard rate | 57 |
| **Prescription drugs** | See the "What you pay for prescription drugs" section | 73 |
| **Preventive care and immunizations** | Preventive care services: Preventive rate<br>Covered preventive immunizations: Preventive rate | 53, 56, 58, 75, 172 |
| **Skilled nursing facility** | Inpatient rate<br>Some services may be billed separately, such as physical therapy | 62, 176 |
| **Spinal and extremity manipulations** | Special rate:<br>You pay a $15 copay per visit, after meeting your deductible, when you see a preferred provider. | 63 |

| Benefit/service | How much you will pay | For information, see page(s): |
|---|---|---|
| **Substance use disorder** | See the "Behavioral health" benefit | 37 |
| **Surgery** | Standard rate | 51, 53, 57, 63, 67, 156, 169, 174 |
| **Therapy: Habilitative and Rehabilitative** | Inpatient rate<br>Outpatient/professional services: Standard rate | 65 |
| **Tobacco cessation** | Preventive rate | 66 |

**Note:** For a list of services requiring prior authorization or plan notice, the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information. Many services require both prior authorization and plan notification. See the "Limits on plan coverage" section for how this works.

# List of benefits

## Acupuncture

The plan covers up to 24 visits for acupuncture treatment per calendar year (see definition of "Limited benefit"). After you meet your deductible, you pay the special rate (a $15 copay) for acupuncture when you see a preferred provider. The copay will apply toward the out-of-pocket limit. All visits apply to the 24-visit limit.

You may receive an office visit at the time of the acupuncture service (see the "Office visits" benefit for details). Not all acupuncture services are covered. See the "What the plan does not cover" section for more information.

**Note:** For participating providers and out-of-network providers, services are paid at the standard rate up to 24 visits per calendar year.

## Ambulance

Ambulance services for personal or convenience purposes are not covered.

### Ground ambulance

You pay 20 percent of the allowed amount for medically necessary ambulance services. You may not be balance billed for services provided in Washington State. Professional ground ambulance services are covered in a medical emergency:

- From the site of the medical emergency to the nearest facility equipped to treat the medical emergency.
- From one facility to the nearest other facility equipped to provide treatment for your condition.

When other means of transportation are considered unsafe due to your medical condition, the plan covers professional ambulance services:

- From one facility to another facility, for inpatient or outpatient treatment;
- From home to a facility; or
- From a facility to home.

### Air ambulance

You pay 20 percent of the allowed amount for medically necessary ambulance services regardless of network status. You may not be balance billed. Professional air ambulance services are covered only when all the following conditions are met:

- Ground ambulance is not appropriate
- The situation is a medical emergency
- Air ambulance is medically necessary
- Transport is to the nearest facility able to provide the care you need

> **ALERT!** The plan will not pay for air ambulance or other forms of air transport to move you to a facility closer to your home. If you travel outside the U.S., consider getting separate insurance that covers such air ambulance services.

### Water ambulance

You pay 20 percent of the allowed amount for medically necessary ambulance services. Professional water ambulance services are covered only when all the following conditions are met:

- Ground ambulance is not appropriate
- The situation is a medical emergency
- Water ambulance is medically necessary
- Transport is to the nearest facility able to provide the care you need

## Applied Behavior Analysis (ABA) Therapy

The plan covers Applied Behavior Analysis (ABA) Therapy only for a diagnosis of autism spectrum disorder. Providers of ABA Therapy services must be appropriately credentialed and qualified to prescribe or perform ABA Therapy services.

The plan requires prior authorization of ABA Therapy services for members age 18 or older before services are performed. No prior authorization is required for members under age 18. Like other services requiring prior authorization, approved ABA prior authorization is specific to the provider who made the ABA prior authorization request. ABA Therapy hours authorized for one provider are not automatically transferable to another provider. A change in the provider requires a new ABA prior authorization. The initial assessment and ABA therapy treatment order or prescription do not require prior authorization for members of any age.

As for other covered services, you receive the highest-level benefit by using preferred providers. See the "Types of services" section for differences in your cost for preferred, participating, and out-of-network providers. To find a preferred provider, visit the UMP provider search or contact UMP Customer Service.

You can also find more information on ABA Therapy by viewing the ABA policy on the UMP Policies that affect your care webpage. See the Directory pages at the beginning of this booklet for links and contact information.

## Autism services

To determine how a service, supply, or intervention is covered for an individual with autism, see that specific benefit. For example, Applied Behavior Analysis (ABA) Therapy is addressed on page 36; speech or occupational therapy is addressed on page 65 under the "Therapy: Habilitative and Rehabilitative" benefit;

and mental health coverage is found under "Behavioral health" on page 37. If a specific benefit is subject to limits, such as number of visits, these limits do not apply when the services, supplies, or interventions are for an autism diagnosis.

# Bariatric surgery

**TIP:** Contact UMP Customer Service to locate a provider.

For the plan to cover bariatric surgery, you must get prior authorization from the plan and follow all your chosen facility's bariatric surgery requirements. This includes working with a multidisciplinary bariatric surgery team and ensuring your surgery and postsurgical treatment meet all plan medical policies.

The plan covers only certain types of bariatric surgery procedures. You may be eligible for covered bariatric procedures if:

- Medicare is not your primary coverage;
- You are age 13 or older; and
- You meet the plan's clinical criteria.

## Related care following bariatric surgery

Panniculectomy (removal of loose skin) is covered following bariatric surgery when specific medical criteria are met. Most panniculectomies are considered cosmetic and are not covered.

UMP will cover medically necessary surgical follow-up care related to a covered bariatric procedure, such as care for complications and needed revisions. The follow-up surgical care must be appropriate and essential to the long-term success of the initial bariatric surgery and requires prior authorization.

Members who had a bariatric procedure before coverage under a UMP plan and have complications or need medically necessary revision are not required to verify prior coverage or that they met the plan's medical policy criteria for the initial bariatric procedure. However, you must follow plan requirements for follow-up care, including requesting prior authorization.

# Behavioral health

The plan covers behavioral health services including care for mental health and substance use disorder. You pay the inpatient rate when admitted to an inpatient facility, and the standard rate for all other care and services.

**In Washington State:**

When you receive nonemergency services at a network hospital, network hospital outpatient department, or a network critical access hospital in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

When you receive covered ground or air ambulance services in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

**Outside of Washington State:**

For nonemergency services performed at a network hospital, network hospital outpatient department, or network critical access hospital outside of Washington State, you still pay the network rate, but in some states an out-of-network provider may be allowed to ask you to waive some of your balance billing protections.

## Mental health

The plan covers mental health services for members with neuropsychiatric and mental health conditions. Marriage or family counseling is not covered. The amount the plan pays depends on the provider's network status (see the "Finding a health care provider" section). See below for details about coverage for substance use disorder treatment.

**Inpatient**

> **ALERT!** Your provider must notify the plan as soon as possible after you are admitted to a facility, but not later than 24 hours after you are admitted when you receive inpatient services. If the plan is not notified of inpatient treatment, the plan may not cover the treatment. Inpatient treatment is subject to clinical review.

Professional services (for example, doctors) may be billed separately from the facility charges. You pay the standard rate for covered professional services, unless it is for emergency services.

Services are considered inpatient when you are admitted to a facility. This may include either psychiatric inpatient hospitalization or care at a residential treatment facility. The plan requires prior authorization for nonemergency inpatient services. See the "Limits on plan coverage" section for details.

Your provider must notify the plan as soon as possible after you are admitted to a facility, but no later than 24 hours after you are admitted to a:

- Hospital
- Partial Hospitalization Program (PHP)
- Residential treatment facility

Contact UMP Customer Service about prior authorization requirements. For a list of services requiring prior authorization or plan notice visit Policies that affect your care webpage. See Directory for link and contact information.

**Outpatient**

> **ALERT!** See page 36 for prior authorization requirements related to Applied Behavior Analysis (ABA) Therapy services.

You pay the standard rate for outpatient mental health services. You pay based on the allowed amount and the network status of the provider. Most outpatient mental health services do not require prior authorization. For a list of services requiring plan notice, visit the Policies that affect your care webpage (see the Directory for the webpage link).

Your provider must notify the plan as soon as possible, but not later than 24 hours after Intensive Outpatient Therapy services are initiated.

## Substance use disorder

Substance use disorder is defined as an illness characterized by a physiological or psychological dependency on a controlled substance or alcohol. Substance use disorder does not include dependence on tobacco, caffeine, or food.

To be covered, treatment programs must be licensed to provide treatment to persons requiring substance use disorder treatment. The amount the plan pays depends on the provider's network status (see the "Finding a health care provider" section). See above for details about coverage for mental health services. Contact UMP Customer Service about prior authorization requirements.

**Inpatient**

> **ALERT!** Your provider must notify the plan as soon as possible after you are admitted to a facility, but not later than 24 hours after you are admitted when you receive inpatient services for substance use disorder treatment. If the plan is not notified of inpatient treatment, the plan may not cover the treatment. Inpatient treatment is subject to clinical review.

Professional services (for example, doctors or lab tests) may be billed separately from the facility charges. You pay the standard rate for covered professional services, unless it is for emergency services.

Your provider must notify the plan when:

- You receive detoxification services
- You are admitted to a hospital
- You begin a Partial Hospitalization Program (PHP)
- You are admitted to a residential treatment facility

**Outpatient**
You pay the standard rate for outpatient substance use disorder treatment. You pay based on the allowed amount and the network status of the provider.

Your provider must notify the plan when you receive the following services:

- Detoxification
- Intensive Outpatient Program (IOP)

Prior authorization for outpatient substance use disorder treatment is not required in most cases. The plan may require that your provider submit a treatment plan to determine medical necessity.

## Behavioral health support resources

UMP provides behavioral health support resources to meet your needs.

**Teladoc Health Mental Health**
Teladoc Health Mental Health is a self-guided health and resiliency online tool clinically proven to improve emotional health. This secure resource is available 24/7 to members age 13 or older at no cost to you. Teladoc Health's interactive and activity-based tools are personalized to you and address conditions such as depression, anxiety, stress, substance use disorders, and chronic pain. Visit Teladoc Health's Mental Health website at **TeladocHealth.com/start/mental-health-digital**, click Register Now and use the Teladoc Health "registration code" WAPEBB to sign up and learn more or download the app in the Apple App Store or on Google Play. The "registration code" is required for your initial enrollment.

**Find Care**
Find additional behavioral health resources, get connected to an in-network facility, office or virtual behavioral health provider or ask for help when you sign in to your regence.com account. If you are having difficulty finding a care provider, or are not sure what type of provider you need, UMP Customer Service and the team of Regence behavioral health experts can help match you to in-person or virtual behavioral health providers that offer a variety of behavioral health resources.

# Breast health screening tests

See the "Mammogram and Digital Breast Tomosynthesis (DBT)" benefit.

# Care Gap Closure Program

The Care Gap Closure Program encourages members to receive recommended preventive and chronic care services and screenings, also known as "gaps in care." These include screenings for cancer, diabetes, and more at no cost to you. This support includes helping members find a PCP, making appointments, ensuring members understand their benefits, and providing members ongoing support through case management, as needed. Contact UMP Customer Service for more information.

# Care management

Regence care management supports the unique needs of members with chronic, serious, or sudden illness or injury and prioritizes those needs by providing personalized services that enhance well-being.

Care management teams can help with:

- Advocating for members and their support systems and improving care through close collaboration with providers.
- Assisting members as they navigate the health care system, including helping members find preferred providers and facilities, and supporting members transitioning to different levels of care.
- Educating members about their care options, benefits, and coverage, as well as helping members make educated decisions regarding their health care.
- Supporting members with coordination of care needs.

We offer a single-nurse model dedicated to delivering personalized and holistic medical and behavioral health support to each member and their family. Once a member is engaged in the care management program, they may be assigned a case manager who is a licensed social worker or registered nurse. Regence case managers work closely with a member and their providers to help meet treatment plan goals and improve a member's overall health.

## How to get started

Providers may refer members, and Regence also proactively reaches out to members most likely to benefit from care management support. Members can also self-refer by calling Regence at 1-866-543-5765 (TRS: 711) for information about care management services.

## What's next

Once a member is identified for care management, the designated case manager calls the member. The Regence case manager will attempt at least three calls and will send a letter to the member. The member can respond to the letter if they wish to engage with a case manager. Providers are sent a letter or contacted by phone when their patient is enrolled in care management.

## Care management newsletters

Newsletters are sent twice per year with an option to opt into care management to all members diagnosed with coronary artery disease (CAD), congestive heart failure (CHF), chronic obstructive pulmonary disease (COPD), asthma, or diabetes.

## Case management as a condition of coverage

An HCA or plan medical director may review medical records and determine that your use of certain services is potentially harmful, excessive, or medically inappropriate. Based on this determination, the plan may require you to participate in and comply with a case management plan as a condition of continued benefit payment.

# Chiropractic physician services

See the "Spinal and extremity manipulations" benefit.

# Dental services

> **ALERT!** Dentists and other dental providers are not included in the UMP provider network, even if they are listed in the Regence provider directory.

The plan does not cover most dental services. For example, dental implants, orthodontic services, and treatment for damage to teeth or gums caused by biting, chewing, grinding, or any combination of these are not covered. However, your PEBB dental plan may cover these services. Refer to your dental plan's COC, found on the HCA website at **hca.wa.gov/erb**, for more information.

## What is covered by the plan

The plan covers oral surgery and other dental services under the medical benefit when they are considered medical. Oral surgery and dental services are considered medical if the condition being diagnosed and treated is either one which is not connected to the teeth and/or gums or is related to a disease or illness that affects the whole body. These medical services may be performed by a dentist or medical professional provider. You can find examples of these medical services in the Regence Medical Policy Administrative Guidelines to Determine Dental vs Medical Services by visiting the Policies that affect your care webpage (see the Directory for the webpage link). You pay 20 percent of the allowed amount for covered dental services unless otherwise stated, and the provider may balance bill you.

**Note:** UMP is not affiliated with the Uniform Dental Plan (UDP). If you are enrolled in UDP, contact UDP for information about services covered under your dental plan. Visit the HCA website at **hca.wa.gov/erb** and select "Contact" to find UDP contact information.

**Fluoride treatment**

See the "Preventive care" benefit for coverage of fluoride supplements and fluoride treatment (varnish) as applicable under the preventive care criteria.

**General anesthesia during a dental procedure**

General anesthesia performed during a dental procedure is covered **only** when:

- It is provided by an anesthesiologist; and
- The charges are covered by the plan (see below).

**Dental procedures**

General anesthesia may be performed in a dental office for covered procedures and is paid at the standard rate. Dental procedures that are performed in a hospital or ambulatory surgery center are covered **only** when necessary to safeguard the member's health because treatment in a dental office would be neither safe nor effective.

**Accidental injuries**

To receive coverage for repair of an accidental injury to natural teeth, the injury must be evaluated, and a treatment plan developed and finalized within 30 days of the injury.

The actual treatment may extend beyond 30 days if your provider determines upon the initial assessment that treatment should start later or continue longer. Treatment must be completed by the end of the calendar year following the accident. The plan does not cover treatment after UMP coverage ends.

**Example:** You have an accident on March 12, 2027, resulting in injuries that are covered by the plan. Your treatment plan must be finalized no later than April 11, 2027. All related treatment must be completed by December 31, 2028 (the calendar year following the accident).

The plan does not cover treatment that:

- Was not included in the treatment plan developed within the first 30 days following the accident;
- Extends past the end of the calendar year following the accident; or
- Extends past the end of your enrollment in the plan.

# Diabetes care supplies

> **TIP:** If a health plan other than UMP is your primary payer, claims for diabetes care supplies may be paid differently (see page 105).

## Medical

Insulin pumps and pump supplies are covered as durable medical equipment (DME). See page 45 for coverage of insulin pumps and related supplies.

## Prescription drug

Diabetes care supplies listed below are only covered under your plan's prescription drug benefit:

- Insulin syringes
- Lancets
- Test strips
- Continuous glucose monitors

To be covered, you must get a prescription for these supplies and purchase them from a pharmacy. Limits, such as quantity limits or prior authorization, may apply. To find the preferred diabetes care supplies, check the UMP Preferred Drug List online by visiting the UMP Prescription drug coverage webpage or contact ArrayRx Customer Service (see Directory for link and contact information). You can also find out the cost of covered diabetes care supplies using the Drug Price Estimator on the UMP Prescription drug coverage webpage.

You save money and avoid having to submit your own claims when you purchase these diabetes care supplies from an ArrayRx network pharmacy. Locate a network pharmacy by visiting the UMP Prescription drug coverage webpage or contact ArrayRx Customer Service.

For more information, see the "Requesting an exception for noncovered prescription drugs" section.

# Diabetes education

The plan covers diabetes self-management training and education, including nutritional therapy by registered dieticians. When diabetes education includes nutritional therapy, the nutritional therapy services are not subject to the 12-visit lifetime limit stated under the "Nutrition counseling and therapy" benefit.

**Note:** Additional services, support, and education are available through Omada Health. See the "Omada for Diabetes" and "Omada for Prevention and Weight Health" benefits for more information.

## Diagnostic tests, laboratory, and x-rays

You pay the standard rate for covered diagnostic tests, laboratory tests, and x-rays when medically necessary. If there are alternative diagnostic approaches with different fees, the plan will cover the least expensive, evidence-based diagnostic method. For a list of services requiring prior authorization, visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information.

**Covered services include:**

- All prostate cancer screening (prostate-specific antigen [PSA testing]), which is subject to your deductible and coinsurance, even if billed as preventive.
- Colonoscopy performed to diagnose disease or illness. See the list on page 58 for coverage of preventive or screening colonoscopy.
- Diagnostic laboratory tests, x-rays, and other imaging studies.
- Electrocardiograms (EKG, ECG).
- Skin allergy testing.

> **TIP:** See page 53 to learn how the plan covers mammograms.

The plan does **not** pay for the following tests (this list does not include all tests not covered by the plan):

- Carotid Intima Media Thickness testing.
- Upright Magnetic Resonance Imaging (uMRI), also known as "positional," "weight-bearing" (partial or full), or "axial loading."

## Dialysis

You pay the standard rate for covered dialysis services. The plan pays based on the allowed amount and the network status of the provider. Other professional providers may bill separately from the facility.

## Durable medical equipment (DME), supplies, and prostheses

> **TIP:** The plan pays for covered DME at the standard rate. To receive the highest benefit, you must get the equipment or supply from a preferred DME supplier or other preferred medical provider. To find preferred DME providers, see the "Finding a preferred DME supplier" section below.

You pay the standard rate for covered DME services and supplies if they are prescribed by a provider practicing within their scope of practice, medically necessary, and used to treat a covered condition, including, but not limited to:

- Artificial limbs or eyes (including implant lenses prescribed by a physician and required due to cataract surgery or to replace a missing portion of the eye).
- Automatic Positive Airway Pressure (APAP) devices and related supplies.
- Bi-level Positive Airway Pressure (BiPAP) devices and related supplies.
- Bone growth (osteogenic) stimulators.

- Breast prostheses and bras as required by mastectomy. See the "Mastectomy and breast reconstruction" benefit.
- Breast pumps for pregnant and nursing members (see "Services covered as preventive" on page 56).
- Casts, splints, crutches, trusses, and braces.
- Compression stockings.
- Continuous Positive Airway Pressure (CPAP) devices and related supplies.
- Diabetic shoes, only as prescribed for a diagnosis of diabetes. See the "Foot orthotics" section below.
- Elemental formulas for Eosinophilic Gastrointestinal Disorders (EGIDs).
- Insulin pumps and related pump supplies (see the "Insulin pumps and related pump supplies" section below).
- Ostomy supplies.
- Oxygen and its equipment, such as all types of concentrators and tanks for administration, are covered on a rental basis only.
- Penile prosthesis when other accepted treatment has been unsuccessful, and impotence is:
  - Caused by a covered medical condition; or
  - A complication directly resulting from a covered surgery; or
  - A result of an injury to the genitalia or spinal cord.
- Rental or purchase (at the plan's discretion) of DME such as wheelchairs, hospital beds, and respiratory equipment. (The combined rental fees cannot exceed full purchase price.)
- Wig or hairpiece to replace hair loss due to radiation therapy (or chemotherapy for a covered condition) is covered 100 percent up to a lifetime maximum of $100, after you meet your medical deductible. See the definition of "Limited benefit." Wigs and hairpieces for any other reason are not covered.

Some items require prior authorization. Find the list of supplies that require prior authorization by visiting forms and publications at **hca.wa.gov/ump-forms-pubs** and search "durable medical equipment" or contact UMP Customer Service.

The plan limits coverage of DME to one item of a particular type of equipment and the accessories needed to operate the item. If you receive a higher-cost DME item when a less expensive, medically appropriate option is available, the plan will not pay for the more expensive item.

The plan also covers the repair or replacement of DME due to normal use or a change in the member's condition (including the growth of a child). You are responsible for the entire cost of any additional pieces of the same or similar DME you purchase or rent for personal convenience or mobility.

**Note:** The plan does not cover replacement of lost, stolen, expired, or damaged DME.

## Foot orthotics

Items such as shoe inserts, foot orthotics, and other shoe modifications are covered only when both of these conditions are met:

- The member has been diagnosed with diabetes.
- Specialized (including customized) orthotics are prescribed to treat or reduce the risk of diabetes complications.

If you have questions about what services are covered, contact UMP Customer Service.

### Insulin pumps and related pump supplies

Insulin pumps and related pump supplies are covered as DME. For the highest benefit level, use a preferred DME supplier.

> **ALERT!** Continuous glucose monitors are not covered under the DME benefit. They are covered under the prescription drug benefit. See the "Diabetes care supplies" benefit for more information.

### Finding a preferred DME supplier

You may purchase DME through a medical supplier. To find a preferred DME supplier, visit the UMP provider search (see Directory for link). You do not have to sign in to the Regence member site to search for a provider, but you will get more personalized results if you do. Click on the "All categories" link (found beneath "Doctors by name" and "Doctors by specialty"). Type "durable medical" into the search box; a drop-down list will appear. Select "Durable Medical Equipment & Supplies." You should now have a list of network DME suppliers. Preferred providers are paid at the highest rate and are noted as a Category 1 provider in the UMP provider search. Different DME suppliers carry different types of supplies. You may need to call to confirm that a supplier has what you need.

### Commercial DME

You may purchase DME and supplies from an approved Commercial Seller. A "Commercial Seller" is a retailer, wholesaler, or commercial vendor that is approved to provide new medical supplies, equipment, and devices. To purchase DME or medical supplies from an approved Commercial Seller, sign in to your regence.com account, select Care, then Care resources and click on Medical supplies.

You pay 100 percent of the billed charge and submit a claim for reimbursement. The plan will reimburse you 85 percent of the allowed amount for covered DME supplies purchased through an approved Commercial Seller. To learn more contact UMP Customer Service.

## Emergency room

> **TIP:** If you need immediate care but your situation is not a medical emergency, see the "Urgent care" benefit for how to get treatment at a lower cost than in an emergency room.

The plan covers facility charges for emergency room treatment when the treatment is for covered diagnoses and treatment of an injury. Covered services are paid at 85 percent of the allowed amount after you pay your deductible.

Charges for professional services may be billed separately from facility (hospital or emergency room) charges. When you receive emergency services, you cannot be balance billed.

If your emergency room visit is determined to be a medical emergency, it will be paid at the network rate for preferred, participating, and out-of-network facilities. Separate professional services charges will also be paid at the network rate if your emergency room visit is determined to be a medical emergency.

## End-of-life counseling

End-of-life counseling involves discussing and planning for your end-of-life care, including treatment options and advanced directives. The plan covers end-of-life counseling for all members up to 30 visits per year. There is no requirement to be terminally ill, on hospice, or in the final stages of life to receive

end-of-life counseling services. End-of-life counseling associated with hospice services is paid at 100 percent after you meet your deductible. Outside of hospice, these services are paid as a medical benefit, subject to your deductible and coinsurance. For more information on hospice care, see page 50.

# Family planning services

If you receive care from a network provider, the plan will pay for the following covered services at 100 percent after you meet your deductible:

- Voluntary and involuntary termination of pregnancy (abortion or miscarriage), including prescription drugs
- Education and counseling related to contraception

**Note:** Covered contraceptive drugs and devices, including condoms, are covered at the preventive rate when obtained from a network pharmacy.

If you receive care from an out-of-network provider, covered services are paid at the standard rate and the provider may balance bill you. If you go to a non-network pharmacy, you will pay the full price and must submit a claim for reimbursement (see the "Non-network pharmacies: retail" section and the "Submitting a claim for prescription drugs" section). You must get over-the-counter contraceptive supplies from a network pharmacy for these items to be covered (see "Over-the-counter contraceptives" in the section below). Prescriptions purchased from an excluded pharmacy will not be covered. See the definition of "Excluded pharmacy."

## Contraceptives

> **ALERT!** Visits for placement and removal of covered contraceptive devices that require professional insertion and removal are covered at the preventive rate.

Contraceptives are covered under the prescription drug benefit. Contraceptives include, but are not limited to, birth control pills, emergency contraception (the "morning after" pill), vaginal rings, patches, implants, injectables, and condoms. Final determination of medical necessity for FDA approved contraceptives is determined by the prescribing provider. Quantity limits still apply.

You may purchase up to a 12-month supply of contraceptives. Contact ArrayRx Customer Service for information on how to obtain a 12-month supply. The replacement of lost, expired, or stolen contraceptives is not covered.

You may obtain emergency contraception over-the-counter without a prescription. If you go to a pharmacy without a prescription and the pharmacy does not have prescribing authority, you will need to submit a claim to ArrayRx for the contraceptive that you paid for out of pocket. When possible, it is best to obtain a prescription, since not all pharmacies have prescribing authority. Members will need to contact the pharmacy directly for information on prescribing authority.

## Barrier devices

All barrier devices requiring a prescription, fitting, insertion (including IUD placement immediately after delivery), or removal are paid at the preventive rate when you see a preferred or participating provider or use a network pharmacy. Barrier devices requiring a prescription or fitting include intrauterine devices (IUDs), diaphragms, and cervical caps.

### Over-the-counter contraceptives

Over-the-counter contraceptives are covered under the Preventive Tier (P) on the UMP Preferred Drug List (see "The UMP Preferred Drug List" section for more information), only if they are approved by and registered with the FDA.

For the plan to cover FDA-registered over-the-counter contraceptives, you must present your UMP member ID card and make your purchase at the pharmacy counter. If you go to a pharmacy without a prescription and the pharmacy does not have prescribing authority, you will need to submit a claim to ArrayRx for the contraceptive that you paid for out of pocket. When possible, it is best to obtain a prescription, since not all pharmacies have prescribing authority.

### Sterilization

When you see a preferred or participating provider, sterilization procedures, such as tubal ligation or vasectomy, are covered as follows:

- **For females:** Covered as preventive (not subject to your deductible and the plan pays 100 percent of the allowed amount).
- **For males:** Paid at 100 percent after you meet your deductible.

### Services and products not covered under the family planning benefit

The plan does not cover the following services and products as a family planning benefit:

- Over-the-counter products not approved by and registered with the FDA
- Reversal of voluntary sterilization
- Treatment of fertility or infertility, including direct complications resulting from such treatment

## Foot care, maintenance

Maintenance foot care includes services such as toenail trimming and corn or callous removal or trimming. These services are covered only when medically necessary and provided by an approved provider type.

## Gender affirming care

With a diagnosis of gender dysphoria, the following services are covered at the standard rate for outpatient services and at the inpatient rate for inpatient services:

- Covered surgical services
- Non-surgical services, including, but not limited to, hormone therapy, office visits, mental health counseling, and tests

This is not a complete list of medical and surgical treatments of gender dysphoria in transgender individuals. For more information and to find clinical criteria for gender affirming care, visit the Policies that affect your care webpage (see the Directory for the webpage link). Some services and prescription drugs associated with gender dysphoria may require prior authorization.

## Genetic services

Covered genetic tests require prior authorization. With prior authorization, the plan covers medically necessary, evidence-based genetic testing services. Some genetics tests are not covered. For information about genetic services related to the fetus during pregnancy, see "Services for obstetric and newborn care" on page 54. Contact UMP Customer Service with any questions.

# Headaches, chronic migraine or chronic tension type

The plan only covers the treatment of chronic migraine with OnabotulinumtoxinA (Botox) when both the following criteria are met:

- The condition has not responded to at least three prior pharmacological prophylaxis therapies from two different classes of prescription drugs; and
- The condition is appropriately managed for medication overuse.

Botox injections must be discontinued when:

- The condition has shown inadequate response to treatment (defined as less than 50 percent reduction in headache days per month after two treatment cycles); or
- The member has received a maximum of five treatment cycles.

The following treatments are **not** covered:

- Treatment of chronic tension-type headaches with Botox or acupuncture; and
- Treatment of chronic migraine or chronic tension-type headaches with massage, trigger point injections, transcranial magnetic stimulation, or manipulation/manual therapy (such as chiropractic services).

# Hearing care (diseases and disorders of the ear)

The plan pays under the medical benefit for covered services for treatment of diseases and disorders of the ear or auditory canal not related to routine hearing loss. These services are not part of the "Hearing exam and hearing aids" benefit.

# Hearing exam and hearing aids (instruments)

## Hearing exam (routine)

> **ALERT!** The plan pays for a hearing exam performed as part of a newborn screening at the preventive rate.

You pay the standard rate for one routine hearing exam per calendar year.

## Hearing aids

You pay $0 of the allowed amount, once you have met your deductible, for one prescribed hearing aid per ear every 36 months. You may be responsible for hearing aid charges exceeding the allowed amount.

You pay the standard rate for the following hearing-related services:

- Ear mold(s)
- Initial battery, cords, and other ancillary equipment
- Warranty (only as included with the initial purchase)
- Follow-up consultation within 30 days after delivery of hearing aid
- Rental charges up to 30 days if you return the rented hearing aid before actual purchase
- Repair of hearing aid equipment
- The initial assessment, fitting, adjustment, auditory training, and other ear molds as necessary to maintain an optimal fit for those who have obtained or intend to obtain a hearing aid

The following hearing-related items are **not** covered:

- Over-the-counter hearing aids that are not prescribed, except for initial assessment, fitting, adjustment, auditory training, and ear molds as necessary to maintain an optimal fit
- Charges incurred after your plan coverage ends, unless you ordered the hearing aid before that date and it is delivered within 45 days after your coverage ended
- Extended warranties, or warranties not related to the initial purchase of the hearing aid(s)
- Purchase of replacement batteries or other ancillary equipment, except those covered under terms of the initial hearing aid purchase

The following ancillary equipment is **not** covered:

- Alerting devices
- Assistive listening devices for FM/DM systems, receivers and transmitters
- Assistive listening devices for microphone transmitters
- Assistive listening devices for TDD machines
- Assistive listening devices for telephones
- Assistive listening devices for televisions (including amplifiers and caption decoders)
- Assistive listening devices for use with cochlear implants
- Assistive listening devices, supplies, and accessories not otherwise specified
- Hearing aid batteries

# Home health care

**ALERT!** See the "What the plan does not cover" section for services the plan does not cover.

In certain circumstances, the plan covers short-term, provider-directed, medically necessary home health services on an intermittent or part-time basis by a licensed home health, hospice, or home care agency, to help a member recover from an acute covered illness, injury, or hospital stay. Home Health care is provided through visits from specialized clinicians, performing specific tasks (rather than time-based shifts), on a short-term basis, until specified individual goals are met. These services must be part of a treatment plan written by your provider (such as your physician or advanced registered practitioner [ARNP]). The provider must certify that you are homebound. These short-term visits may include:

- Skilled nursing care, physical, occupational, or speech therapy
- Home health aides and clinical social services, provided in conjunction with the skilled services of a registered nurse (RN), licensed practical nurse (LPN) or physical, occupational, or speech therapist
- Disposable medical supplies as well as prescription drugs provided by the home health agency
- Home infusion therapy
- Home care of wounds resulting from injury or surgery
- End-of-life counseling (see page 45)

For services that may be covered under another benefit, such as nutritional counseling or follow-up care for bariatric surgery, see that benefit in this COC for coverage rules and limitations. These limitations

apply even if the services are provided in the home or by a home health provider. For information on substitution of private duty nursing as an alternative benefit in lieu of hospitalization or in lieu of admission to a skilled nursing facility, see page 51 (hospital services) or page 62 (skilled nursing facility). Contact UMP Customer Service if you have questions.

# Hospice care

Hospice (inpatient, outpatient, and respite care) is services provided by a state-licensed hospice program in the home or in a hospice facility to terminally ill patients. Services include pain relief care and support services that address the needs of terminally ill members and their families without intent to cure.

## Medical

Hospice services received from preferred and participating providers are covered at 100 percent of the allowed amount after you meet your deductible. The plan covers hospice care for terminally ill members for no more than six months. See page 45 for coverage of end-of-life counseling.

If you need hospice care, your provider will refer you to the program. For additional assistance, you may contact UMP Customer Service.

## Prescription drugs

For covered prescription drugs, UMP members in hospice care receive special coverage when using network pharmacies, including the network specialty drug pharmacy and the network mail-order pharmacies.

- **Until your deductible is met,** the member pays the full cost (allowed amount at a network pharmacy) for covered prescription drugs, except for covered insulins. There is no deductible for covered insulins.
- **After you pay your deductible,** the plan pays for all covered prescription drugs purchased through a network pharmacy at 100 percent of the allowed amount for members in hospice care.

This applies **only to the member in hospice care**.

All quantity limits, prior authorization requirements, and coverage limits apply.

If you are in hospice care, contact ArrayRx to activate special coverage for covered prescriptions.

> **ALERT!** The member pays the full cost for noncovered prescription drugs. If the member purchases covered prescription drugs from a non-network pharmacy, the plan pays under the normal prescription drug benefit as described in the "What you pay for prescription drugs" section. Prescriptions purchased from an excluded pharmacy will not be covered. See the definition of "Excluded pharmacy."

## Respite care

Respite care is continuous care of more than four hours a day to give caretakers temporary relief from caring for a member who is homebound or in hospice. The plan covers these services at 100 percent of the allowed amount after you meet your deductible, up to 14 visits per the member's lifetime.

## Death with Dignity

The Washington Death with Dignity Act allows terminally ill adults seeking to end their life to request lethal doses of prescription drugs. These terminally ill patients must be Washington residents who have less than six months to live. For more information about this Act, see chapter 70.245 RCW.

Care described under this Act includes services covered by UMP, subject to standard plan requirements.

If you have questions about medical services UMP covers, contact UMP Customer Service. If needed, UMP may assign a case manager to support you.

If you have questions about prescription drugs UMP covers, contact ArrayRx Customer Service.

If your current provider is unable to meet your needs, or if you need assistance in finding a provider for these services, visit End of Life Washington's website at **endoflifewa.org**. End of Life Washington is a community resource available to support the public in finding available providers.

For more information about the Death with Dignity Act, visit:

- The Washington State Department of Health (DOH) website at **doh.wa.gov/YouandYourFamily/IllnessandDisease/DeathwithDignityAct**.
- The Washington State Hospital Association's website at **wsha.org/for-patients/end-of-life/**.

# Hospital services

> **ALERT!** Many services provided in a hospital setting require prior authorization, notice, or both. Failure to request or receive prior authorization, or to notify the plan, may result in complete denial of claims. See the "Limits on plan coverage" section for how prior authorization and notice work.

This benefit covers hospital accommodations and inpatient, outpatient, and ambulatory care services, supplies, equipment, and prescribed drugs to treat covered conditions. Room and board is limited to the hospital's average semiprivate room rate, except where a private room is determined to be medically necessary. Some services require prior authorization. For a list of services that require prior authorization, visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information.

If you receive a higher-cost service or device at a hospital when a less expensive, medically appropriate option is available, you may have to pay the difference in cost. A preferred hospital cannot charge you for the difference in cost between the standard and higher-cost item (unless you agreed in writing to pay before receiving the services).

If benefits change under the plan while you are in the hospital (or any other facility as an inpatient), coverage is based on the benefit in effect when the stay began.

## Inpatient

> **ALERT!** Your provider must notify the plan as soon as possible after you are admitted to a facility, but not later than 24 hours after you are admitted when you receive inpatient services. If the plan is not notified of inpatient treatment, the plan may not cover the treatment. Inpatient treatment is subject to clinical review.

Services are considered "inpatient" when you are admitted as inpatient to a hospital. Professional services — such as lab tests, surgery, or other services — may be billed separate from the hospital. The plan pays these services according to the network status of the provider, unless your condition is a medical emergency (see the "Emergency room" benefit for more information) or qualifies for the network rate as specified under the "Not all providers at a network hospital are network providers" section below. The plan pays for all covered professional services at the standard rate.

Private duty nursing furnished by a licensed home health agency may be substituted as an alternative to hospitalization only if:

- Inpatient hospitalization is medically necessary and would be covered by the plan;
- Private duty nursing is the most cost-effective setting (private duty nursing must be an equal or lesser cost compared to hospitalization); and
- The member's provider agrees that private duty nursing is medically appropriate and will adequately meet the member's needs.

Private duty nursing is shift-based, hourly nursing care at home for adults and children, typically with a chronic illness, injury, or disability.

Substitution of private duty nursing in lieu of inpatient hospitalization has the same requirements as the hospital benefit. For example, all deductibles and coinsurances apply.

### Outpatient

Services are considered "outpatient" when you are not admitted to the hospital. Your cost depends on the services provided, such as lab tests, and the network status of the provider(s) involved in your care. Some services require prior authorization. For a list of services that require prior authorization visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information.

### Not all providers at a network hospital are network providers

Some providers who work in a network hospital or other network facility, including, but not limited to, anesthesiologists and emergency room doctors, may not be network providers.

**In Washington State:**

When you receive nonemergency services at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

At certain out-of-network hospitals in Washington State, when you receive nonemergency inpatient or outpatient services you pay the out-of-network cost-sharing amount and cannot be balance billed. Contact UMP Customer Service if you have any questions about the status of a hospital in Washington State.

**Outside of Washington State:**

For nonemergency services performed at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center outside of Washington State, you still pay the network rate, but in some states, an out-of-network provider may be allowed to ask you to waive some of your balance billing protections.

## Infusion drug site of care program

The plan covers provider-administered infusion drugs when administered at an approved site of care. Approved sites of care include standalone infusion sites, doctor's offices, home infusion and some outpatient hospital facilities. Infusion drugs in the site of care program require prior authorization by the plan before services are performed, or services will not be covered. Your provider must submit a prior authorization request for an unapproved site of care. See the "Limits on plan coverage" section for prior authorization instructions.

Contact UMP Customer Service for the infusion drugs covered under the site of care program, more information, or help finding an approved site of care near you.

## Knee arthroplasty, total

Covered services are paid at the standard rate. Computer navigated and unicompartmental knee arthroplasty for treatment of end-stage osteoarthritis and rheumatoid arthritis of the knee are covered only as follows:

- Total knee arthroplasty performed with computer navigation is covered.
- For individuals with unicompartmental disease, unicompartmental partial knee arthroplasty is covered.
- Multi-compartmental partial knee arthroplasty (including bicompartmental and bi-unicompartmental) is not covered.

## Mammogram and Digital Breast Tomosynthesis (DBT)

### Screening (preventive)

**For members age 40 or older,** with or without a clinical breast exam, the plan covers screening mammograms and Digital Breast Tomosynthesis (DBT) every year, and they are not subject to your medical deductible.

**For members under age 40,** the plan covers screening mammograms and DBT for members who are at an increased risk for breast cancer. A covered health care provider must order the service, and the claim must be billed with an "at risk" diagnosis to be covered under the preventive care benefit.

**Note:** Digital Breast Tomosynthesis (DBT) is only covered when you receive it along with a screening or a medically necessary diagnostic mammogram.

**How much you will pay**

For all members, services are covered at 100 percent of the allowed amount when they are screening or medically necessary diagnostic and supplemental breast examinations.

## Massage therapy

The plan covers up to 24 visits per calendar year for massage therapy (no more than 90 minutes per visit). See the definition of "Limited benefit". After you meet your deductible, you pay the special rate (a $15 copay) for up to 24 massage therapy visits per calendar year for covered diagnoses when you see a preferred provider. The copay will apply toward the out-of-pocket limit. All visits will apply to the 24-visit limit.

You must have a prescription for massage therapy treatment from a covered provider type, such as a physician. The plan does not cover massage therapy when you see a participating or out-of-network provider.

**ALERT!** The plan only covers preferred massage therapists. To find a preferred massage therapist, use the UMP provider search or contact UMP Customer Service (see Directory for link and contact information).

## Mastectomy and breast reconstruction

**ALERT!** See page 47 for coverage of breast reconstruction or mastectomy services related to gender affirming care.

You pay the standard rate for a mastectomy as treatment for disease, illness, or injury, as well as:

- Physical complications of all stages of a mastectomy.
- Prostheses.
- Reconstruction of the breast on which the mastectomy was performed.
- Surgery and reconstruction of the other breast to produce a symmetrical appearance.

# Mental health

See the "Behavioral health" benefit.

# Naturopathic physician services

While naturopaths are a covered provider type, naturopaths may recommend services that the plan does not cover. You will pay all costs for excluded and non-medically necessary services, even if your naturopathic physician recommends or prescribes them (see the "Medically necessary or medical necessity" definition for more information).

The plan does not cover herbal, homeopathic, or other dietary supplements (including vitamins and minerals, except as described on page 74), even if a covered provider type prescribes them.

# Nurse line

UMP members have access to registered nurses by phone 24/7 or by secure chat from 8 a.m. to 8 p.m. (Pacific) at no cost to you. Nurses provide immediate symptom assessment, health information, and advice. They can help you decide if you need to go to the emergency room, see a doctor either virtually or in-person, or care for your symptoms at home. UMP members can call the nurse line at 1-877-375-2599 (TRS: 711) or by signing into your Regence account for live and secure nurse chat.

# Nutrition counseling and therapy

> **TIP:** See the "Diabetes education" benefit for how these services are covered for members with diabetes.

The plan covers up to 12 visits per lifetime for nutrition counseling and therapy services. Similar services may be covered under other benefits that are not subject to the 12-visit limit, including, but not limited to, the "Omada for Diabetes program," "Diabetes education," and the "Omada for Prevention and Weight Health program."

# Obstetric and newborn care

## Pregnancy program

As an expectant parent, the program helps you manage your health throughout pregnancy and offers access to a nurse line, pregnancy support, and education 24/7. It also includes a smartphone application to help you track milestones, identify symptoms, access education, and get one-click access to the nurse line. To enroll in the program, call 1-888-569-2229 (TRS: 711) or sign in to your Regence account (see Directory for link). This program is covered at no cost to you.

## Services for obstetric and newborn care

See the "Covered and noncovered provider types" section for providers whose services are covered by the plan. Covered professional services include:

- Amniocentesis and related genetic counseling and testing during pregnancy.

- Care of complications associated with pregnancy, including pregnancies resulting from fertility or infertility treatment.
- Prenatal and postnatal care.
- Prenatal testing (as stated in Washington Administrative Code [WAC] 246-680-020).
- Vaginal or cesarean delivery.
- Placement of IUD including immediately after delivery.

**Note:** Early elective deliveries may not be covered. See "Deliveries before 39 weeks gestation" below.

Member's responsibility for inpatient hospital charges related to childbirth:

**Mother's care:**

- Meet your deductible.
- Pay the coinsurance for professional services while hospitalized.

**Newborn's care:**

- Meet the newborn's deductible.
- Pay the coinsurance for professional services while hospitalized.

**Note:** If only covered preventive care services (see the "Preventive Care" section) are billed for the newborn and services are provided by a preferred or participating provider, you will not meet the newborn's deductible and will not pay the coinsurance.

Circumcision of the penis is covered as a medical benefit. Because this is not a preventive service, your out-of-pocket cost may include the newborn's deductible and coinsurance for professional provider services.

**Note**: The Newborns' and Mother's Health Protection Act (NMHPA) requires a hospital length of stay in connection with childbirth for a mother or her newborn and may not restrict benefits for the stay to less than 48 hours following a vaginal delivery or 96 hours following a delivery by cesarean section.

Under Washington State law, a newborn child of a covered gestational parent must be enrolled as a dependent under the gestational parent's plan from birth through at least the first 21 days of life. During this period, the gestational parent's plan covers the newborn's covered services, and the newborn is subject to applicable cost-sharing as an enrolled dependent (see "Member's responsibility for inpatient hospital charges related to childbirth" above). Even if the newborn is later enrolled in different coverage, the newborn will remain enrolled under the gestational parent's plan for the first 21 days. Visit the HCA website at **hca.wa.gov/erb** for what you need to do for continued coverage beyond 21 days.

If your obstetric care began while covered under another health plan, and your providers are not part of the UMP network, contact UMP Customer Service to discuss your options.

## Deliveries before 39 weeks gestation

Vaginal or cesarean deliveries before 39 weeks of gestation are covered when the services are medically necessary. Examples include:

- A medical emergency affecting the member or baby.
- A medical condition of the member or baby for which a delivery is medically necessary.
- Labor begins naturally (without medical intervention) before the member reaches 39 weeks of gestation.

Vaginal or cesarean deliveries before 39 weeks of gestation are **not** covered when the services are:

- Scheduled for convenience and not for medical necessity or medical emergency affecting the member or baby.
- Neither the member nor baby have a medical condition for which immediate delivery is medically necessary.

Talk to your provider about whether early delivery is for a medically necessary reason. For questions about this policy, contact UMP Customer Service.

## Birth doula services

> **ALERT!** This benefit only covers certain services provided in Washington State from a birth doula certified by the DOH. Contact UMP Customer Service to find a DOH certified birth doula or visit the Washington State provider credentialing search tool at wahelms.my.site.com/s/license-search.

The plan covers birth doula services during and after pregnancy, provided in Washington State, up to $1,000 per calendar year. Covered services are reimbursed at 100 percent up to the $1,000 calendar year benefit limit after you meet your deductible. See the definition of "Limited benefit."

Covered services are limited to:

- Prenatal intake visit
- Labor and delivery support
- Prenatal and postnatal visits

To submit a claim to UMP for reimbursement of covered birth doula services:

- Request an itemized bill from your birth doula.
- Complete a UMP (Regence) Medical Claim Form (see "How to submit a claim") and follow the instructions to submit your claim to the plan.

Contact UMP Customer Service with questions about this benefit. See the Directory pages at the beginning of this booklet for links and contact information.

## Services covered as preventive

The following services are covered as preventive (not subject to your deductible or coinsurance when you see a preferred or participating provider):

- HIV counseling and testing.
- Purchase of manual and electric breast pumps for pregnant and nursing members, plus supplies included with the initial purchase. Hospital-grade pumps are not covered.
- Screening for diabetes during pregnancy.

See “The UMP Preferred Drug List” section for prescription drugs that are preventive. They are listed as “Preventive” in the Tier column.

## Lactation (breastfeeding) counseling

Lactation counseling is covered at the preventive rate during pregnancy and after birth to support breastfeeding when members receive services by a covered provider type.

**Note:** Donor human milk from an approved milk bank for inpatient use may be covered when medically necessary.

### Ultrasounds during pregnancy

**The following limits do not apply to high-risk pregnancies**. **For example, a multiple pregnancy is considered high risk.** Contact UMP Customer Service to learn what is covered for high-risk pregnancies.

Routine ultrasounds during pregnancy are covered as follows:

- One in week 13 or earlier
- One during weeks 13-28

### Adding a new dependent to your coverage

For information about how to enroll new dependents in your health plan, refer to the Employee Enrollment Guide or the Retiree Enrollment Guide on the HCA website at **hca.wa.gov/employee-retiree-benefits/forms-and-publications**. You can also refer to "Making changes" in the "Eligibility and enrollment" section and in the "Eligibility and enrollment for a retiree or survivor" section of this certificate of coverage for more information.

## Office visits

The plan pays for office visits for covered conditions under the medical benefit. Preventive care visits with preferred providers as described under the definition of "Preventive care" are covered in full and are not subject to your deductible.

## Omada for Diabetes

Omada for Diabetes provides access to a virtual (online) diabetes management program through Omada Health.

You may be eligible if:

- Medicare is not your primary coverage;
- You are age 18 or older; and
- You have Type 1 or Type 2 diabetes.

You may take an online screening questionnaire to see if you meet the program's criteria by visiting Omada Health's website at **go.omadahealth.com/wapebb**, creating an account, and signing in. If you meet the criteria, you may participate in the program at no cost to you. The virtual program includes extensive support tools such as glucose tracking, live coaching, and mental and emotional care to help you improve your health and manage your diabetes.

To learn more, visit the UMP Care programs webpage at **ump.regence.com/pebb/benefits/programs** or contact UMP Customer Service.

## Omada for Prevention and Weight Health

Omada for Prevention and Weight Health provides access to a virtual (online) diabetes prevention program through Omada Health.

You may be eligible if:

- Medicare is not your primary coverage;
- You are age 18 or older;
- Your provider ordered a blood sugar test in the last 12 months; and
- Laboratory results showed you are in the prediabetes range.

You may take an online screening questionnaire to see if you meet the program's criteria by visiting Omada Health's website at **go.omadahealth.com/wapebb**, creating an account, and signing in. If you

meet the criteria, you may participate in the program at no cost to you. The virtual program includes a professional health coach, a wireless scale, and weekly online classes with a small group of participants who provide real-time support.

To learn more, visit the UMP Care programs webpage at **ump.regence.com/pebb/benefits/programs** or contact UMP Customer Service.

## Orthognathic and Telegnathic surgery

Orthognathic and telegnathic surgery require prior authorization by the plan. Contact UMP Customer Service if you have questions. See page 64 for treatment of temporomandibular joint (TMJ) disorder.

## Pain and joint management, interventional

Interventional pain management is a medical subspecialty that treats pain with invasive interventions like injections, spinal cord stimulations, and implantable drug delivery systems. The purpose of interventional pain management is to help members have less pain, so they can return to normal activities, when possible.

Prior authorization is required for interventional pain and joint management, such as:

- Epidural injections
- Facet blocks
- Pain pumps
- Radiofrequency ablations
- Sacroiliac joint injections

Prior authorization is not required for post-procedural pain management in an inpatient setting, including, but not limited to, treating acute pain due to trauma, acute post-thoracotomy pain, and acute postoperative pain.

## Prescription drugs

See the "Your prescription drug benefit" section.

## Preventive care

> **ALERT!** This benefit covers **only** services that meet the requirements below. If you receive services during a preventive care visit that do not meet these requirements, or your provider bills your visit as medical treatment instead of a preventive service, the services are not covered as preventive. Instead, when medically necessary, they are covered under the standard rate.

Covered preventive care services are paid at the preventive rate. You do not have to meet your deductible before the plan pays the allowed amount for services covered under the preventive care benefit. When you see a preferred or participating provider for these services, you pay $0. If you see an out-of-network provider, you pay 40 percent of the allowed amount, and the provider may balance bill you. If you do not have access to a preferred or participating provider for preventive care services, see the "When you do not have access to a preferred provider: network waiver" section for how to request a network waiver.

Preventive care services will be covered if the services are in accordance with age limits and frequency guidelines as recommended by the United States Preventive Service Task Force (USPSTF), the Health Resources and Services Administration (HRSA), including the Women's Preventive Services Initiative (WPSI) and the Bright Futures guidelines, or by the Advisory Committee on Immunization Practices of the Centers for Disease Control and Prevention (CDC). In the event any of these bodies adopts a new or revised recommendation, the plan has up to one year before coverage of the related services must be available under this benefit.

Covered services that do not meet the above criteria (for example, diagnostic colonoscopies) are not covered under the preventive care benefit but may be covered at the standard rate when medically necessary. For a list of immunizations covered as preventive, see the "Covered immunizations" section below.

Contact UMP Customer Service to ask if a medical service is covered as preventive. Contact ArrayRx Customer Service for questions about preventive prescription drugs.

Services that meet the above criteria and are covered under the preventive care benefit may include, but are not limited to:

- Routine physical examinations, well-baby care, and women's care (including screening for cervical cancer).
- Immunizations for adults and children as recommended by the USPSTF, HRSA, CDC, and as specified under the "Covered immunizations" benefit below.
- Certain radiology tests, lab tests, health screenings, and procedures, such as:
  - Colorectal preventive screening.
  - Preventive mammography services, including tomosynthesis (see the "Mammogram and Digital Breast Tomosynthesis (DBT)" benefit for more details).
  - Certain screening tests performed during pregnancy, including screening for gestational diabetes (see "Services covered as preventive" in the "Obstetric and newborn care" benefit for more on prenatal care).
  - Hepatitis B panel.
  - One-time screening by ultrasound for abdominal aortic aneurysm for men.
  - HIV antibody/antigen testing.
  - Routine screenings for certain sexually transmitted infections for women.
  - Human Papillomavirus (HPV) testing for women.
  - Depression screening for adults.
  - Interpersonal and domestic violence assessment.

  **Note:** Services performed to diagnose or treat disease or illness are covered under the "Diagnostic tests, laboratory, and x-rays" benefit at the standard rate. If a preventive screening results in diagnosis and treatment during the same visit, those services will be paid at the standard rate rather than the preventive rate. For example, if a colonoscopy begins as a preventive screening but the provider diagnoses and treats a condition during the procedure, the services will be paid at the standard rate.
- Intensive behavioral counseling for adults who are overweight or obese and have additional cardiovascular disease risk factors.
- Provider counseling and prescribed medications for tobacco use cessation (see the "Tobacco cessation services" benefit for more details).
- Counseling for HIV, interpersonal and domestic violence, and for sexually transmitted infections.
- Education and counseling regarding contraception (see the "Family planning services" benefit for more details about contraception and sterilization services that are covered at the preventive rate).
- Breastfeeding support and one new non-hospital grade breast pump including its accompanying supplies (see "Services covered as preventive" and "Lactation (breastfeeding) counseling" in the "Obstetric and newborn care" benefit for more details).
- Fluoride for prevention of dental decay (as recommended by USPSTF):
  - Fluoride supplements as prescribed by a primary care provider to children ages 5 and under, and when the water source is fluoride deficient.

- Fluoride varnish application by a primary care provider.
- Certain statin prescription drugs to adults age 40 or older (statin prescription drugs that are listed as "Preventive" in the Tier column on the UMP Preferred Drug List [see "The UMP Preferred Drug List" section]).

**Note:** Prostate cancer screening (prostate-specific antigen [PSA testing]) is not covered under the preventive care benefit but is covered as a medical benefit (subject to your deductible and coinsurance). For more information, see page 43.

**ALERT!** Follow-up visits or tests as a result of your preventive care visit are not covered under the preventive care benefit. If the plan normally covers the test or visit and it is medically necessary, it is covered under the medical benefit.

## Preventive care for specified chronic conditions

UMP covers services and supplies when prescribed to prevent the worsening of a diagnosed chronic condition or the development of a secondary condition. These services or supplies are not subject to your deductible when received from a preferred or participating provider or a network pharmacy. You pay the applicable coinsurance based on the network status of the provider.

**Prescription drug benefit**

- Certain medications covered through the prescription drug benefit as outlined on the UMP Preferred Drug List;
- Continuous glucose monitors (purchase at a network pharmacy to avoid balance billing); or
- Glucometers (purchase at a network pharmacy to avoid balance billing).

**Medical benefit**

- Blood pressure monitor if you have a diagnosis of high blood pressure;
- Hemoglobin A1c testing, and retinopathy screening if you have a diagnosis of diabetes;
- International Normalized Ratio (INR) testing if you have a diagnosis of liver disease and/or bleeding disorders;
- Low-Density Lipoprotein (LDL) testing if you have a diagnosis of heart disease; or
- Peak flow meter if you have a diagnosis of asthma.

## Covered immunizations

You pay the standard rate for covered immunizations that are not considered preventive.

The plan covers immunizations, at the preventive rate, as recommended by the USPSTF, HRSA, and CDC. The plan may cover immunizations in addition to those recommended by these organizations, at the preventive rate, based on reasonable scientific evidence and judgement from relevant expert organizations. For a list of immunizations covered as preventive, contact UMP Customer Service.

Some covered immunizations are classified as "may be recommended" by the CDC depending on medical condition or lifestyle. For those immunizations to be covered as preventive, you must meet the criteria specified on the CDC schedule.

Immunizations covered under the preventive rate are not subject to your deductible. Covered immunizations given by the providers listed under the "Where to get immunizations" section below are paid under the preventive care benefit. If you see an out-of-network provider for covered preventive immunizations, you pay 40 percent of the allowed amount, and the provider may balance bill you.

Covered flu shots and COVID-19 vaccines are paid at 100 percent regardless of the provider's network status. Flu shots and COVID-19 vaccines that are received from an excluded pharmacy will not be covered. See the definition of "Excluded Pharmacy."

> **TIP:** The plan covers flu shots and COVID-19 vaccines listed on the applicable CDC immunization schedule. For a list of immunizations covered as preventive, contact UMP Customer Service.

### Where to get immunizations

You pay $0 for immunizations covered under the preventive care benefit when received from a:

- Preferred or participating provider.
- Network vaccination pharmacy (Many network pharmacies have vaccination pharmacists who may give covered preventive immunizations to members. Contact a network pharmacy ahead of time to make sure the pharmacy has the vaccine you need. Present your UMP member ID card at the pharmacy counter for billing before receiving a vaccine.)
- Public health department.

The plan does not cover immunizations for travel or employment, even when recommended by the CDC or required by travel regulations. **Exception:** COVID-19 vaccines are covered when required for employment.

> **TIP:** If you get a vaccine from an out-of-network provider, submit your claim to Regence BlueShield as a medical claim (see the "Billing and payment: submitting a claim" section). If you get a vaccine from an out-of-network pharmacy, submit your prescription drug claim to ArrayRx (see the "Submitting a claim for prescription drugs" section). If you use an out-of-network provider or an out-of-network pharmacy, services will be paid at the out-of-network rate and you may be balance billed. Vaccines from an excluded pharmacy will not be covered. See the definition of "Excluded pharmacy."

## Radiology

Prior authorization is required for all nonemergency diagnostic imaging. Providers should obtain prior authorization before scheduling or performing any elective outpatient imaging service. Examples of imaging tests that require prior authorization are:

- Computed tomography (CT)
- Computed tomography angiography (CTA)
- Magnetic resonance angiography (MRA)
- Magnetic resonance imaging (MRI)
- Myocardial perfusion imaging (MPI)
- Nuclear cardiology:
  - Blood pool imaging
  - First pass ventriculography

    - Infarct imaging
    - Multiple-gated acquisition (MUGA) scan
- Positron emission tomography (PET and PET-CT)

# Second opinions

This benefit covers:

- **Second opinions you choose to get.** The plan covers these under the medical benefit, once you meet your deductible and pay the coinsurance.
- **Second opinions required by the plan.** The plan covers these at 100 percent (you do not have to meet your deductible or pay the coinsurance). If you do not get a second opinion when required by the plan, coverage for services may be denied.
- **Expert second opinion program.** This program, a treatment decision support service provided by 2nd.MD, offers members the opportunity to consult virtually with specialists. It also provides expert-led consultation and post-consultation follow-up support. To learn more, visit the 2nd.MD website at 2nd.md/ump or call 1-866-982-1434 (TRS: 711).

# Skilled nursing facility

Skilled nursing facility services are paid at the inpatient rate. Prior authorization is required before you are admitted to a skilled nursing facility (see the "Limits on plan coverage" section). In addition, the facility must notify the plan within 24 hours of your admission (see page 89).

This benefit covers skilled nursing facility charges for services, supplies, and room and board, including charges for services such as general nursing care made in connection with room occupancy. The plan covers up to 150 days per calendar year. Room and board is limited to the skilled nursing facility's average semiprivate room rate, except where a private room is determined to be medically necessary.

The plan does not cover stays at a skilled nursing facility that are primarily convalescent or custodial in nature.

Private duty nursing furnished by a licensed home health agency may be substituted as an alternative to placement at a skilled nursing facility only if:

- Skilled nursing facility care is medically necessary, not primarily convalescent or custodial in nature, and would be covered by the plan;
- Private duty nursing is the most cost-effective setting (private duty nursing must be an equal or lesser cost compared to a nursing facility); and
- The member's provider agrees that private duty nursing is medically appropriate and will adequately meet the member's needs.

Private duty nursing is shift-based, hourly nursing care at home for adults and children, typically with a chronic illness, injury, or disability.

Substitution of private duty nursing in lieu of placement in a skilled nursing facility has the same requirements and limitations as the facility benefit. For example, all deductibles and coinsurances apply and the benefit is limited to the equivalent of a maximum of 150 skilled nursing facility days per calendar year.

# Sleep therapy

Prior authorization is required for any facility-based diagnostic or titration study (free-standing or hospital), and for sleep treatment equipment and related supplies, such as:

- Initial treatment order and supplies (APAP, CPAP, BiPAP).
- In-lab sleep study (PSG, MSLT, MWT).
- Ongoing Treatment Order (APAP, CPAP, BiPAP).
- Titration study.

### Exception

The following supplies do **not** require prior authorization:

- Ongoing APAP supplies
- Ongoing BiPAP supplies
- Ongoing CPAP supplies

### Locations where sleep therapy services are not covered

Sleep therapy services are not covered:

- In the emergency room
- At urgent-care facilities
- During inpatient hospitalization

## Spinal and extremity manipulations

The plan covers up to 24 visits per calendar year for manipulations (adjustments) of the spine and extremities (arms and legs). See the definition of "Limited benefit."

After you meet your deductible, you pay the special rate (a $15 copay) for up to 24 visits for spinal and extremity manipulations when you see a preferred provider. The copay will apply toward the out-of-pocket limit. All visits apply to the 24-visit limit.

You may receive an office visit (see the "Office visits" benefit for more details) and/or x-ray (see the "Diagnostic tests, laboratory, and x-rays" benefit for more details) at the time of your spinal and extremity manipulation service.

**Note:** For participating providers and out-of-network providers, services are paid at the standard rate up to 24 visits per calendar year.

## Spinal injections

Prior authorization is required for some spinal injections (see the "Limits on plan coverage" section for how this works). The following therapeutic injections are covered for treatment of chronic pain:

- Cervical-thoracic epidural injections
- Lumbar epidural injections
- Sacroiliac joint injections

**Note:** See page 99 for a list of spinal injections that are not covered by the plan.

Spinal injections not specified in this section may be covered subject to the plan's medical policy. Contact UMP Customer Service for more information.

## Spinal surgery

Prior authorization is required for inpatient and outpatient spinal surgery.

## Substance use disorder

See the "Behavioral health" benefit.

## Surgery

Your provider must notify the plan when you are admitted for inpatient treatment and when you receive certain services. Some outpatient procedures require prior authorization. For a list of services that require prior authorization, visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information.

The plan covers the following services as outpatient:

- Outpatient surgery at a hospital
- Short-stay obstetric (childbirth) services (released within 24 hours of admission)
- Surgery and procedures performed at an ambulatory surgery center

You pay the standard rate for covered surgical services. The plan pays for covered surgical services according to the network status of the provider. The surgeon and other professional providers may bill separately from the facility.

**In Washington State:**

When you receive nonemergency services at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

At certain out-of-network hospitals in Washington State, when you receive nonemergency inpatient or outpatient services you pay the out-of-network cost-sharing amount and cannot be balance billed. Contact UMP Customer Service if you have any questions about the status of a hospital in Washington State.

At an out-of-network facility, when you receive emergency services you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

**Outside of Washington State:**

For nonemergency services performed at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center outside of Washington State, you still pay the network rate, but in some states, an out-of-network provider may be allowed to ask you to waive some of your balance billing protections.

At an out-of-network facility, when you receive emergency services you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

> **ALERT!** All surgeries must follow the plan's coverage rules. We recommend that you contact UMP Customer Service before any procedure to ask if it is covered or requires prior authorization.

## Temporomandibular joint (TMJ) disorder treatment

The plan covers diagnosis and medically necessary treatment of temporomandibular joint (TMJ) disorders, including surgery and non-surgical services. Treatment must follow the plan's medical policy and may require prior authorization. Treatment that is experimental or investigational, or primarily for cosmetic purposes, is not covered.

# Therapy: Habilitative and Rehabilitative

**Note:** The total limit for therapies for inpatient habilitative and inpatient rehabilitative services is a combined limit of 60 days annually. The total limit for therapies for outpatient habilitative and outpatient rehabilitative services is a combined limit of 60 visits annually.

## Habilitative (Neurodevelopmental) Services

The plan covers inpatient and outpatient habilitative (neurodevelopmental) services to assist a person to keep, learn, or improve skills and functioning for daily living. This could be related to issues such as:

- A congenital anomaly (such as cleft lip or palate).
- Conditions of developmental delay, including autism.

For the purposes of this benefit, developmental delay means a significant lag in achieving skills such as:

- Cognitive (thinking).
- Language (speech, reading, writing).
- Motor (crawling, walking, feeding oneself).
- Social (getting along with others).

You must have a prescription for occupational, physical, and speech therapy services from a covered provider type (see the "Covered and noncovered provider types" section), such as a physician.

**Inpatient habilitative services**

Prior authorization is required for inpatient habilitative admissions. The plan covers therapy services when they are provided during inpatient habilitative admission, up to 60 days combined per calendar year, counting all types of therapies listed here (see definition of "Limited benefit"). You must pay your coinsurance for inpatient services.

**Outpatient habilitative services**

The plan covers medically necessary outpatient occupational, physical, and speech therapy services up to 60 visits combined per calendar year, counting all types of therapies listed here (see definition of "Limited benefit").

## Rehabilitative Services

The plan covers inpatient and outpatient services to improve or restore function lost due to issues such as:

- An illness.
- An acute injury.
- Worsening or aggravation of a chronic injury.

You must have a prescription for occupational, physical, and speech therapy services from a covered provider type (see the "Covered and noncovered provider types" section), such as a physician.

**Inpatient rehabilitation services**

Prior authorization is required for inpatient rehabilitation admissions. The plan covers therapy services when they are provided during inpatient rehabilitation admission, up to 60 days combined per calendar year, counting all types of therapies listed here (see definition of "Limited benefit"). You must pay your coinsurance for inpatient services.

**Outpatient rehabilitation services**

The plan covers medically necessary outpatient neurodevelopmental, occupational, physical, and speech therapy services up to 60 visits combined per calendar year, counting all types of therapies listed here (see definition of "Limited benefit").

# Tobacco cessation services

**ALERT!** If you get nicotine replacement therapy or prescription drugs for tobacco cessation at a non-network pharmacy, or purchase at a cash register other than the pharmacy counter, and submit a claim, you may not receive full reimbursement from the plan. See the "Where to buy your prescription drugs" section for how to locate a network pharmacy. Prescriptions purchased from an excluded pharmacy will not be covered. See the definition of "Excluded pharmacy."

The services described in this section are covered only for tobacco cessation. Nicotine replacement therapy and prescription drugs for tobacco cessation that are listed as "Preventive" in the Tier column on the UMP Preferred Drug List are not subject to your deductible or coinsurance. Nicotine replacement therapy and prescription drugs for tobacco cessation have durational limits. You will find the limits listed in the UMP Preferred Drug List (PDL) under the "Smoking Deterrents" section. Coverage is available up to the durational limit per calendar year which applies to all covered nicotine replacement therapy and prescription drugs for tobacco cessation combined under the prescription drug benefit. Once you reach that limit, you will pay the full cost for additional nicotine replacement therapy and prescription drugs for tobacco cessation. Any amount you pay up to the allowed amount will apply towards your prescription drug deductible and the prescription drug out-of-pocket limit until met.

**Note:** Nicotine replacement therapy through the tobacco cessation program has a separate durational limit.

If you purchase an over-the-counter tobacco cessation product without a valid prescription, the plan will not cover it and you will pay the full cost.

**TIP:** You do not have to enroll in the tobacco cessation program to get coverage of nicotine replacement therapy or prescription drugs for tobacco cessation. See below for limits and rules on accessing these services.

## Nicotine replacement therapy

The plan covers only certain nicotine replacement therapy products (such as gum, patches, sprays, inhalers, tablets, or lozenges) at the preventive rate. Those that are preventive are listed as "Preventive" in the Tier column on the UMP Preferred Drug List. The plan does not normally cover over-the-counter drugs, but the plan covers nicotine replacement products when they are purchased at a network pharmacy using your UMP member ID card.

You may get nicotine replacement therapy directly from the tobacco cessation program (see the "Tobacco cessation program" section below), or by following these steps:

1. Get a prescription from your provider.
2. Take the prescription to a network pharmacy.
3. Make your purchase at the pharmacy counter of a network pharmacy. Give your prescription and your UMP member ID card to the pharmacist. The purchase must be submitted through the prescription drug system to be covered. If you do not provide a valid prescription at the pharmacy counter, the plan will not cover it.

If you get a nicotine replacement therapy product not designated as preventive, the plan will not cover it, and you will pay the full cost. To request full coverage of non-preventive nicotine replacement therapy for a medical reason, see "How to request an exception" below.

The plan does not cover e-cigarettes or vaporizers ("vapes").

### Counseling

The plan covers in-person counseling related to tobacco cessation at the preventive rate when you see a preferred or participating provider.

Phone or online counseling is covered only through the Tobacco cessation program described below. UMP members ages 17 and under may use the Smokefree Teen program as explained below.

### How to request an exception

To request coverage of a prescription drug or nicotine replacement therapy not usually covered under this benefit, see the "Requesting an exception for noncovered prescription drugs" section for how to request an exception. If your exception is approved, you will receive the approved product or prescription drug at no cost.

### Tobacco cessation program

You may participate in the tobacco cessation program if Medicare is not your primary coverage and you are age 18 or older. The program offers a variety of resources, including unlimited 1:1 personalized support with a dedicated certified tobacco treatment specialist, access to digital cognitive behavioral therapy (24/7) through app-based tools, educational materials, nicotine replacement therapy, and other resources to help you stop tobacco use. Visit the Pelago website at **my.pelagohealth.com/regence-bswa** or call 1-877-349-7755 to learn more. You can also search for and download the Pelago app in the Apple App Store or on Google Play. To learn more, visit the UMP Care programs webpage at **ump.regence.com/pebb/benefits/programs**.

If you get nicotine replacement therapy or prescription drugs for tobacco cessation that are not listed as "Preventive" in the Tier column on the UMP Preferred Drug List, you will pay as described in the "Nicotine replacement therapy" section on page 66.

For nicotine replacement therapy, you may get supplies sent to you from the program or get a prescription from your provider and purchase as described under "Nicotine replacement therapy" on page 66.

**ALERT!** You may attest for an exemption to the PEBB tobacco use premium surcharge if you or a dependent are tobacco-free for two months, enroll in the tobacco cessation program (for members age 18 or older), or access the information and resources aimed at teens in Smokefree Teen (for members ages 13 through 17). Visit the HCA website at **hca.wa.gov/erb** for details about the surcharge.

### Smokefree Teen

UMP members ages 17 and under may access tobacco cessation support services through the Smokefree Teen program online at **teen.smokefree.gov** or by calling 1-800-QUIT-NOW (784-8669) (TRS: 711).

## Transplants

Prior authorization is required for all transplants (except kidney and cornea). This benefit covers services related to transplants, including professional and facility fees for inpatient accommodation, diagnostic tests and exams, surgery, and follow-up care.

### Donor coverage

If a UMP member receives an organ, eye, or tissue donation from a live donor, the plan pays the standard rate for the donor's covered expenses as primary, regardless of any other coverage the donor may have. Covered donor expenses include costs to remove the donor's organ and treat complications directly resulting from the donor's surgery.

## Urgent care

See the "Emergency room" benefit for care during a medical emergency.

If you need immediate care or need care when your usual provider is closed, and your situation is not a medical emergency, you may use urgent care facilities to receive care at a lower cost than an emergency room. These services are paid at the standard rate, according to the provider's network status. Visit the UMP provider search to find preferred urgent care facilities (see Directory for link).

## Virtual care

### Telemedicine services

Telemedicine is the delivery of health care services through audio and visual technology, allowing real-time communication between the member at the originating site and a provider for the purpose of diagnosis, consultation, or treatment. Telemedicine includes audio-only telemedicine. Telemedicine does not include the use of fax or email.

"Store and forward technology" is a term used for the transfer of a member's medical information from one health care provider to another at a distant site, which results in medical diagnosis and management of the covered person. The purpose of telemedicine and store and forward technology is diagnosis, consultation, or treatment of the member. It does not include the use of fax or email.

If you see a network provider, telemedicine services are paid at the network rate. If you see an out-of-network provider, telemedicine services are paid at the out-of-network rate.

The plan covers store and forward technology and telemedicine from authorized originating sites under the medical benefit if:

- The plan provides coverage for the service when provided in person by the provider;
- The service is medically necessary;
- The service is determined to be safely and effectively provided through telemedicine or store and forward technology according to generally accepted health care practices and standards;
- The technology used to provide the service meets the standards required by state and federal laws governing the privacy and security of protected health information; and
- The service is recognized as an essential health benefit under section 1302(b) of the federal Patient Protection and Affordable Care Act (PPACA) in effect on January 1, 2015.

If services are provided through store and forward technology, there must be an associated office visit between the covered person and the referring health care provider. The associated office visit may be in person or via telemedicine. For audio-only telemedicine, the member must have an established relationship with the provider.

The originating site (the member's physical location) for telemedicine services must be one of the following sites:

- Community mental health center
- Federally qualified health center
- Home or any location determined by the individual receiving the service
- Hospital

- Physician's or other health care provider's office
- Renal dialysis center (except independent renal dialysis center)
- Rural health clinic
- Skilled nursing facility

Any originating site, except home, may charge a facility fee for infrastructure and preparation of the member.

Telemedicine and store and forward technology are subject to all terms and conditions of the plan, including utilization review, prior authorization requirements, and deductible. Services obtained from out-of-network providers will be reimbursed at the out-of-network rate.

The following are **not** covered by the plan:

- Email or fax transmissions between provider and member
- Home health monitoring
- Installation or maintenance of any telecommunication devices or systems
- Originating sites' professional fees
- Services that are not medically necessary
- Services that would not be covered if delivered in person
- Store and forward technology without an associated office visit between the covered member and the referring health care provider
- Telemedicine or store and forward services for services that are not recognized as essential health benefits under section 1302(b) of the PPACA in effect on January 1, 2015
- Telemedicine or store and forward services that cannot be safely and effectively provided through telemedicine or store and forward technology
- Telemedicine or store and forward services that use technology that does not meet state and federal requirements for privacy and security of protected health information
- Telemedicine visits originating from a location other than the specified originating sites

## Doctor On Demand

Doctor On Demand is a virtual care service that gives you access to high quality virtual primary care, on-demand urgent care, and behavioral health providers 24/7. On-demand urgent care and behavioral health services are a good option to consider when you need medical attention, but not emergency room care. First-time primary care appointments can be scheduled within days, and you are able to choose a provider for an annual preventive exam and ongoing routine care. Doctor On Demand providers are board-certified, U.S.-based providers who are specifically trained in video medicine. Members can schedule or connect in minutes with doctors face-to-face through a smartphone, tablet, or computer via the Regence website or Doctor On Demand smartphone application. Visit the Doctor On Demand website at **doctorondemand.com/regence-wa** or you can also search for and download the Doctor On Demand app in the Apple App Store or on Google Play. To learn more, visit the Telemedicine (virtual care) webpage at **ump.regence.com/pebb/benefits/telemedicine**.

Providers review a member's history and symptoms, perform a virtual exam, and recommend treatment, which may include prescription drugs and lab work. Doctor On Demand providers can treat most common health conditions, including, but not limited to:

- Asthma.
- Colds and allergies.
- Diabetes.
- Eczema and acne.

- Heartburn and indigestion.
- High blood pressure and high cholesterol.
- Migraines.
- Pink eye.
- Preventive exams.
- Primary care visits.
- Urinary Tract Infections (UTIs).

A Doctor On Demand virtual care appointment is paid at the standard rate and you will know exactly how much it will cost before you start your visit. Doctor On Demand providers are considered preferred providers. An annual preventive exam with a Doctor On Demand primary care provider qualifies as your once-a-year preventive visit.

In certain circumstances, virtual appointments may not be appropriate for a member's needs. When this occurs, Doctor On Demand may redirect the member to an in-person care setting. Doctor On Demand does **not** include the use of audio-only telephone, fax, or email. For additional questions, contact UMP Customer Service.

## Vision care (diseases and disorders of the eye)

> **ALERT!** The plan does not cover routine vision services. However, your vision plan may cover these services. Refer to your vision plan's COC, found on the HCA website at **hca.wa.gov/erb**, for more information about routine vision coverage.

You pay the standard rate under the medical benefit for treatment of diseases and disorders of the eye that are not part of a routine vision exam. Orthoptic therapy is not covered except for the diagnosis of strabismus, a muscle disorder of the eye. LASIK surgery is not covered.

Following cataract surgery, vision hardware (contact lenses or eyeglasses, including frames and prescription lenses) is covered as DME (see page 43). Services are paid at the standard rate.

# Your prescription drug benefit

**See the Directory for prescription drug contact information.**

Your plan's prescription drug benefit is managed by a partnership of companies known as ArrayRx:

- **Prior authorization, appeals, and customer service:** Moda Health
- **Network mail-order pharmacies:**
  - Costco Mail-Order Pharmacy
  - Postal Prescription Services (PPS)
  - **Note:** The plan does not cover prescription drugs purchased through mail-order pharmacies located outside the U.S.
- **Network specialty drug pharmacy:** Ardon Health

Visit the UMP Prescription drug coverage webpage to:

- Find a link to the UMP Preferred Drug List and the Drug Price Estimator.
- Find a link to your online pharmacy account.
- Find information on mail-order and specialty drugs.
- Learn about submitting prescription drug claims.

- Locate network pharmacies.
- Review prescription drug policies and programs.

Contact the mail-order or specialty drug pharmacy directly for help placing or tracking prescription orders. If additional help is needed, contact ArrayRx Customer Service.

**Note:** Regence BlueShield does not provide prescription drug benefits for UMP. Always contact ArrayRx with questions about your prescription drug coverage.

> **TIP:** The UMP Preferred Drug List is available on the UMP Prescription drug coverage webpage (see Directory for link). On this webpage, you will also find a link to the Drug Price Estimator (the prices for drugs listed in this tool assume you have met your deductible).

# The UMP Preferred Drug List

The UMP Preferred Drug List (sometimes called a "formulary") lists all the preferred drugs that UMP covers. Drugs not listed on the Preferred Drug List are not covered unless an exception is requested and approved by UMP. Excluded prescription drugs and products are not eligible for an exception. For the exception process, refer to the "Requesting an exception for noncovered prescription drugs" section.

The UMP Preferred Drug List includes:

- If a prescription drug requires prior authorization (see the "Preauthorization of prescription drugs" section)
- If you need to purchase a prescription drug from a specialty drug pharmacy (see the "Specialty drugs" section)
- If there are any limits on a prescription drug's coverage (see the "Limits on your prescription drug coverage" section)
- If there are less expensive alternatives

The UMP Preferred Drug List is updated online throughout the year, and how a prescription drug is covered may change at any time. You may look up your prescription drugs online on the UMP Prescription drug coverage webpage or by contacting ArrayRx. New brand-name prescription drugs may not be covered during the first 180 days they are available. To check if a new prescription drug is covered, contact ArrayRx Customer Service.

## Preferred drugs

Preferred drugs, including preferred specialty drugs, have been reviewed by the Washington State Pharmacy and Therapeutics (P&T) Committee or by Moda Health and found to be safe and clinically effective when compared to other drugs in the same therapeutic class or category. HCA or Moda Health has found preferred drugs to be safe, efficacious, and among the most cost-effective drugs for their therapeutic class.

> **ALERT!** The UMP Preferred Drug List may not show every alternative prescription drug you must try before an exception may be granted. See the "Step therapy" section for more information. If your exception request is denied, the plan's response letter will include the reason for the denial and the steps you can take next.

## Excluded drugs

Excluded drugs are not covered by the plan and are not eligible for an exception or reimbursement. See the "Prescription drugs and products UMP does not cover" section.

## Noncovered prescription drugs

Noncovered prescription drugs are not covered unless a Preferred Drug List exception is requested and approved (see the "Requesting an exception for noncovered prescription drugs" section). These prescription drugs have been reviewed by the Washington State Pharmacy and Therapeutics (P&T) Committee or Moda Health. HCA or Moda Health has found these drugs are not as cost-effective and do not have a clinically significant therapeutic advantage over the Preferred Drug List alternative(s).

**ALERT!** When a generic equivalent for a brand-name prescription drug becomes available, the brand-name drug **immediately** becomes noncovered. An exception must be requested and approved for coverage of the brand-name drug if you want to continue using the brand-name drug. Always ask your provider to allow substitutions on your prescriptions to save you money. See the "Substitution under Washington State law" section for information on transitioning from a brand-name to generic prescription drug.

# Requesting an exception for noncovered prescription drugs

**ALERT!** The exception process for noncovered drugs cannot be used for drugs that UMP excludes. For more information about drugs UMP excludes, see the "Prescription drugs and products UMP does not cover" and the "What the plan does not cover" sections.

If you are prescribed a noncovered drug, and you have tried all the alternative drugs and none are found to be effective, or if the alternatives are found to not be medically appropriate, you or your prescribing provider can request an exception by contacting ArrayRx Customer Service.

Your prescribing provider can also use CoverMyMeds to request an exception. CoverMyMeds is a free online platform that facilitates the review of exception requests directly through the CoverMyMeds portal. To get started, have your provider visit the CoverMyMeds website (see Directory for link).

Your prescribing provider must submit clinical information to request an exception. When the plan approves an exception based on the criteria below, you will pay 15 percent of the drug's cost per 30-day supply after you pay your deductible, except for noncovered insulins. For noncovered insulins, when the plan approves an exception based on the criteria below, you will pay the Tier 2 cost share (30 percent of the allowed amount, $35 maximum payment per 30-day supply.

If your exception request is denied, the plan's response letter will include the reason for the denial and the steps you can take next.

Preferred drug list exceptions and coverage determinations require medical information and are based on medical necessity. Therefore, your prescribing provider must submit clinical information for review and will need to provide ArrayRx with the following information:

- The prescribing provider's contact information;
- An explanation of why the plan should grant an exception;

- An explanation of how the requested medication therapy is evidence-based and generally accepted medical practice;
- Documentation of medical necessity for the requested prescription drug over all other preferred therapeutic alternatives; and
- At least one of the following step therapy requirements must also be documented and included with the exception request (see the "step therapy" section for more information):
  - Confirmation and documentation from your prescribing provider that all preferred therapeutic alternatives were tried for a clinically appropriate duration of treatment and failed to produce a therapeutic response. If the requested exception is for a brand-name prescription drug that has an FDA-approved generic equivalent, your prescribing provider must document your inadequate response to at least two manufacturers of the generic drug, or to all manufacturers of the generic drug if there are fewer than two manufacturers, in addition to all other preferred therapeutic alternatives, before an exception is granted; **or**
  - Confirmation and documentation from your prescribing provider that all preferred therapeutic alternatives caused an adverse drug reaction that prevents you from taking the prescription drug as directed. If the requested exception is for a brand-name prescription drug that has an FDA-approved generic equivalent, your prescribing provider must document your adverse drug reaction to at least two manufacturers of the generic drug, or to all manufacturers of the generic drug if there are fewer than two manufacturers, in addition to all other preferred therapeutic alternatives, before an exception is granted.

If the plan determines the information submitted confirms you have tried all the alternative drugs and none are found to be effective, or if the alternatives are found to not be medically appropriate, then your exception request may be approved. If your exception request is denied, the plans response letter will include the reason for the denial and the steps you can take next.

## How UMP decides which prescription drugs are preferred

Washington State P&T Committee and Moda Health P&T Committee provide recommendations to HCA. Moda Health and HCA review the recommendations and determine which medications are included on the UMP Preferred Drug List. The UMP Preferred Drug List includes the committees' coverage recommendations.

Not all prescription drug classes are reviewed by the Washington State P&T Committee. For these prescription drugs, the Moda Health P&T Committee makes coverage recommendations for HCA's review and final determination of coverage requirements.

For the plan to cover a prescription drug for you, it must be medically necessary for your health condition. Your provider may prescribe a drug or drug dosage that does not meet the plan's definition of medically necessary and therefore will not be covered.

**ALERT!** A prescription drug may be noncovered even if no generic equivalent is available.

## Guidelines for prescription drugs UMP covers

To be covered, a prescription drug must meet all the following:

- Is listed on the UMP Preferred Drug List (the Preferred Drug List is subject to change).

- Does not have a nonprescription alternative, including an over-the-counter alternative with similar safety, effectiveness, and ingredients.
- Is dispensed from a licensed pharmacy employing licensed, registered pharmacists.
- Is not dispensed from an excluded pharmacy.
- Has been prescribed by a provider with prescribing authority within their scope of license.
- Has been reviewed and approved by either the Washington State P&T Committee or Moda Health P&T Committee (see the "How UMP decides which prescription drugs are preferred" section).
- Is approved by the FDA.
- Is medically necessary.
- Is not classified as a vitamin, mineral, dietary supplement, homeopathic drug, or medical food.
- Is not a non-covered prescription drug or product, unless an exception is granted.
- Is not an excluded prescription drug or product.
- May be legally obtained in the U.S. only with a written prescription.
- Meets plan coverage criteria.

The plan may cover FDA-approved prescription drugs for off-label use (prescribed for a use other than its FDA-approved label) only if they are not an excluded drug or product (see the "Prescription drugs and products UMP does not cover" section), the use is not considered experimental or investigational by ArrayRx, and the use is recognized as effective for treatment:

- In a standard reference compendium and supported by peer-reviewed clinical evidence; or
- In most relevant peer-reviewed medical literature, if not recognized in a standard reference compendium; or
- By the federal Secretary of Health and Human Services.

**Note:** The plan may require that you try standard treatment(s) before it covers a prescription drug for off-label use (prescribed for a use other than its FDA-approved label).

The plan will not cover any prescription drug when the FDA has determined its use to be unsafe.

> **ALERT!** The plan does not cover prescription drugs purchased through mail-order pharmacies located outside the U.S.

## Nonprescription drugs and supplies

Your prescription drug benefit also includes the following nonprescription drugs and supplies:

- FDA-approved over-the-counter contraceptives. For the plan to cover FDA-approved over-the-counter contraceptives, you must present your UMP member ID card and make your purchase at the pharmacy counter. When possible, get a prescription, as not all pharmacies have prescribing authority. If you go to a pharmacy without a prescription or the pharmacy does not have prescribing authority, you will need to submit a claim to ArrayRx.
- Diabetes care supplies, such as test strips, lancets, insulin syringes, and continuous glucose monitors used in the treatment of diabetes. See the "Diabetes care supplies" benefit for more information.

- Other over-the-counter products that are specifically noted in the UMP Preferred Drug List as covered by the plan.
- Each month, up to two over-the-counter COVID test kits will be covered as described in the UMP Preferred Drug List.

## Products covered under the preventive care benefit

> **ALERT!** For products covered as preventive — even if normally available over-the-counter without a prescription — you must have a prescription and buy it at the pharmacy register at a network pharmacy to receive reimbursement. You will not receive reimbursement if the receipt is not from a pharmacy register. You will not receive full reimbursement for claims from non-network pharmacies. See the definition of "Out-of-network rate, prescription drugs". Prescription drugs purchased from an excluded pharmacy will not be covered. See the definition of "Excluded pharmacy."

Some products are covered under the preventive care benefit if they:

- Have an A or B grade recommendation determined by the U.S. Preventive Services Task Force (USPSTF). A and B grade recommendations are services the USPSTF has determined to have high or moderate benefit for patients (see the "Preventive care" benefit)
- Are recommended by the Advisory Committee on Immunization Practices (ACIP) of the Centers for Disease Control and Prevention (CDC) (see the "Preventive care" benefit)
- Conform to coverage guidelines (see the "How UMP decides which prescription drugs are preferred" section).

The brands and types of products covered are limited. For a list of covered preventive care products, check the Preferred Drug List, or contact ArrayRx Customer Service. You pay $0 if your provider writes you a prescription and you purchase these products from the pharmacy counter at a network pharmacy. If you purchase them at a non-pharmacy register and send in a claim, you may pay part of the cost. See the definition of "Out-of-network rate, prescription drugs".

Contraceptive drugs and supplies are covered as preventive (see the "Family planning services" benefit for details). See the "Tobacco cessation services" benefit for products covered as preventive for tobacco cessation.

## Injectable drugs available under the prescription drug benefit

Certain injectable drugs may only be available through your prescription drug benefit. You can check the UMP Preferred Drug List to see if an injectable drug is covered through your prescription drug benefit. Some injectable drugs may need to be administered by a health care provider, and these drugs are typically available through your medical benefit administered by Regence. Vaccines administered at network pharmacies may be covered under your prescription drug or medical benefit.

## Compounded prescription drugs

Compounded prescription drugs are the result of combining, mixing, or altering ingredients by a pharmacist in response to a physician's prescription to create a new drug tailored to the specialized medical needs of an individual member. Traditional compounding typically occurs when an FDA-approved prescription drug is unavailable, or a licensed health care provider decides that an FDA-approved drug is not appropriate for a member's medical needs. Non-covered ingredients require prior authorization. Excluded ingredients are not covered, but the other ingredients that are covered will be eligible for

reimbursement. Contact ArrayRx to find out whether prior authorization is required. Claims for compounded drugs require additional information submitted on the claim form. This information is available from the compounding pharmacy. You save money and avoid having to submit your own claims when you purchase these compound prescription drugs from a compounding pharmacy in the ArrayRx network. Locate a network compounding pharmacy by visiting the UMP Prescription drug coverage webpage or contacting ArrayRx Customer Service.

# What you pay for prescription drugs

**ALERT!** Hospice care: See page 50 for special prescription drug coverage while in hospice.

You will pay the entire cost of your medical services and prescription drugs until you meet your deductible. Covered preventive drugs, covered insulin, and certain other drugs as indicated on the UMP Preferred Drug List are not subject to your deductible. Once you have met your deductible, you pay 15 percent of the allowed amount for covered prescription drugs when purchased through a network pharmacy up to your out-of-pocket limit. Once you reach your out-of-pocket limit, the plan pays 100 percent of the allowed amount for covered prescription drugs at a network pharmacy. You may purchase up to a 90-day supply for most prescription drugs. For most specialty drugs, you may purchase up to a 30-day supply. You pay $0 for covered preventive drugs when you use a network pharmacy.

## Covered Insulins

For covered insulins, you must fill your prescription at a network pharmacy to pay the appropriate cost-share, even if you have not met your deductible. If you have not met your deductible, your cost-share will be applied to your deductible. See the table below for how much you will pay for covered insulins when you fill your prescription at a network pharmacy.

| Tier and description | How much you pay for covered insulins<br>All network pharmacies (retail and mail-order) |
|---|---|
| **Value Tier (Value)** | • **0–30-day supply:**<br>▪ 5% coinsurance or $10, whichever is less<br>• **31–60-day supply:**<br>▪ 5% coinsurance or $20, whichever is less<br>• **61–90-day supply:**<br>▪ 5% coinsurance or $30, whichever is less |
| **Tier 1 (Tier 1)** | • **0–30-day supply:**<br>▪ 10% coinsurance or $25, whichever is less<br>• **31–60-day supply:**<br>▪ 10% coinsurance or $50, whichever is less<br>• **61–90-day supply:**<br>▪ 10% coinsurance or $75, whichever is less |
| **Tier 2 (Tier 2)** | • **0-30 day-supply:** |

| Tier and description | How much you pay for covered insulins<br>All network pharmacies (retail and mail-order) |
|---|---|
| | ▪ 30% coinsurance or $35, whichever is less<br>• **31–60 day-supply:**<br>▪ 30% coinsurance or $70, whichever is less<br>• **61–90 day-supply:**<br>▪ 30% coinsurance or $105, whichever is less |

To check your cost, do either of the following:

- Use the Drug Price Estimator on the UMP Prescription drug coverage webpage (the prices for drugs listed in this tool assume you have met your deductible).
- Contact ArrayRx Customer Service.

See the Directory pages at the beginning of this booklet for link and contact information.

If a prescription drug is not covered by the plan, it does not apply to your deductible or your out-of-pocket limit.

**Note:** You may use your HSA to pay for many prescription drugs not covered by the plan. See the "Health savings account (HSA)" section for more information.

**Note:** You must use UMP's network mail order pharmacies to fill mail-order prescriptions. If you use any other mail-order service to fill your prescription drugs, including mail-order services outside of the U.S., the plan will not cover these drugs if UMP is your primary plan.

**ALERT!** When you use network pharmacies, retail or mail-order, you pay based on the prescription drug's allowed amount, a discounted price negotiated by the plan. If you use a non-network pharmacy, you will be reimbursed at the out-of-network rate for covered prescription drugs. See the definition of "out-of-network rate, prescription drugs." If the pharmacy charges more than the plan's allowed amount, you will have to pay this additional amount, which does not apply to your prescription drug deductible or out-of-pocket limit. Prescriptions purchased from an excluded pharmacy are not covered. See the definition of "Excluded pharmacy." Discount cards cannot be used together with your UMP benefits for covered drugs.

# If you have other prescription drug coverage

If you have primary medical coverage through another plan that covers prescription drugs, some of the limits and restrictions to prescription drug coverage listed on page 106 will apply when UMP pays secondary to another plan. See the "Submitting a claim for prescription drugs" section for how to submit your prescription drug claim.

## Using network pharmacies when UMP is secondary

If you have primary coverage through another plan that covers prescription drugs, show both plan cards to the pharmacy and make sure they know which plan is primary. It is important that the pharmacy bills the plans in the correct order, or claims may be denied or paid incorrectly.

### Using mail-order pharmacies when UMP is secondary

If your primary plan uses UMP's network mail-order pharmacies, these pharmacies may process payments for both plans and charge you only what is left. Make sure that UMP's network mail-order pharmacies have your information for both plans and know which plan is primary.

However, if your primary plan uses a different mail-order pharmacy than what is in network with UMP, you will have to use your primary plan's mail-order pharmacy, then submit a claim for reimbursement. See the "Submitting a claim for prescription drugs" section for how to do this.

## Where to buy your prescription drugs

> **ALERT!** If you use a non-network retail pharmacy, you will pay the entire cost of the prescription drug at the time of purchase and must submit a claim for reimbursement. However, only the allowed amount for covered drugs will apply to your deductible or out-of-pocket limit, if applicable. Excluded drugs or products and prescriptions purchased from an excluded pharmacy are not covered.

Pharmacies are contracted through a different network than medical providers.

## Retail pharmacies

ArrayRx has a large national network of retail pharmacies, which includes many independent and regional pharmacies in Washington State, as well as national chains. Search for a network pharmacy on the UMP Prescription drug coverage webpage or contact ArrayRx Customer Service (see Directory for link and contact information).

When you get your prescriptions at a network pharmacy, the pharmacy sends the claim to the plan for you, and you pay only your cost-share (coinsurance and deductible) as described in the "What you pay for prescription drugs" section. There are no network pharmacies outside the United States (U.S.). Drugs purchased at pharmacies outside the U.S. will be reimbursed at the out-of-network rate (see definition of "Out-of-network rate, pharmacies").

**Note:** You will pay the entire cost for any prescription drug not covered by the plan, which does not apply to your deductible or your out-of-pocket limit.

> **TIP:** If you take a prescription drug regularly, you may be able to save money by filling up to a 90-day supply at a network retail pharmacy or through UMP's network mail-order pharmacies. Search for a network pharmacy and compare prices on the UMP Prescription drug coverage webpage.

### Network vaccination pharmacies

Many network retail pharmacies have vaccination pharmacists who may give covered preventive immunizations at no cost to you. Contact a network pharmacy ahead of time to make sure the pharmacy has the vaccine you need and offers vaccine services. Present your UMP member ID card at the pharmacy counter before receiving a vaccine.

# Mail-order pharmacies: Costco Mail-Order Pharmacy and Postal Prescription Services (PPS)

> **ALERT!** UMP's network mail-order pharmacies cannot ship outside of the U.S. See the "Travel overrides for prescription drugs" section if you will be traveling.

Costco Mail-Order Pharmacy and PPS are the plan's **only** network mail-order pharmacies. For more information about mail-order, visit the UMP Prescription drug coverage webpage, contact UMP's network mail-order pharmacies, or contact ArrayRx Customer Service. See the Directory pages at the beginning of this booklet for links and contact information.

**Note:** You may mail in a prescription, however only a provider can call, fax, or electronically submit a prescription to the pharmacy. You must follow these instructions to avoid a delay in filling your prescription.

> **ALERT!** The plan does not cover other mail-order pharmacies outside of UMP's network mail-order pharmacies if UMP is your primary insurance. If UMP is your secondary insurance, you may use another mail-order pharmacy.

Refills may be ordered through UMP's network mail-order pharmacies. When using UMP's network mail-order pharmacies, the same deductible, coinsurance, prior authorization requirements, and limits on coverage apply as for prescription drugs purchased at retail network pharmacies.

Drugs can be expensive. If you are not able to be home to receive the drug in the mail, it may be a good idea to request that it be sent with a requirement for a signature, or find an alternate point of delivery to ensure the package is not stolen. The plan is not responsible for replacement of prescription drugs or products that are lost, stolen, expire, or unusable after they are delivered to you (see the "What the plan does not cover" section).

If you use a mail-order pharmacy and your prescription is either delayed more than one calendar day after the original delivery date, or arrives in an unusable condition (and you have confirmed with a pharmacist that it is unsafe to use), you may contact ArrayRx Customer Service to fill your prescription drug at another network pharmacy. To access this option, contact ArrayRx Customer Service and advise that your mail-order prescription was delayed or arrived in an unusable condition. ArrayRx will help you obtain your prescription drug at another network pharmacy. Your prescription may need to be transferred from the mail-order pharmacy to the other network pharmacy, or your provider may need to send a new prescription to the other network pharmacy. If you later choose to return to the mail-order pharmacy for refills, you may need to contact the mail-order pharmacy to transfer the prescription back. Some controlled substances may have limits under state or federal law for transferring prescriptions.

Prescriptions mailed, or orders placed in December but not filled until January 1 or after, are subject to your deductible applicable on the date the prescription is processed. Because of increased volume at the end of the year, prescriptions submitted to the network mail-order pharmacies in December may not be processed during the current plan year.

If there is a shortage of a specific prescription drug that UMP's network mail-order pharmacies cannot control, or they do not have the quantity you ordered, the network mail-order pharmacies will contact you to discuss your options for obtaining your prescription(s).

**ALERT!** Some Durable Medical Equipment (DME) items are not available through UMP's network mail-order pharmacies. You will need to get them through a network retail pharmacy or preferred DME provider.

## Specialty drugs

**ALERT!** Specialty drugs can be expensive. If you are not able to be home to receive the drug in the mail, it may be a good idea to call the specialty pharmacy and request that it be sent with a requirement for a signature or find an alternate point of delivery to ensure the package is not stolen or damaged. The plan is not responsible for replacement of prescription drugs or products that are lost, stolen, expire, or unusable after they are delivered to you (see the "What the plan does not cover" section).

Specialty drugs are typically used to treat complex, rare conditions or chronic diseases. These drugs often have additional requirements including but not limited to special handling, storage, or administration (injectable, infused, inhaled). You may find out if a drug is a specialty drug by checking the UMP Preferred Drug List on the UMP Prescription drug coverage webpage, or by contacting ArrayRx Customer Service (see Directory for link and contact information).

You may purchase your specialty drugs through the plan's network specialty drug pharmacy. If that pharmacy does not have access to a specialty drug, you will be notified to fill your prescription at another specialty drug pharmacy. If the network specialty drug pharmacy gains access to the specialty drug, you will receive notification to transfer your prescription to that pharmacy.

You may receive up to a 30-day supply for most specialty drugs per prescription or refill. However, some may be limited to a 15-day supply due to high discontinuation rate, short duration of use, or to make sure that the prescription drug is not causing harmful side effects.

Specialty drugs require prior authorization. See the "Preauthorization of prescription drugs" section for how to request prior authorization. A patient care coordinator will work with you to schedule a delivery time for the prescription drug. The specialty drug pharmacy will deliver your prescription drugs anywhere in the U.S. that you choose, such as to your workplace or to a neighbor if you cannot be home for the delivery. The plan is not responsible for replacement of prescription drugs or products that are lost, stolen, expire, or unusable after they are delivered (see the "What the plan does not cover" section).

If your provider will administer the specialty drug, you may have it shipped to the provider's office. However, once the provider's office receives the specialty drug, the provider takes responsibility for it.

The plan's network specialty drug pharmacy cannot ship outside the U.S. See the "Travel overrides for prescription drugs" section if you will be traveling.

# How to get the plan discount

Be sure to fill your prescriptions at a network pharmacy and show your ID card at the pharmacy counter. If you do, you pay for covered prescription drugs based on the allowed amount (ArrayRx's standard reimbursement). If you do not show your UMP member ID card at a network pharmacy or you use a non-network pharmacy, then you may be charged a higher amount. If the amount charged is more than the UMP allowed amount at a network pharmacy or out-of-network rate at a non-network pharmacy, you will pay the difference in addition to your coinsurance. Prescriptions purchased from an excluded pharmacy, excluded drugs, and excluded products are not covered.

If you have more than one insurance plan and UMP is your secondary insurance, show both plan member ID cards at the pharmacy and make sure they know which plan pays first so the pharmacy may bill the plans in the correct order.

## Non-network pharmacies: Retail

You will save money when you buy your prescriptions at network retail pharmacies and network mail-order pharmacies. You may buy your prescriptions (except specialty drugs) at a non-network retail pharmacy, but you will pay more if you do. If you get your prescriptions filled at a non-network retail pharmacy:

A. You will have to pay the full price for your prescription drugs and you will need to submit your claim to ArrayRx for reimbursement (see the "Submitting a claim for prescription drugs" section).

B. You do not get the plan discount.

C. You will pay the difference between the out-of-network rate for prescription drugs and what the pharmacy charges, and it will not apply to your deductible or out-of-pocket limit. See definition of "out-of-network rate, prescription drugs."

D. What the plan pays is based on the allowed amount for prescription drugs covered by the plan, whether from a network or non-network retail pharmacy.

E. Non-network pharmacies will not know if a prescription drug requires prior authorization, has a quantity limit, or has other coverage limits. If you purchase a drug from a non-network pharmacy and limits apply, the plan may not cover or reimburse it.

F. Prescriptions purchased from an excluded pharmacy will not be covered. See the definition of "Excluded pharmacy."

G. Specialty drugs must be purchased from the plan's network specialty drug pharmacy. See the "Specialty drug" section.

**TIP:** To submit claims for prescriptions purchased from non-network pharmacies (U.S. retail or international retail pharmacies), see the "Submitting a claim for prescription drugs" section.

## Prescription drugs purchased outside the U.S.

If you purchase prescription drugs outside the U.S. for any reason, the following rules apply:

- If the prescription drug is available only by prescription in the U.S. but does not require one outside the U.S., the drug is covered only if prescribed by a provider practicing within their scope of practice.
- If you get a prescription drug that is approved for use in another country but not in the U.S., the plan will not cover it.
- If you get a prescription drug that is available over-the-counter in the U.S., the plan will not cover the drug, even if you have a prescription from a provider prescribing within their scope of practice. The plan does not cover most over-the-counter drugs.
- If you get a prescription drug that is not covered by UMP, or is an excluded drug or product, the plan will not pay any amount of the cost of the drug. You will be responsible for the full cost of the drug.
- There are no network pharmacies outside the U.S. All claims for drugs purchased outside the U.S. will be based on the out-of-network rate for that drug. Balance billing may apply which does not count towards your deductible or out-of-pocket limit.

- Letters A-F of the above section ("Non-network pharmacies: Retail") applies to this section.

To submit a claim for a prescription drug purchased at pharmacies outside the U.S., see the "Submitting a claim for prescription drugs" section. All necessary information must be included on the prescription drug claim form with drugs and dosage documented.

## Limits on your prescription drug coverage

ArrayRx may exclude, not cover, discontinue, or limit coverage for any prescription drug or manufacturer's version of a drug for any of the following reasons:

- A more cost-effective alternative is available to treat the same condition.
- A nonprescription alternative, including an over-the-counter alternative, becomes available.
- A prescription drug receives FDA approval for a new use.
- A prescription drug is used off label to treat an indication that is not determined to be medically necessary by the plan.
- A prescription drug is purchased from an excluded pharmacy.
- Generic, biosimilar, interchangeable biosimilar, or follow-on biologic prescription drugs become available.
- New prescription drugs are developed.
- The FDA denies, withdraws, or limits the approval of a prescription drug.
- The FDA's Drug Efficacy Study Implementation (DESI) classifications find a prescription drug to be less than effective.
- The Washington State P&T Committee or Moda Health P&T Committee recommends a change (see the "How UMP decides which prescription drugs are preferred" section).
- There is a sound medical reason.
- There is lack of scientific evidence that a prescription drug is as safe and effective as other existing drugs used to treat the same or similar conditions.
- There is new scientific evidence demonstrating a prescription drug has been found to be less safe or effective than existing drugs to treat the same or similar conditions.

Using free prescription drug samples does not guarantee coverage or waive requirements for prior authorization, step therapy, quantity limits, day supply limits, or other limitations.

Prescription drugs will only be covered if:

- They are not excluded by the plan, are listed on the UMP Preferred Drug List, and are medically necessary for your health condition; or
- You received an exception to cover a noncovered drug.

Although your provider may prescribe a prescription drug that is not considered medically necessary, the above conditions must be met in order for a prescription drug to be covered.

The plan excludes experimental or investigational prescription drugs. You may be liable for all charges if you receive prescription drugs or products that are determined to be experimental or investigational (see the "What the plan does not cover" section). If you disagree with the plan's determination, you have the right to an appeal (see the "Complaint and appeal procedures" section for that process).

The limits and restrictions described in the "Limits on your prescription drug coverage" section help ArrayRx monitor drug usage, safety, and costs. These limits and restrictions may be added or removed

from prescription drugs at any time. You may find out if your prescription drug falls under any of these limits and restrictions by checking the UMP Preferred Drug List on the UMP Prescription drug coverage webpage or contacting ArrayRx Customer Service (see Directory for link and contact information).

## Risk Evaluation and Mitigation Strategies (REMS)

Risk Evaluation and Mitigation Strategy (REMS) is a drug safety program that the FDA can require for certain prescription drugs with serious safety concerns to help make sure the benefits of the drug outweigh its risks. REMS are designed to reinforce prescription drug use behaviors and actions that support the safe use of that drug. While all drugs have labeling that describe possible risks, only a few prescription drugs require a REMS. REMS are not designed to mitigate all the adverse events of a prescription drug. These are communicated to health care providers in the drug's prescribing information. Rather, REMS focuses on preventing, monitoring, and/or managing a specific serious risk by informing, educating, and/or reinforcing actions to reduce the frequency and/or severity of the event.

If the REMS program is not followed, the plan may not cover the restricted drug.

## Preauthorization of prescription drugs

Preauthorization is a process that helps make sure that prescription drug benefits are administered as designed and that plan members receive a drug therapy that is safe and effective for their conditions, and provides the greatest value. Some prescription drugs require preauthorization to determine whether they are medically necessary and meet all applicable coverage criteria, or the plan will not cover them. You may find out if your prescription drug requires preauthorization by contacting ArrayRx Customer Service or checking the UMP Preferred Drug List on the UMP Prescription drug coverage webpage (see the Directory pages at the beginning of this booklet for links and contact information). You and your prescribing provider may also find the coverage criteria for your prescription drug by referring to the *UMP Prescription Drugs (ArrayRx) preauthorization requirements* on forms and publications at **hca.wa.gov/ump-forms-pubs**.

If your prescription drug requires preauthorization, your pharmacist or prescribing provider may initiate a request through CoverMyMeds, a free online platform that accepts requests from electronic health records. To get started, your pharmacist or prescribing provider can go to the CoverMyMeds website. They may also contact ArrayRx Customer Service to request it.

If you have an existing authorization from UMP for a brand-name drug, and a generic drug becomes available, you may need to renew your authorization to continue filling the brand-name drug. However, if you switch to the generic drug, a new preauthorization is not required until the existing authorization expires.

**Note:** Prescription drugs covered under the medical benefit rather than the prescription drug benefit have different rules for preauthorization. Contact UMP Customer Service for details.

## How long the plan has to make a decision

The plan will respond to standard prior authorization requests submitted by providers within five days of receipt for non-electronic requests and within three days of receipt of electronic requests, excluding major holidays. You will be notified of the decision by mail. If your provider believes that waiting for a decision under the standard prior authorization timeframe could place your life, health, or ability to regain maximum function in serious danger, they can request an expedited prior authorization request. For expedited electronic prior authorization requests, the plan will respond within one day of receipt. For expedited non-electronic prior authorization requests, the plan will respond within two days of receipt. If additional information is required, the plan will notify the provider within the timelines described in this section. You will also be notified of the decision.

# Emergency fill

Emergency fills let you get a limited quantity of certain prescription drugs while the plan processes your prior authorization request. This option is only available when a delay could result in emergency care, hospital admission, or a serious threat to your health or others in contact with you. Contact ArrayRx Customer Service for questions about which prescriptions drugs may qualify for emergency fills.

You must call ArrayRx to initiate a request for an emergency fill (while the prior authorization is being processed). If the emergency fill request is approved, you pay 15 percent coinsurance after you pay your deductible.

The plan may cover an emergency fill of up to a seven-day prescription drug supply.

If your prior authorization request is denied, you will pay the full cost of the prescription drug for any quantity you receive after the emergency fill.

## Emergency fill limits

The following limits still apply to emergency fill prescription drugs:

- Quantity limits: You cannot get more than the stated quantity limit under an emergency fill. If you have a current filled prescription for a prescription drug (or its therapeutic equivalent) and it was filled to the quantity limit, you cannot get an emergency fill until you have used 84 percent or more of the filled prescription.
- Refill too soon: If you have a filled prescription for a prescription drug (or its therapeutic equivalent), you cannot get an emergency fill until you have used 84 percent or more of the filled prescription.

# Quantity limits

Certain prescription drugs have a per prescription limit on how much you get for each fill. If you need more than this limit allows, your pharmacist or prescribing provider may initiate a request through CoverMyMeds on the CoverMyMeds website or contact ArrayRx Customer Service. See the Directory pages at the beginning of this booklet for links and contact information.

If ArrayRx denies your request or your provider or pharmacist does not get prior authorization, the plan will cover the prescription drug only up to the quantity limit amount. You will pay for any extra amount.

# Refill too soon

The plan will not cover a refill until 84 percent of the last prescription has been used. Claims for therapeutic equivalents of the previously prescribed drug will also be denied. This also applies if your prescription is, lost, stolen, or unusable. For example, if you get a 90-day supply and you try to refill this prescription before 76 days have passed, coverage will be denied.

However, in the event of an emergency or other urgent circumstance, you may request an exception to override the refill too soon policy. The plan may require documentation to support your request. Approval of your request is at the sole discretion of the plan.

For mail-order prescriptions, the plan will cover a fill at a network pharmacy if the prescription is delayed more than one day after the expected delivery date provided by the mail-order pharmacy, or if the prescription arrives in an unusable condition.

# Prescription drug discount cards

Prescription drug discount cards are not considered insurance and cannot be used in combination with your plan. Prescription drugs purchased using a prescription drug discount card will not be covered by the plan, even if you pay out of pocket and submit a claim. If you use a prescription drug discount card and

the prescription drug is covered by your plan, the amount you pay will not apply to your prescription drug deductible or prescription drug out-of-pocket limit.

Show your UMP member ID card first at the pharmacy counter for covered prescription drugs. If a prescription drug is not covered by the plan, you may use a prescription drug discount card to save money. Be sure to verify at the pharmacy which card they are using to fill your prescriptions to avoid accidentally using the prescription discount drug card for prescription drugs covered under your plan.

**Important:** If a pharmacy uses a discount card for your prescription, UMP will not cover it even if you paid for it yourself and then submit a claim for reimbursement. If you plan to ask for reimbursement, check with the pharmacy when you pick up your prescription medication to make sure they did not use a discount card.

## Step therapy

The step therapy program is a type of prior authorization requirement to lower risk and ensure utilization of cost-effective prescription drugs before approving other covered prescription drugs. When a prescription drug is part of the step therapy program, you have to try certain drugs (Step 1) before the plan will cover the prescribed drug (Step 2). When a prescription for a step therapy drug is submitted "out of order," meaning you have not first tried a Step 1 drug before submitting a prescription for a Step 2 drug, the plan will not cover your prescription. When this happens, your provider will need to prescribe a Step 1 drug for you.

If you or your provider feels that you need the Step 2 prescription filled as originally written without first trying a Step 1 drug, your pharmacist or prescribing provider may contact ArrayRx Customer Service and request coverage. You will have to pay the entire cost of the prescription drug if you have not tried a Step 1 drug and coverage has not been authorized before you get a Step 2 drug. For certain prescription drugs, you may be required to try more than one Step 1 drug. The drug originally prescribed may be approved if the other options are determined to be ineffective or not tolerated.

To find out if step therapy applies to your drug, check the UMP Preferred Drug List on the UMP Prescription drug coverage webpage, or contact ArrayRx Customer Service (see Directory for link and contact information).

**Note:** Only network pharmacies will check to see if step therapy applies to your prescription drug. If you get a Step 2 drug at a non-network pharmacy, the plan may not cover the drug.

## Substitution under Washington State law

> **ALERT!** New generic prescription drugs are released throughout the year. If you want to save money by using generics, ask your provider to allow substitutions on your prescriptions, even if a generic drug is not available now. That way, when one becomes available, the pharmacist may automatically refill with the generic.

When a brand-name or biologic prescription drug has a generic equivalent or interchangeable biosimilar, pharmacists in Washington State must substitute the generic equivalent or interchangeable biosimilar drug for the brand-name or biologic prescription drug. When a generic equivalent for a brand-name prescription drug becomes available, the brand-name drug immediately becomes noncovered.

Your provider may write the prescription "dispense as written" if they want you to get only the prescribed brand-name or biologic prescription drug. Or you may tell the pharmacist you want the brand-name or biologic drug. Regardless of whether you or your prescribing provider ask the pharmacist to "dispense as

written," if you get the noncovered prescription drug, the plan may not cover it. To request an exception for a noncovered drug, see the "Requesting an exception for noncovered prescriptions" section.

For FDA-approved contraceptives, final determination of medical necessity is determined by the attending prescribing provider.

### Therapeutic Interchange Program

The Washington State Therapeutic Interchange Program allows a pharmacist to substitute a "therapeutic alternative" drug for a noncovered drug in certain cases. Therapeutic alternatives are drugs that are chemically different from your prescribed drug but provide the same therapeutic benefit. The pharmacy will contact your provider to request authorization for the substitution. If approved by the provider, you will receive the alternative preferred drug. If the pharmacy cannot get an authorization from your provider within 48 hours, the prescription will be filled as written, and you will be charged the full price of the drug.

You may find out if your prescription drug is affected by the Therapeutic Interchange Program by checking the UMP Preferred Drug List on the UMP Prescription drug coverage webpage or by contacting ArrayRx Customer Service (see Directory for link and contact information). The Therapeutic Interchange program does not affect all noncovered prescription drugs.

The pharmacist will substitute the preferred drug when your prescribing provider has "endorsed" the Washington Preferred Drug List, and:

- You are filling your prescription in Washington State or through UMP's network mail-order pharmacies.
- Your prescribing provider allows substitution on your prescription.

If you do not want your prescription drug to be changed, the plan may not cover your drug if you ask the pharmacist to fill the prescription as written.

Regardless of whether you or your prescribing provider ask the pharmacist to "dispense as written," if you get the noncovered prescription drug, the plan may not cover it.

## Travel overrides for prescription drugs

You may request a travel override to get an extra supply of prescription drugs for extended travel. All the conditions listed below apply.

- You may request a travel override up to two weeks before your departure.
- You may be authorized for no more than two travel overrides per calendar year, including all travel within or outside the U.S.
- When traveling within the U.S., you may request up to a 90-day supply per prescription, or as allowed under that prescription.
- When traveling outside the U.S., you may request up to a six-month supply per prescription, or as allowed under that prescription.
- Travel overrides will be granted only while you are covered by the plan. If your eligibility is ending, the plan does not cover prescription drugs past the time your enrollment in the plan ends.
- You will pay applicable charges (deductible and coinsurance) for each extra supply received.

To request a travel override, contact ArrayRx Customer Service. You are responsible for the safe storage and handling of drugs when traveling as well as verifying laws and regulations in areas you will be traveling for prescription drugs. The plan is not responsible for replacement of lost, stolen, expired, or damaged prescription drugs or products (see the "What the plan does not cover" section).

## Early refill for a natural disaster

You may request an early refill for your prescription when you need to evacuate for a natural disaster. To request an early refill or to locate pharmacies that remain open near you, contact ArrayRx Customer Service.

# What to do if the plan denies coverage

> **TIP:** If your prescription coverage is denied by the pharmacy due to plan enrollment issues, contact:
> **Employees:** Your employer's payroll, or benefits office.
> **Retirees, PEBB Continuation Coverage, and retired employees of a former employer group:** PEBB Program at 1-800-200-1004 (TRS: 711).

If a network pharmacy (including mail-order or specialty drug) tells you that prior authorization is required, coverage is denied, or quantities are limited, you, your pharmacist, or your prescribing provider may contact ArrayRx Customer Service to request a coverage review or prior authorization.

If ArrayRx denies the coverage request or prior authorization, you have the right to submit an appeal (see the "Complaint and appeal procedures" section).

If your provider thinks you need the prescription drug immediately, they may request an expedited review by submitting all clinically relevant information (i.e. supporting documentation and chart notes) to the plan by phone or fax. ArrayRx will decide on coverage of the prescription drug within one calendar day of the expedited request.

# Prescription drugs and products UMP does not cover

Prescription drugs and products not covered under the prescription drug benefit include, but are not limited to, noncovered prescription drugs, drugs purchased at an excluded pharmacy, and excluded drugs and products.

## Noncovered prescription drugs

- Noncovered prescription drugs are not covered without an approved UMP Preferred Drug List exception request

For more information, see the "Noncovered prescription drugs" section.

## Excluded drugs and products

- Dental preparations, such as rinses and pastes.
- Dietary/food supplements, vitamins, minerals, herbal supplements, and medical foods.
- Experimental or investigational prescription drugs.
- Homeopathic drugs, including FDA-approved prescription products.
- Over-the-counter drugs, products containing an over-the-counter drug, or prescription drugs that have a nonprescription alternative, except for the drugs listed on the UMP Preferred Drug List.

  **Note:** Prescription drugs with a nonprescription alternative — including an over-the-counter alternative having similar safety, efficacy, and ingredients — are excluded.
- Over-the-counter products not approved by and registered with the FDA.

- Prescription drug costs covered by other insurance (see page 106 for coordination with other plans).
- Prescription drugs not approved by the FDA.
- Prescription drugs prescribed for excluded conditions.
- Prescription drugs provided to a member, in whole or in part, while the member is admitted to an inpatient facility. Drugs provided in an inpatient setting are covered under the medical benefit.
- Prescription drugs that are not medically necessary.
- Prescription drugs that are repackaged.
- Prescription drugs that the FDA's DESI classifications have found to be less than effective.
- Prescription drugs under a REMS program required by the FDA when prescribed outside REMS guidelines (see the "Risk Evaluation and Mitigation Strategies (REMS)" section for details).
- Most products considered as a medical device by the FDA. Medical devices may be covered under your medical benefit.

The plan also excludes prescription drugs to treat conditions that are not covered under the medical benefit. These include, but are not limited to, prescription drugs for:

- Cosmetic purposes.
- Fertility or infertility.
- Obesity (or weight loss).
- Promoting hair growth.
- Sexual dysfunction.

# Limits on plan coverage

If you receive a service that is not medically necessary, is experimental or investigational, is listed as an exclusion in the "What the plan does not cover" section, or is listed as a noncovered or excluded prescription drug, you are responsible for paying all associated charges.

## Prior authorization of medical services

> **ALERT!** This section does not apply to prescription drugs. See page 83 for how to request prior authorization of drugs covered under the prescription drug benefit.

Prior authorization is required for some medical services and supplies to determine whether the service or supply meets the plan's medical necessity criteria to be covered. **The fact that a service or supply is prescribed or furnished by a provider does not, by itself, make it a medically necessary covered service.** Prior authorization is not a guarantee of coverage.

A change after the plan has approved a prior authorization request — including, but not limited to, a change of provider or different/additional services — requires your provider to submit a new prior authorization request and for the plan to approve it.

If your prior authorization is denied, your provider may request a peer-to-peer review where they can talk to a Regence BlueShield provider about your condition prior to submitting an appeal. For more information contact UMP Customer Service.

## Your prior authorization role

**ALERT!** Excluded, experimental, and investigational services do not require prior authorization because they are not covered by the plan. To confirm whether a service is covered, contact UMP Customer Service.

To be covered, some services, including, but not limited to, Applied Behavior Analysis (ABA) Therapy for members age 18 or older (see page 36) and bariatric surgery (see page 37), must have prior authorization before services are received. A preferred or participating provider may be required to request prior authorization before providing services. Contact UMP Customer Service to ask if a service requires prior authorization and how to submit a request.

An out-of-network provider is not obligated to obtain prior authorization for services that require prior authorization because they do not have a contract with Regence. If an out-of-network provider does not obtain required prior authorization in advance of the service, you may be responsible for all associated charges.

You are encouraged to request that an out-of-network provider obtain prior authorization for certain services on your behalf to determine medical necessity before the services are provided. They have the clinical details and technical billing information needed to submit a request.

You may be liable for all charges if you receive services that are determined to be not medically necessary, experimental or investigational, or not covered under this plan (see the "What the plan does not cover" section).

**ALERT!** See the "Complaint and appeal procedures" section for how to appeal denial of a prior authorization request before receiving services.

## List of services and supplies requiring prior authorization or notice

For a list of services and supplies requiring prior authorization or plan notice:

- Visit the Policies that affect your care webpage.
- Contact UMP Customer Service to request a printed list or ask questions.

See the Directory pages at the beginning of this booklet for the webpage link and contact information.

**ALERT!** The UMP prior authorization list is updated throughout the year. The fact that a service does not require prior authorization or notice does not guarantee coverage.

## Site of care

For certain covered services, the plan may require that services be performed at a specific site of care (such as an outpatient hospital setting) to be covered. When a site of care review is required, the plan evaluates whether the requested site of care is medically necessary based on your individual health status

and clinical criteria, or whether the services could be safely provided at a less intensive setting, such as an ambulatory surgery center (ASC).

If a service is performed at a site of care that has not been approved by the plan, or if prior authorization for the site of care is not obtained in advance, the plan may deny coverage for that service and the member may be responsible for all associated charges.

For a list of services subject to site of care review, visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information.

## Notice for facility admissions

Your provider must notify the plan upon your admission to a facility for services requiring plan notice. For a list of services requiring plan notice visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information. Facility admissions for which the plan is not notified may not be covered. Notice is usually done by the facility at the time you are admitted. Notice is not the same as prior authorization and many services require both.

## What is the difference between prior authorization and notice?

> **ALERT!** Many services, including, but not limited to, inpatient services, require both prior authorization and notice. Contact UMP Customer Service or talk to your provider if you have questions about services needing prior authorization or notice.

"Prior authorization" is plan approval for coverage of specific services, supplies, or medical prescription drugs before they are provided to the member. Prior authorization is usually requested by the provider performing the services. The plan sends either an approval or denial of coverage. Prior authorization is not a guarantee of coverage.

If the plan does not approve services that require prior authorization before services are received, the plan may deny coverage. The plan does not approve or deny prior authorization for services that are not on the UMP prior authorization list.

"Notice" means that your provider must contact the plan to let us know when you receive services. Notice is usually done by the facility when you are admitted.

> **ALERT!** If the plan denies prior authorization and you receive those services anyway, you are responsible for the provider's entire billed charge.

## How long the plan has to make a decision

The plan will respond to standard prior authorization requests submitted by contracted providers within five days of receipt for non-electronic requests and within three days of receipt for electronic requests. For expedited prior authorization requests, the plan will respond within one to two days of receipt. If additional information is required, the plan will notify the provider within the timelines described in this section. You will also be notified of the decision.

If your provider believes that waiting for a decision under the standard prior authorization timeframe could place your life, health, or ability to regain maximum function in serious danger, they can request an expedited prior authorization request.

## General information from UMP Customer Service

For services not requiring prior authorization, you may contact UMP Customer Service to ask if a particular service is generally covered by the plan, and for an estimate of how much you will pay. **The plan does not approve or deny prior authorization for services that are not on the UMP prior authorization list.**

Until a claim is submitted and reviewed, the plan cannot guarantee that your service will be covered or give you an exact amount you will pay out of pocket. This is because when a provider bills for a service, the plan pays for it based on procedure codes. Each code describes a service in some detail, and there are many codes for similar-sounding services. Your provider, not the plan, determines which of these codes is used on the submitted claim.

### Alternative benefits

Alternative benefits mean benefits for services or supplies that are not otherwise covered as specified in this COC, but for which the plan may approve coverage after case management evaluation. The plan may cover alternative benefits through case management (see the "Care management" benefit) if the plan determines that alternative benefits are medically necessary and will result in overall reduced covered costs and improved quality of care.

Alternative benefits are approved on a case-specific basis only. Approval of an alternative benefit applies to only the services and member listed in the alternative benefit approval letter. The rest of this COC remains in effect.

# What the plan does not cover

> **TIP:** If you have any questions about services the plan does not cover, contact UMP Customer Service or ArrayRx Customer Service.

This plan covers only the services and conditions specifically identified in this COC. Unless a service or condition fits into one of the specific benefit definitions, it is not covered. You may pay all costs associated with a noncovered service.

Here are some examples of common services and conditions that are not covered. Many others are also not covered — these are examples only, not a complete list. These examples are called exclusions, meaning these services are not covered, even if the services are medically necessary.

1. Activity therapy services including, but are not limited to:
    - Aroma;
    - Creative arts;
    - Dance;
    - Equine or other animal-assisted;
    - Music;
    - Play;

- Recreational or similar therapy; and
- Sensory movement groups.

2. Air ambulance, if ground ambulance would serve the same purpose
3. Ambulance (all types), to move you to a facility closer to your home or for purposes that are not medically necessary
4. Assisted reproductive technologies, regardless of underlying condition or circumstance, including, but not limited to:
    - Infertility or fertility testing or treatment after initial diagnosis;
    - Complications resulting from infertility or fertility treatment;
    - Cryogenic or other preservation, storage and thawing (or comparable preparation) of egg, sperm, or embryo;
    - In Vitro Fertilization (IVF) and all related services and supplies (including all procedures involving selection of embryo for implantation);
    - Artificial insemination;
    - Embryo transfer;
    - Other artificial means of conception; or
    - Any associated surgery, visits for infertility, prescription drugs, testing or supplies.
5. Autologous blood and platelet-rich plasma injections
6. Bariatric surgery using intragastric balloons.
7. Bone growth stimulators for:
    - Nonunion of skull, vertebrae, or tumor related
    - Ultrasonic stimulator – delayed fractures and concurrent use with another noninvasive stimulator.
8. Bone morphogenetic protein-7 (rhBMP-7) for use in lumbar fusion
9. Bronchial thermoplasty for asthma
10. Carotid artery stenting of intracranial arteries
11. Carotid intima media thickness testing
12. Catheter ablation for non-reentrant supraventricular tachycardia
13. Cervical spinal fusion for Degenerative Disc Disease for neck pain without evidence of radiculopathy or myelopathy
14. Complications arising directly from services that would not be covered by the plan during the current plan year. The plan will cover complications arising directly from services that a PEBB plan covered for you in the past.
15. Continuous glucose monitoring for adults and children with type 2 diabetes not on insulin
16. Corneal Refractive Therapy (CRT), also called Orthokeratology
17. Coronary or cardiac artery calcium scoring
18. Cosmetic services or supplies, including drugs and pharmaceuticals, unless part of the following care:
    - Reconstructive breast surgery following a mastectomy necessitated by disease, illness, or injury

- Reconstructive surgery of a congenital anomaly, such as cleft lip or palate, to improve or restore function

19. Court-ordered care, unless determined by the plan to be medically necessary and otherwise covered
20. Custodial care (see definition on page 158)
21. Deep brain stimulation and transcranial direct current stimulation when used as nonpharmacological treatments for treatment-resistant depression
22. Dental care for the treatment of problems with teeth or gums, other than the specific covered dental services (see pages 41–42)
23. Dietary/food supplements, including, but not limited to:
    - Herbal supplements, dietary supplements, and homeopathic drugs
    - Infant or adult dietary formulas
    - Medical foods (except when prescribed for inborn errors of metabolism)
    - Minerals
    - Prescription or over-the-counter vitamins (see page 75)
24. Dietary programs
25. Discography for patients with chronic low back pain and lumbar degenerative disc disease. This does not apply to patients with the following conditions:
    - Degenerative disease associated with significant deformity
    - Fracture, tumor, infection, and inflammatory disease
    - Functional neurologic deficits (motor weakness or Electromyography [EMG] findings of radiculopathy)
    - Isthmic spondylolysis
    - Primary neurogenic claudication associated with stenosis
    - Radiculopathy
    - Spondylolisthesis greater than Grade 1
26. Drugs or medicines not covered by the plan, as described in the "Your prescription drug benefit" section, see pages 70–88
27. Drugs or medicines obtained through mail-order pharmacies located outside the U.S.
28. Educational programs, except as described under:
    - "Diabetes education" on page 42
    - "Omada for Diabetes" on page 57
    - "Omada for Prevention and Weight Health" on page 57
    - "Tobacco cessation services" on page 66
29. Electrical Neural Stimulation (ENS), which includes Transcutaneous Electrical Nerve Stimulation (TENS) units, outside of medically supervised facility settings (e.g., in-home use).
30. Email consultations or e-visits, except as described under the telemedicine benefit.
31. Equipment not primarily intended to improve a medical condition or injury, including, but not limited to:

- Air conditioners or air purifying systems
- Arch supports
- Communication aids
- Elevators
- Exercise equipment
- Massage devices
- Overbed tables
- Residential accessibility modifications
- Sanitary supplies
- Telephone alert systems
- Vision aids
- Whirlpools, portable whirlpool pumps, or sauna baths

32. Erectile or sexual dysfunction treatment with drugs or pharmaceuticals
33. Experimental or investigational services, supplies, or drugs (see page 160)
34. Extracorporeal shock wave therapy for musculoskeletal conditions
35. Eye surgery to alter the refractive character of the cornea, such as radial keratotomy, photokeratectomy, or LASIK surgery
36. Facet neurotomy for headache
37. Facet neurotomy for thoracic spine
38. Fecal microbiota transplantation for treatment of inflammatory bowel disease
39. Fees, Taxes, and Interest, except as required by law, the following fees taxes, and interest are not covered:
    - Charges for shipping and handling, postage, interest, or finance charges that the provider might bill;
    - Excise, sales, or other taxes;
    - Surcharges;
    - Tariffs;
    - Duties;
    - Assessments; or
    - Other similar charges whether made by federal, state, or local government or by another entity
40. Functional neuroimaging for primary degenerative dementia or mild cognitive impairment
41. Gene expression profile testing for multiple myeloma or colon cancer
42. Headaches:
    - Treatment of chronic tension-type headache with Botox or acupuncture
    - Treatment of chronic migraine or chronic tension-type headache with massage, trigger point injections, transcranial magnetic stimulation, or manipulation/manual therapy (e.g., chiropractic services).

    **Note:** For chronic migraines and tension-type headaches, see page 48
43. Hearing aid items:
    - Over-the-counter hearing aids that are not prescribed, except for initial assessment, fitting, adjustment, auditory training, and ear molds as necessary to maintain an optimal fit

- Charges incurred after your plan coverage ends, unless you ordered the hearing aid before that date and it is delivered within 45 days after your coverage ended
- Extended warranties, or warranties not related to the initial purchase of the hearing aid(s)
- Purchase of replacement batteries or other ancillary equipment, except those covered under terms of the initial hearing aid purchase

  The types of ancillary equipment not covered are:

- Alerting devices
- Assistive listening devices for FM/DM systems, receivers and transmitters
- Assistive listening devices for microphone transmitters
- Assistive listening devices for TDD machines
- Assistive listening devices for telephones
- Assistive listening devices for televisions (including amplifiers and caption decoders)
- Assistive listening devices for use with cochlear implants
- Assistive listening devices, supplies, and accessories not otherwise specified

44. Hip resurfacing
45. Hip surgery for treatment of Femoroacetabular Impingement (FAI) Syndrome
46. Home health care, except as described on page 49. The plan does not cover the following services:
    - Housekeeping or meal services
    - Care in any nursing home or convalescent facility
    - Care provided by a family member
47. Hospital inpatient charges for non-essential services or features, such as:
    - Admissions solely for diagnostic procedures that could be performed on an outpatient basis
    - Personal or convenience items
    - Reserved beds
    - Services and devices that are not medically necessary (see definition on page 166)
48. Hyaluronic acid treatment for treatment of knee or hip osteoarthritis
49. Hyperbaric oxygen therapy treatment for:
    - Brain injury including traumatic (TBI) and chronic brain injury
    - Cerebral palsy
    - Migraine or cluster headaches
    - Multiple sclerosis
    - Non-healing venous, arterial, and pressure ulcers
50. Imaging of the sinus for rhinosinusitis using x-ray or ultrasound
51. Immunizations, physical exams and associated services (laboratory or similar tests) for the purpose of travel or employment, even if recommended by the CDC
52. Implantable drug delivery systems (IDDS or infusion pumps) for chronic, non-cancer pain

53. Incarceration: Services and supplies provided while confined in a prison or jail
54. Knee arthroplasty: Multi-compartmental arthroplasty and partial knee arthroplasty (including bi-compartmental and bi-unicompartmental)
55. Knee arthroscopy for osteoarthritis of the knee
56. Labial frenotomy for breastfeeding difficulties
57. Late fees, finance charges, or collections charges
58. Learning disabilities treatment after diagnosis, except as covered under the following benefits:
    - "Applied Behavior Analysis (ABA) Therapy" on page 36;
    - "Therapy: Habilitative and Rehabilitative" on page 65;
    - When part of treating a mental health disorder; or
    - When part of treating a substance use disorder.
59. Lumbar artificial disc replacement
60. Lumbar fusion for degenerative disc disease
61. Lumbar radiculopathy/sciatica surgery: Minimally invasive procedures that do not include laminectomy, laminotomy, or foraminotomy, including, but not limited to, energy ablation techniques, Automated Percutaneous Lumbar Discectomy (APLD), percutaneous laser, nucleoplasty, etc.
62. Maintenance care (see definition on page 165)
63. Manipulations of the spine or extremities, except as described under "Spinal and extremity manipulations" on page 63
64. Marriage, family, or other counseling or training services, except as provided to treat an individual member's neuropsychiatric, mental health, or substance use disorder
65. Massage therapy services when the massage therapist is not a preferred provider
66. Medicare-covered services or supplies delivered by a provider who does not offer services through Medicare, when Medicare is the member's primary coverage
67. Microprocessor-controlled lower limb prostheses (MCP) for the feet and ankle
68. Migraine and tension-type headaches:
    - Treatment of chronic tension-type headache with Botox or acupuncture
    - Treatment of chronic migraine or chronic tension-type headache with massage, trigger point injections, transcranial magnetic stimulation, or manipulation/manual therapy (e.g., chiropractic services)

    **Note:** For chronic migraines and tension-type headaches, see page 48
69. Missed appointment charges
70. Negative pressure wound therapy in patients with contraindications referred to by the FDA Safety Communication dated February 24, 2011
71. Noncovered provider types: Services delivered by providers not listed as a covered provider type (see page 16)
72. Novocure (i.e., Optune) (tumor treating fields)
73. Orthoptic therapy except for the diagnosis of strabismus, a muscle disorder of the eye

74. Orthotics, foot or shoe: Items such as shoe inserts and other shoe modifications, except as specified on page 44
75. Out-of-network provider charges that are above the allowed amount
76. Peripheral nerve ablation, using any technique, to treat limb pain for adults and children, including for knee, hip, foot, or shoulder due to osteoarthritis or other conditions
77. Pharmacogenetic testing for patients being treated with oral anticoagulants
78. Pharmacogenomics testing for depression, mood disorders, psychosis, anxiety, attention deficit hyperactivity disorder (ADHD), and substance use disorder
79. Positron Emission Tomography (PET) scans for routine surveillance of lymphoma
80. Prescription drug charges over the allowed amount, regardless of where purchased
81. Prescription drugs that require prior authorization, unless the request is:
    - Approved by the plan
    - Supported by medical justification from a clinician other than the member or the member's family
82. Printing costs for medical records
83. Private duty nursing or continuous care in the member's home, except as described on pages 51 and 62
84. Proton beam therapy for individuals age 21 or older for conditions other than:
    - Brain/spinal
    - Esophageal
    - Head/neck
    - Hepatocellular carcinoma
    - Ocular
    - Skull-based
    - Other primary cancers where all other treatment options are contraindicated after review by a multidisciplinary tumor board
85. Provider administrative fees: Any charges for completing forms, copying records, or finance charges, except for records requested by the plan to perform retrospective (i.e., post-payment) review
86. Repetitive transcranial magnetic stimulation for tinnitus
87. Replacement of lost, stolen, or damaged durable medical equipment (DME)
88. Replacement of prescription drugs that once provided to the member are any of the following:
    - Confiscated or seized by Customs or other authorities
    - Unusable
    - Expired
    - Lost or stolen
89. Residential treatment programs offered at facilities that do not meet the definition of Residential Treatment Facility (see definition of "Residential treatment facility")
90. Reversal of voluntary sterilization (vasectomy, tubal ligation, or similar procedures)
91. Riot, rebellion, and illegal acts: Services and supplies for treatment of an illness, injury, or condition caused by a member's voluntary participation in a riot, armed invasion or aggression, insurrection or rebellion, or sustained by a member arising directly from an act deemed illegal by a court of law
92. Routine ultrasounds during pregnancy, except one in week 13 or earlier, one during weeks 13-28, or high-risk pregnancies (see description on page 57)

93. Sacroiliac joint fusion: Minimally invasive and open sacroiliac joint fusion procedures in adults, age 18 or older, with chronic sacroiliac joint pain related to degenerative sacroiliitis and/or sacroiliac joint dysfunction

94. Screening and monitoring tests for osteopenia/osteoporosis:
    - Once treatment for osteoporosis has begun, serial monitoring is not covered
    - Development of a fragility fracture alone is not a covered indication

95. Separate charges for records or reports

96. Service animals: Any expenses related to a service animal

97. Services covered by other insurance, including, but not limited to:
    - Automobile no-fault
    - Commercial premises
    - General no-fault
    - Homeowner's
    - Medical payments (Med-Pay)
    - Motor vehicle
    - Personal injury protection (PIP)
    - Renter's
    - Underinsured or uninsured motorist

    See page 117 for more about how this works.

98. Services delivered by providers or facilities delivering services outside the scope of their licenses

99. Services or supplies:
    - For which no charge is made, or for which a charge would not have been made if you had no health care coverage
    - For which you are not obligated to pay
    - Provided by a resident physician or intern acting in that capacity
    - Provided by someone in the member's family or household
    - That are not medically necessary for the diagnosis and treatment of injury or illness or restoration of physiological functions and are not covered as preventive care. This applies even if services are prescribed, recommended, or approved by your provider.
    - That are solely for comfort

100. Services performed during a noncovered service

101. Services performed primarily to ensure the success of a noncovered service, including, but not limited to, a hiatal hernia repair done to ensure the success of a noncovered laparoscopic adjustable gastric banding surgery

102. Services, supplies, or drugs related to occupational injury or illness (see page 115)

103. Services, supplies, or items that require prior authorization unless the request is:
    - Approved by the plan
    - Supported by medical justification from a clinician other than the member or the family of a member

104. Skilled nursing facility services or confinement:
    - When primary use of the facility is as a place of residence
    - When treatment is primarily custodial

105. Sleep apnea diagnosis and treatment as indicated in referenced Medicare national and local coverage determinations

106. Sleep therapy services performed at the following locations are not covered:
    - Emergency room services
    - Inpatient hospitalization
    - Urgent-care facilities

107. Sound therapies for treatment of tinnitus, including, but not limited to:
    - Masking devices (sound maskers)
    - Altered auditory stimuli
    - Auditory attention training

108. Spinal cord stimulation for treatment of complex regional pain syndrome, or when any of the following are present:
    - Life expectancy less than one (1) year
    - Hemoglobin A1C (HbA1C) >10 (for PDN)
    - Body mass index (BMI) >45
    - Maximum daily morphine milligram equivalent (MME) ≥120
    - Concurrent, untreated, substance use disorder (including alcohol, prescription or illicit drugs) per American Society of Addiction Medicine (ASAM) guidelines
    - Active, substantial chronic pain in other regions that have required treatment in the past year
    - Related or pending worker's compensation claim (for FBSS and NSRBP)
    - Pending or existing litigation for the condition being treated with SCS

109. Spinal injections, therapeutic (except as described under "Spinal injections" on page 63) of the following types:
    - Facet injections
    - Intradiscal injections
    - Medial branch nerve block injections

110. Spinal surgical procedures known as vertebroplasty, kyphoplasty, and sacroplasty

111. Stem cell therapies for musculoskeletal conditions

112. Stereotactic body radiation therapy for the treatment of primary tumor of the following cancer types:
    - Bone
    - Head and neck
    - Adrenal
    - Melanoma
    - Merkel cell
    - Breast
    - Ovarian

- Cervical

113. Stereotactic radiation surgery for the treatment of cancerous tumors, except when treatment is for central nervous system primary and metastatic tumors

114. Subscription, membership, and access-related fees. Fees for accessing care, treatment, or advice are not covered, whether the access is for virtual or in-person care. Excluded fees include but are not limited to:
    - Concierge fees;
    - Subscription fees
    - Membership fees
    - Retainer fees
    - VIP or priority access fees; and
    - Any other access-related fees

115. Surrogacy

116. Telephone or virtual consultations or appointments, except as described under "Telemedicine services" on page 68

117. The following treatments for chondral defects are not covered for:
    - Any Joint:
        - Autologous matrix-induced chondrogenesis (AMIC)
        - Cell-free implants
        - Uncorrected malalignment or ligamentous deficiency (unless a corrective procedure is performed prior to or simultaneously)
    - Any joint except the knee:
        - Matrix-induced autologous chondrocyte implantation (MACI)
        - Osteochondral allograft transplantation (OCA)
        - Osteochondral autologous transplantation (OATS)

118. Tinnitus specific therapies including, but not limited to:
    - Tinnitus retraining therapy (TRT)
    - Neuromonics tinnitus treatment (NTT)
    - Tinnitus activities treatment (TAT)
    - Tinnitus-masking counseling

119. Transcranial Magnetic Stimulation (TMS) for treatment of:
    - Generalized anxiety disorder (GAD)
    - Obsessive-compulsive disorder (OCD)
    - Posttraumatic stress disorder (PTSD)
    - Smoking cessation
    - Substance use disorder (SUD)

120. Transcutaneous vagal nerve stimulation (does not include or apply to support of previous implanted VNS)

121. Transcutaneous vagal nerve stimulation for epilepsy or depression

122. Travel, transportation, and lodging expenses, except as specified for ambulance services covered by the plan (see page 35)

123. Treatment of varicose veins with Endovenous Laser Ablation (EVLA), Radiofrequency Ablation (RFA), Sclerotherapy, and Phlebectomy in patients with pregnancy, active infection, peripheral arterial disease, or deep vein thrombosis (DVT)

124. Upright magnetic resonance imaging (uMRI), also known as "positional," "weight-bearing" (partial or full), or "axial loading"

125. Vagal nerve stimulation (VNS) for treatment-resistant depression

126. Vagal nerve stimulation (VNS) for the treatment of depression (does not include or apply to support of previously implanted VNS)

127. Vision services and supplies except as described under the "Vision Care (diseases and disorders of the eye)" benefit on page 70

128. Vitamin D screening and testing as part of routine screening

129. Weight control, weight loss, and obesity treatment:

    - Non-surgical: Any program, drugs, services, or supplies for weight control, weight loss, or obesity treatment. Exercise or diet programs (formal or informal), exercise equipment, or travel expenses relating to non-surgical or surgical services are not covered. Such treatment is not covered even if prescribed by a provider, except as covered under "Bariatric surgery" (see page 37), "Omada for Diabetes" (see page 57), "Omada for Prevention and Weight Health" (see page 57), "Nutrition counseling and therapy" (see page 54), or "Preventive care" (see page 58).
    - Surgical: Any bariatric surgery procedure, any other surgery for obesity or morbid obesity, and any related medical services, drugs, or supplies, except when approved by prior authorization review.

130. Whole exome sequencing for:

    - Uncomplicated autism spectrum disorder, developmental delay, mild to moderate global developmental delay.
    - Other circumstances (e.g. environmental exposures, injury, infection) that reasonably explain the constellation of symptoms.
    - Carrier testing for "at risk" relatives.
    - Prenatal or pre-implantation testing.

131. Whole Genome Sequencing for:

    - Carrier testing for "at risk" relatives.
    - Prenatal or pre-implantation testing

132. Workers' compensation: When a claim for workers' compensation is accepted, all services related to that injury or illness are not covered, even if some services are denied by workers' compensation

If you have questions about whether a certain service or supply is covered, contact UMP Customer Service.

# If you have other HDHP coverage

## Coordination of benefits

Coordination of benefits (COB) happens when you have health coverage through two or more plans (such as your employer and your spouse's employer), and these two health plans both pay a portion of your health care claims.

The rules beginning under the "Who pays first" section determine which plan pays first (primary payer) and which pays second (secondary payer). See page 104 for a description of how the plan coordinates benefits when it pays second.

The plan processes claims differently depending on if it is the primary payer or the secondary payer. The differences are described in the next several pages.

> **TIP:** If you have other health coverage, it is important that you let your providers know, including the pharmacies where you get your prescription drugs.

## Contact UMP and ArrayRx

If you or your dependents have other insurance, you must let UMP and ArrayRx know so claims are processed correctly. To do this, you must complete and submit a separate form for medical services and prescription drugs. Each person claiming payment for benefits under the plan is required to give Regence and ArrayRx any facts needed to apply these COB rules.

If your coverage under other plans changes, you must let UMP and ArrayRx know so claims are processed correctly. See the Directory pages at the beginning of this booklet for contact information and ways to get and submit COB forms.

## Who pays first

**Note:** If you cannot determine which plan pays first, contact UMP Customer Service.

> **Alert!** Medicare will be the primary payer for covered benefits provided to Medicare-eligible members who are enrolled in UMP as a dependent of a currently employed state-registered domestic partner. Medicare will remain the secondary payer for benefits payable under UMP for (1) individuals age 65 or over who have UMP as a result of their own current employee status, or (2) individuals age 65 or over who have UMP as a result of the current employment status of a spouse of any age.

When UMP coordinates benefits with other plans, the following rules determine which plan pays first. These rules apply in order, so the first rule below that applies to your situation will determine which plan is your primary coverage, and subsequent rules will not apply.

The following plan pays first:

- Any group plan that does not coordinate benefits.
- The plan that covers the member as a subscriber, not a dependent.

**Exception:** When the subscriber is a Medicare beneficiary, the plan covering the person as the retiree is secondary to the plan covering the person as a dependent.

- The plan that covers the member (or their spouse or state-registered domestic partner) as an active employee pays before a plan that covers the member as a retired employee.
- A plan covering the member as an employee, subscriber, retiree, or the dependent of such an employee, subscriber, or retiree will pay before a COBRA or a state right of continuation plan.
- If the other plan is Medicare, UMP pays first if the member is age 65 or over who have UMP as a result of their own active employee status or the active employee status of a spouse of any age.

  **Note:** Medicare benefits are primary to UMP for members who are enrolled in UMP as a dependent of a currently employed state-registered domestic partner.
- The plan that has covered the member (or their spouse or state-registered domestic partner) as a subscriber the longest, if there are two plans and the first five bullets do not determine which plan pays first.
- If none of the preceding rules determines the order of benefits, the allowable expenses must be shared equally between the plans.

# For dependent children

A group plan is usually primary over Medicaid programs that cover children. If a dependent child has group coverage through their employer, the child's coverage pays first.

## Dependent children of married parents

The group plan of the parent whose birthday is earlier in the year pays first. For example, the plan of a parent born April 14 is primary over the plan of a parent born August 21. This is called the "birthday rule." This rule looks only at the month and day, not the year. If both parents have the same birthday, the plan that has covered either parent the longest is primary.

**Exception for newborn children**

Under Washington State law, a newborn child must be enrolled as a dependent under the mother's plan from birth through at least the first 21 days of life. Therefore, the mother's plan is primary for covered charges during the first 21 days of a newborn's life. Even if the newborn is later enrolled in different coverage, the newborn will remain enrolled under the mother's plan for the first 21 days.

## Dependent children of legally separated or divorced parents

When no court order specifies which parent is responsible for providing health insurance coverage, the following standard COB rules determine which plan pays first:

1. The plan of the custodial parent.
2. The plan of the custodial parent's spouse, if the custodial parent has remarried.
3. The plan of the non-custodial parent.
4. The plan of the non-custodial parent's spouse, if the non-custodial parent has remarried.

The custodial parent is the parent awarded custody by a court decree or, in the absence of a court decree, is the parent with whom the child resides more than one half of the calendar year, excluding any temporary visitation.

The birthday rule is used to determine which parent's plan pays first if:

- The court order states that both parents are responsible for the child's/children's health coverage and expenses.

- The court order awards joint custody without specifying that one parent is responsible for the child's/children's health coverage and expenses.

If the court order states one parent is to assume primary financial responsibility for the child but does not specify that one parent is responsible for health coverage and health care expenses, the plan of the parent assuming financial responsibility is the primary payer.

In some cases, a court order determines payment for health care expenses. In those cases, standard COB rules may not apply. You must promptly provide the plan with copies of the court order for the plan to determine which plan pays first.

If a dependent child is covered under more than one plan through persons who are not the child's parent or stepparent (e.g., a grandparent or other guardian), the plan will use the birthday rule to determine which plan pays first.

If none of the preceding rules determine who pays first, then each plan covers half of the allowed expenses.

## Federal and military plans

UMP usually pays first over certain federal or military programs for veterans (retired military members).

## When UMP pays first

When the plan is the primary payer (pays first), UMP pays its normal benefit as described in this COC. You may need to send UMP's Explanation of Benefits and a copy of your provider's bill to your secondary payer to receive payment. Check with that plan for more information.

### When UMP is supposed to pay first, but another plan did instead

If another plan pays first on claims where UMP should have paid first:

- UMP may pay the other plan the amount UMP should have paid.
- Amounts UMP pays to the other plan are considered benefits paid by UMP.

## How UMP coordinates benefits when it pays second

UMP uses a type of COB called nonduplication of benefits (see definition of "Nonduplication of benefits"). When UMP pays second to another plan that covers the member, UMP will pay only an amount needed to bring the total benefit up to the amount UMP would have paid if the member did not have another plan.

The intent of this type of COB is to maintain the level of benefits available through UMP. The nonduplication of benefits type of coordination is not designed to pay your covered expenses in full.

When UMP pays second, it coordinates with these types of plans:

- Governmental programs including, but not limited to, Medicare and Medicaid.
- Group, blanket, or franchise health or disability insurance policies; health care service contractor and health maintenance organization group agreements issued by insurers; health care service contractors; and health maintenance organizations.
- Labor management trusteed plans, labor organization plans, employer plans, or employee benefit organization plans.

**ALERT!** If you have other primary coverage that pays for services, those services will apply to the UMP benefit limit.

## How much you may pay when UMP pays second

When you see preferred providers under UMP, you will owe only the balance of the UMP allowed amount after your primary plan and UMP pay benefits for covered services. Your cost will usually be higher if you see out-of-network providers. See the "Sample payments to different provider network status" section for examples.

The examples in the table below assume that you have met your deductible.

| Situation | Example | Preferred provider charge | UMP allowed amount | UMP normal benefit | Other plan pays | UMP pays | You pay your provider |
|---|---|---|---|---|---|---|---|
| **UMP is primary, other plan is secondary** | **EXAMPLE 1:** When UMP pays first (or is the only plan) | $200 | $100 | $85 (85% of $100) | N/A | $85 | $15 |
| **UMP is secondary, other plan is primary** | **EXAMPLE 2:** The other plan pays less than the normal UMP benefit | $200 | $100 | $85 | $80 | $5 | $15 |
| **UMP is secondary, other plan is primary** | **EXAMPLE 3:** The other plan pays as much as (or more than) the normal UMP benefit | $200 | $100 | $85 | $85 | $0 | $15 |

Contact UMP Customer Service for help with any questions if you are covered by more than one plan.

## Submit secondary claims promptly

All health plans have a "timely submitting" deadline. The timely submitting deadline for UMP is 12 months from the date of service. If a claim is not submitted within the plan's timely submitting deadline, UMP will deny it. If your primary plan delays payment on a claim, you must still submit to UMP within the submitting deadline to prevent denial of the claim.

UMP may try to contact your primary plan for their benefit payment information or may estimate it to provide timely processing of your secondary benefit. Adjustments may be made when the primary plan pays their portion of your claim. Notifying your providers of any change to your coverage will help avoid errors and delays in processing of claims (see the "Billing and payment: submitting a claim" section).

## How diabetes care supplies are covered when UMP pays second

UMP covers diabetes care supplies under the prescription drug benefit. If you get your supplies from a pharmacy, ask if the pharmacy can bill both your primary plan and UMP. If your pharmacy does, you do not need to do anything further. If not, you will need to send a claim to ArrayRx for secondary payment (see the "Submitting a claim for prescription drugs" section for instructions).

If you get your supplies from a diabetes care supplier, the primary plan may process the claim as medical. In this case, you need to send your Explanation of Benefits and a claim form to ArrayRx for secondary

payment (see the "Submitting a claim for prescription drugs" and the "False claims or statements" sections for instructions).

**Note:** Nonduplication of benefits applies to these claims, which means that UMP may pay nothing after your primary plan pays.

See the "Diabetes care supplies" benefit for more information.

> **ALERT!** You must submit secondary claims for diabetes care supplies to ArrayRx. Regence BlueShield will deny these claims.

### How COB works with prescription drugs

Some of the limits and restrictions to prescription drug coverage listed in the "Limits on your prescription drug coverage" section will apply when UMP pays second to another plan. See the "Submitting a claim for prescription drugs" section for how to submit your prescription drug claim.

Nonduplication of benefits applies when UMP pays second to another plan. This means that UMP may pay nothing after your primary plan pays.

**Using network pharmacies when UMP is secondary**

If you have primary coverage through another plan that covers prescription drugs, show both plan member ID cards to the pharmacy and make sure they know which plan pays first and which plan pays second. It is important that the pharmacy bills the plans in the correct order, or claims may be denied or paid incorrectly.

**Using mail-order pharmacies when UMP is secondary**

If your primary plan also uses one of UMP's network mail-order pharmacies as the plan's network mail-order pharmacy, UMP's network mail-order pharmacies may process payments for both plans and charge you only the remainder. Make sure that UMP's network mail-order pharmacies have the information for both plans and know which plan is primary.

However, if your primary plan uses a different mail-order pharmacy, you will have to use your primary plan's mail-order pharmacy, then submit a claim to UMP. See the "Submitting a claim for prescription drugs" section for how to do this.

# Billing and payment: submitting a claim

## Submitting a claim for medical services

When UMP is your primary insurance and your provider is preferred, or participating, you do not need to submit claims. The provider will do it for you. If you have a question about whether your provider's office has submitted a claim, sign in to your Regence account or contact UMP Customer Service (see Directory for link and contact information). See the "Blue Cross Blue Shield Global® Core" section for instructions on submitting a claim for services received outside of the United States.

> **TIP:** In the following section, UMP refers to the administrative functions for submitting claims to UMP. Regence BlueShield handles medical claims, and ArrayRx handles prescription drugs claims.

## When you need to submit a claim

You may need to submit a claim to UMP for payment if:

- You receive services from an out-of-network provider.
- You have other insurance that pays first and UMP is secondary.

Out-of-network providers may submit a claim on your behalf. Ask your provider.

## How to submit a claim

To submit a claim yourself, you may sign in to your Regence account and go to the Submit claim webpage at **regence.com/member/submit-claim/** or you may complete a medical claim form and mail the following documents:

- *UMP (Regence) Medical Claim Form* — You may find the form by visiting forms and publications at **hca.wa.gov/ump-forms-pubs** or you may request a form by contacting UMP Customer Service.
- An itemized bill from your provider that describes the services you received and the charges.

The following information must appear on the provider's itemized bill for the plan to consider the claim for payment:

- Member's name and member ID number, including the alpha prefix (three letters and the 'W' before member ID number)
- Procedure and diagnosis code(s) or description of the injury or illness
- Date and type of service
- Provider's name, address, phone number, and National Provider Identifier (NPI) or Tax ID number
- For ambulance claims, also include the ZIP code of where the member was picked up and where they were taken

If UMP is secondary, you must include a copy of your primary plan's Explanation of Benefits, which lists the services covered and how much the other plan paid. You should wait until the primary plan has paid to submit a secondary claim to UMP, unless the primary plan's processing of the claim is delayed. Claims not submitted to UMP within 12 months of the date of service will not be paid.

If we must request additional information, the processing of your claim may be delayed.

**Note:** Be sure to make copies of your documents for your records.

Mail both the claim form and the provider's claim document (or bill) to:

Regence BlueShield
Attn: UMP Claims
PO Box 1106
Lewiston, ID 83501-1106

Or you can fax documents to Regence at 1-877-357-3418.

The plan may send reimbursement for services received from an out-of-network provider to the provider or to you in the form of a check listing both you and the provider as payees. If you paid up front for services, proof of payment may be required. Contact UMP Customer Service if you have a question about the processing of your claim or for information on what is acceptable as proof of payment.

# Important information about submitting claims

**ALERT!** Out-of-network claim submissions must be submitted within 12 months of the date you received health care services. This is called the "timely submitting" deadline. The plan will not pay claims submitted more than 12 months after the date of service. See "Submit secondary claims promptly" on page 105 for how this works when you have other coverage that pays first.

For information about submitting claims for services outside of the U.S., contact UMP Customer Service. You may have to pay for services upfront and submit a claim for reimbursement.

If you have other health care coverage, see the "If you have other HDHP coverage" section for information on how the plan coordinates benefits with other plans.

Services apply to your UMP deductible in the order claims are received and processed, not necessarily in the order the member receives services.

## Claims reimbursement

Most of the time, the plan will pay preferred or participating providers directly. For claims submitted by you or an out-of-network provider, the plan will determine whether to pay you, the provider, or both. For a child covered by a legal qualified medical child support order (see page 103) the plan may pay the child's custodial parent or legal guardian.

## Claims determinations

The plan will notify you of action taken on a claim within 30 days of the plan receiving it. This 30-day period may be extended by 15 days when action cannot be taken on the claim due to:

- Circumstances beyond the plan's control. Notice will include an explanation why an extension is needed and when the plan expects to act on the claim.
- Lack of information. The plan will notify you within the 30-day period that an extension is necessary, with a description of the information needed and why it is needed.

# Submitting a claim for prescription drugs

You may need to submit your own prescription drug claim to ArrayRx for reimbursement if you:

- Buy prescription drugs at a non-network retail pharmacy.
- Fail to show your UMP member ID card at a network pharmacy.
- Have other prescription coverage that pays first, and UMP is secondary.

**TIP:** If you get a vaccine from an out-of-network provider, make sure that you submit your claim to Regence BlueShield as a medical claim (see the "Submitting a claim for medical services" section).

Prescription drug claim forms are available by visiting forms and publications at **hca.wa.gov/ump-forms-pubs** or by contacting ArrayRx Customer Service. Send the completed claim form, along with your pharmacy receipt(s), to:

Pharmacy Manual Claims
PO Box 999
Appleton, WI 54912-0999

Or you can fax documents to ArrayRx at 1-855-668-8550.

It is a good idea to keep copies of all your paperwork for your records.

When you submit a prescription drug claim to ArrayRx, the plan pays the claim based on the following rules, no matter where you purchased the drug:

- Claims for prescription drugs purchased at a network pharmacy will be reimbursed based on the allowed amount. If the pharmacy charged you more than the allowed amount, you will pay your usual coinsurance (and deductible if applicable), plus the difference between what the plan paid and the pharmacy's charge.
- Claims for prescription drugs purchased at non-network pharmacies will be reimbursed at the out-of-network rate (see definition of "Out-of-network rate, prescription drugs"). If the pharmacy charged you more than the out-of-network rate, you will pay your usual coinsurance (and deductible if applicable), plus the difference between what the plan paid and the pharmacy's charge.
- The plan pays all prescription drug claims, including non-network retail pharmacies, based on coinsurance.
- If your claim exceeds the quantity limit or the maximum days' supply allowed by the plan, the plan will pay only for the amount of the prescription drug up to the quantity limit or maximum days' supply.
- If you receive a refill before 84 percent of the last supply you received should have been taken, the plan will not pay for it (see the "Refill too soon" section).

You must submit prescription drug claims within 12 months of purchase. The plan will not pay claims for prescription drugs submitted more than 12 months after purchase or prescription drugs purchased from an excluded pharmacy. See the definition of "Excluded pharmacy."

Do not use a discount card to fill your prescription(s) or you will not be able to be reimbursed. Additionally, discount cards cannot be used together with your UMP benefits for covered prescription drugs. Claims submitted for prescription drugs purchased with a discount card will not be covered.

**ALERT!** If you do not show your UMP member ID card when purchasing a prescription at a network pharmacy, you will have to pay the full cash price and submit a Prescription Drug Claim Form. You will not receive the plan discount. Prescription drug discount cards are not considered insurance and cannot be used in combination with your plan.

## False claims or statements

Neither you nor your provider (or any person acting for you or your provider) may submit a claim for services or supplies that were not received, were resold to another party, or for which you are not expected to pay.

In addition, neither you nor any person acting for you may make any false or incomplete statements or any false claims on any document for your plan coverage.

The plan may recover any payments or overpayments made because of a false claim or false statement by withholding future claim payments, by suing you, or by other means. False claims may also be crimes.

If you represent yourself as being enrolled in this plan when you are not, the plan will deny all claims.

If the plan asks you for more information, you will be allowed at least 45 days to provide it. If the plan does not receive the information requested within the time allowed, the plan will deny the claim.

# Complaint and appeal procedures

**ALERT!** In the following section, UMP refers to the administrative functions for appeals for UMP CDHP. Regence BlueShield handles medical appeals and ArrayRx handles appeals involving prescription drugs.

Appeals procedures may change during the year if required by federal or Washington State law.

## What is a complaint (aka: grievance)?

A complaint is an oral statement or written document submitted by or on behalf of a member regarding:

- Dissatisfaction with medical care.
- Dissatisfaction with service provided by the health plan.
- Provider or staff attitude or demeanor.
- Waiting time for medical services.

**Note:** If your issue is regarding a denial, reduction, or termination of payment or nonprovision of medical services, it is an appeal.

## How to submit a complaint (aka: grievance)

For all medical complaints, it's recommended that you first contact UMP Customer Service. For prescription drug complaints or grievances, we recommend contacting ArrayRx Customer Service. Many issues may be resolved with a phone call. If an initial phone call does not resolve your complaint, you may submit your complaint:

- Over the phone: If you want a written response, you must request one.
- By mail, fax, or email (see the "Where to send complaints or appeals" section below).

You will receive notice of the action on your complaint or grievance within 30 calendar days of our receiving it. The plan will notify you if it needs more time to respond.

## What is an appeal?

An appeal is an oral or written request submitted by you or your authorized representative to Regence BlueShield or ArrayRx to reconsider:

- A decision to deny, modify, reduce, or terminate payment, coverage, certification, or provision of health care services or benefits, including the admission to, or continued stay in, a health care facility.
- A prior authorization.
- A retroactive decision to deny coverage based on eligibility (see the "Appeals related to eligibility" section below).
- Claims payment, processing, or reimbursement for health care services or supplies.

# The appeals process

**ALERT!** If your appeal is for an urgent or life-threatening condition, see the "Expedited appeals process" section below.

You or someone you authorize to represent you (see "How to designate an authorized representative" on page 121) may submit an appeal. There are three levels to the appeals process:

1. First-level appeal
2. Second-level appeal
3. External review (independent review)

Each of those parts are described in further detail below.

## Coverage during each review

If your request involves a decision to change, reduce, or terminate coverage for services, supplies, or prescription drugs already being covered, the plan must continue to cover the disputed service until the outcome of the review. If the plan upholds the decision to change, reduce, or terminate coverage, you will be responsible for the cost of the services received during the review period. If you request payment for denied claims or approval of services, supplies, or prescription drugs not yet covered by the plan, the plan will not cover the services, supplies, or prescription drugs while the appeal is under consideration.

## First-level and second-level appeal reviewers

Claim processing disputes will be reviewed by administrative staff. The plan will consult with a health care professional employed by Regence BlueShield on medical appeals, or with a clinical pharmacist from ArrayRx on prescription drug appeals, when appeals involve issues requiring medical judgment about covering, authorizing, or providing health care. That includes decisions based on determinations that a treatment, prescription drug, or other item is experimental, investigational, or not medically necessary. Your appeal will be reviewed by Regence BlueShield or ArrayRx employees who have not been involved in, or subordinate to anyone involved in, reviewing the previous decisions.

## How to submit an appeal

You or your authorized representative (including a relative, friend, advocate, attorney, or provider) may submit an appeal by using the methods described below in the "Where to send complaints or appeals" section. You may authorize a representative to submit an appeal on your behalf in writing or by contacting UMP Customer Service (medical appeals) or ArrayRx Customer Service (prescription drug appeals).

For each appeal request, you must appeal within 180 days of receiving the plan's decision. You may include written comments, documents, and any other information, such as medical records and letters from your provider, to support your appeal request. The plan will consider all information submitted when reviewing your appeal. You may also request copies of documents the plan has that are relevant to your appeal, which the plan will provide at no cost to you.

The plan will mail you a written response within 14 days of receiving your appeal request. If more time is needed to thoroughly research and review your appeal, the plan is allowed up to 30 days to respond. The plan will ask your permission if it needs more time to respond. You can access the UMP (Regence) Medical appeals and grievance form by visiting forms and publications at **hca.wa.gov/ump-forms-pubs**.

## Information to provide with an appeal

You can submit information, documents, written comments, records, evidence, and testimony, including second opinions, with your appeal. When you provide all the necessary documentation, it allows the plan to review your appeal faster. Include the following when requesting an appeal:

- The member's full name (the name of the employee, retiree, or dependent covered by the plan)
- The member ID number (starting with a "W" on your UMP member ID card)
- The name(s) of any providers involved in the issue you are appealing
- Date(s) of service or incident
- Your mailing address
- Your daytime phone number(s)
- A statement describing the issue and your desired outcome
- A copy of the Explanation of Benefits, if applicable, or a list of the claim numbers you are appealing
- Medical records from your provider, if applicable. Your provider should supply clinically relevant information, such as medical records for services denied based on medical necessity or for other clinical reasons. The plan must receive all relevant information with the appeal to make sure the most accurate decision is made.

## First-level appeals

You or your authorized representative may submit a first-level appeal no more than 180 days after you receive the plan's decision. If you do not submit an appeal within this time, your appeal will not be reviewed, and you will not be able to continue further appeals (second-level and external review). You may authorize a representative to submit an appeal on your behalf in writing or by contacting UMP Customer Service (medical appeals) or ArrayRx Customer Service (prescription drug appeals).

Regence BlueShield manages first-level appeals for medical services, and ArrayRx manages first-level appeals for prescription drugs. Employees from Regence BlueShield and ArrayRx reviewing the appeals will not have been involved in, or subordinate to anyone involved in the decision you are appealing. Administrative staff review claim processing disputes. A staff of health care professionals at Regence BlueShield or a clinical pharmacist from ArrayRx evaluate appeals that involve issues requiring medical judgment about covering, authorizing, or providing health care.

> **ALERT!** Deadlines for submitting an appeal are based on the first date you are notified of how a claim was processed, usually when you receive an Explanation of Benefits (including services that applied to your deductible or were denied). The plan does not waive deadlines based on untimely billing by your provider.

## Second-level appeals

If you disagree with the decision made on your first-level appeal, you or your authorized representative may submit a second-level appeal. You must submit second-level appeals no more than 180 days after you receive the letter responding to your first-level appeal. If you do not submit an appeal within this time, your appeal will not be reviewed, and you will not be able to continue further appeals (external review). You may authorize a representative to submit an appeal on your behalf in writing or by contacting UMP Customer Service (medical appeals) or ArrayRx Customer Service (prescription drug appeals).

Regence BlueShield manages second-level appeals for medical services, and ArrayRx manages second-level appeals for prescription drugs. Employees from Regence BlueShield and ArrayRx reviewing the appeals will not have been involved in, or subordinate to anyone involved in, reviewing the first-level appeal or initial decision. If new or additional evidence or rationale is considered in reviewing your appeal, the plan will provide you with this information free of charge, and you may respond before the final decision.

# Expedited appeals process

## Expedited appeals for medical services

You or your authorized representative may submit an expedited appeal within 180 days of receiving the previous decision if:

- You are currently receiving or prescribed treatment or benefits that would end because of the denial; or
- Your provider determines that taking the usual time allowed could seriously affect your life, health, or ability to regain maximum function, or would subject you to severe pain that cannot be adequately managed without the disputed care or treatment; or
- The issue is related to admission, availability of care, continued stay, or emergency health care services and you have not been discharged from the emergency room or transport service.

You may authorize a representative to submit an expedited appeal on your behalf in writing or by contacting UMP Customer Service.

An expedited appeal replaces both the first- and second-level appeals. Regence BlueShield will call you, or your authorized representative, with a decision on your expedited appeal within 72 hours of the request. Regence BlueShield will also mail a written response within 72 hours of the decision.

Your provider must submit all clinically relevant information to the plan by phone or fax at:

- **Phone:** 1-888-849-3682 (TRS: 711)
- **Fax:** 1-877-663-7526

If you disagree with the expedited appeal decision, your provider may request an expedited external review (see the "External review (independent review)" section below).

## Expedited appeals for prescription drugs

You or your authorized representative may submit an expedited appeal within 180 days of receiving the previous decision if you or your provider thinks you need a prescription drug immediately. You may authorize a representative to submit an appeal on your behalf in writing or by contacting ArrayRx Customer Service.

An expedited appeal replaces both the first- and second-level appeals. ArrayRx will call you, or your authorized representative, with a decision on your expedited appeal within 72 hours of the request. ArrayRx will also mail a written response within 72 hours of the decision.

You or your provider must submit all clinically relevant information to the plan by phone or fax at:

- **Phone:** 1-888-361-1611 (TRS: 711)
- **Fax:** 1-866-923-0412

During an expedited appeal, you may choose to purchase a three-day supply at your own expense. If ArrayRx decides to cover the prescription drug, ArrayRx will reimburse you up to the allowed amount minus the member cost-share (coinsurance and deductible, if applicable). If ArrayRx decides not to cover the prescription drug (denies the appeal), you are responsible for the full cost of the drug.

If you disagree with the expedited appeal decision, you or your provider may request an expedited external review (see the "External review (independent review)" section below).

# Time limits for the plan to make appeal decisions

**ALERT!** The plan will comply with shorter time limits than those below when required by federal or Washington State law.

The time limits for both first- and second-level appeals are calculated from when the plan receives the appeal. The plan will decide your appeal within 14 days of receiving it but may take up to 30 days unless a different time limit applies as explained below. The plan will request written permission from you or your authorized representative if an extension to the 30-day time limit is needed to get medical records or a second opinion.

For expedited appeals, the plan will make a determination as soon as possible but always within 72 hours. The plan will notify you (or your authorized representative) of the decision verbally within 72 hours and will mail a written notice within 72 hours of the decision.

# External review (independent review)

You or your authorized representative may submit a request for an external review by an independent review organization (IRO) if you have gone through both a first- and second-level appeal (or expedited appeal) and your appeal was based on the plan's decision to deny, modify, reduce, or terminate coverage of or payment for a health care service.

You may also submit a request for an external review:

- If the plan has exceeded the timelines for response to your appeal without good cause and without reaching a decision; or
- If the plan has failed to adhere to the requirements of the appeals process.

You may submit a request for an expedited external review if you meet the requirements for the expedited process as described above. You may also request an expedited external review at the same time that you request an expedited appeal, called a concurrent expedited review. When you request a concurrent expedited review, you are not required to go through both a first- and second-level appeal.

An IRO will conduct the external review. An IRO is a group of medical and benefit experts certified by the Washington State Office of the Insurance Commissioner and not related to the plan, Regence BlueShield, ArrayRx, or HCA. An external review provides unbiased, independent clinical and benefit expertise to determine whether the plan's decision is consistent with state law and the *2027 UMP CDHP (PEBB) Certificate of Coverage*.

## Requesting an external review

To request an external review, see the contact information listed in the "Where to send complaints or appeals" section below.

You or your authorized representative must submit a request for an external review no more than 180 days after you receive the letter responding to your second-level appeal (or expedited appeal). Only the member or an authorized representative may submit a request for an external review. You may authorize a representative to submit a request for an external review on your behalf in writing or by contacting UMP Customer Service (medical appeals) or ArrayRx Customer Service (prescription drug appeals).

The plan — Regence BlueShield for medical services, and ArrayRx for prescription drugs — will send the IRO all the relevant information and correspondence they considered in making the decision. You may send more information directly to the IRO. The IRO will notify you of their decision.

### Additional legal options

You are required to exhaust the plan's appeals process before you may bring a cause of action in court against the plan or HCA. If an IRO reviews your appeal, their decision is binding on both the plan and you except to the extent that other remedies are available under state or federal law.

If the IRO overturns the plan's decision the plan will provide benefits (including making payment on the claim) according to the IRO's decision without delay, regardless of whether the plan intends to seek judicial review of the IRO's decision and unless and until there is a judicial decision otherwise.

## Complaints about quality of care

For complaints or concerns about the quality of care you received from preferred and participating providers only, contact UMP Customer Service or send a secure email through your Regence account (see Directory for link and contact information).

For complaints or concerns about the quality of care you received from any provider:

- Call the DOH at 1-360-236-4700 (TRS: 711) or 1-800-562-6900 (TRS: 711).
- Email the DOH at **HSQAComplaintIntake@doh.wa.gov**.
- Visit the DOH website at **doh.wa.gov/about-us/file-complaint**.

## Appeals related to eligibility

Appeals related to eligibility and enrollment are handled by the PEBB Program and governed by chapter 182-16 WAC.

Information on how to file an appeal is available:

- On the HCA website at **hca.wa.gov/pebb-appeals**.
- By contacting the PEBB Appeals Unit at 1-800-351-6827 (TRS: 711).

## Where to send complaints or appeals

It is recommended that you call first with a complaint or appeal, since many problems may be resolved quickly over the phone. The Directory at the beginning of this booklet includes links and contact information to contact UMP (medical services) or ArrayRx (prescription drugs) with a complaint or appeal.

# When another party is responsible for injury or illness

You may receive a letter from the plan asking if your injury or illness was the result of an accident or might be someone else's responsibility. To make sure claims are paid in a timely manner, it is important that you respond as directed in the letter, even if the answer is no. If you do not, the plan may deny coverage. You may contact UMP Customer Service if you have questions.

## Occupational injury or illness (workers' compensation) claims

When a claim for occupational injury or illness (workers' compensation) is accepted by your employer's workers' compensation carrier, UMP will not cover any services related to that injury or illness, even if the

compensation carrier denies some services. You must file a workers' compensation claim with your workers' compensation carrier. If your claim for workers' compensation is denied because it is determined the injury or condition is not related to an occupational injury or illness, UMP will pay for covered services under the terms of this COC.

## Legal rights and responsibilities

Coverage under the plan is not provided for medical, dental, prescription, or vision expenses you incur for treatment of an injury or illness if the costs associated with the injury or illness may be covered by another first party insurance or may be recoverable from any of the following:

- A third party;
- Any other source, including no fault automobile medical payments (Med-Pay), no fault automobile personal injury protection (PIP), homeowner's no-fault coverage, commercial premises no-fault medical coverage, and sports policies. This includes excess, underinsured, or uninsured motorist coverage, or similar contract or insurance, when the contract or insurance is either issued to or makes benefits available to you, whether or not you make a claim under such coverage; or
- Services or supplies for work-related injury or illness, even when the service or supply is not a covered workers' compensation benefit under the workers' compensation plan.

> **ALERT!** You must respond to any communication sent to you about other sources of benefits, or the plan may deny claims.

However, after expiration or exhaustion of the above benefits, if you also have a potential right of recovery for illnesses or injuries from a third party who may have legal responsibility or from any other source, benefits may be provided or advanced by the plan pending the resolution of a claim to the right of recovery subject to all the following conditions:

- By accepting or claiming benefits, you agree that the plan is entitled to reimbursement of the full amount of benefits paid out of any settlement or recovery from any source to the extent that the settlement or recovery exceeds full compensation to you for the injury or illness that you sustained. This includes any arbitration award, judgment, settlement, disputed claim settlement, underinsured or uninsured motorist payment, or any other recovery related to the injury or illness for which benefits under the plan have been provided or advanced.
- The plan may choose to recover expenses directly from the third party (or third party's insurer) responsible for your injury or illness. This is called subrogation. The plan is authorized, but not obligated, to recover any expenses, to the extent that they were paid under the plan, directly from any party liable to you, upon mailing of a written notice to the potential payer, to you, or to your representative.
- The plan's rights apply without regard to the source of payment for medical expenses, whether from the proceeds of any settlement, arbitration award, or judgment, or other characterization of the recovery by you or any third party or the recovery source. The plan is entitled to reimbursement from the first dollar received from any recovery to the extent that the settlement or recovery exceeds full compensation to you for the injury or illness that you sustained. This applies regardless of whether:
  - The third party or third party's insurer admits liability;
  - The health care expenses are itemized or expressly excluded in the recovery; or

- The recovery includes any amount (in whole or in part) for services, supplies, or accommodations covered under the plan.

- You may be required to sign and deliver all legal papers and take any other actions requested to secure the plan's rights (including an assignment of rights to pursue your claim if you fail to pursue your claim of recovery from the third party or other source). If you are asked to sign a trust/reimbursement agreement or other document to reimburse the plan from the proceeds of any recovery, you will be required to do so as a condition to advancement of any benefits. If you or your agent or attorney fail to comply during the course of the case, we may request refunds from the providers or offset future benefits.
- You will not do anything to prejudice the plan's rights and that you will cooperate fully with the plan, including signing any documents within the required time and providing prompt notice of any settlement or other recovery. You must notify the plan of any facts that may impact the right to reimbursement or subrogation, including, but not necessarily limited to, the following:
  - The filing of a lawsuit;
  - The making of a claim against any third party;
  - Scheduling of settlement negotiations with a minimum of 21 days advance notice of the date, time, location and participants to be involved in any settlement conferences or mediations); or
  - Intent of a third party to make payment of any kind to your benefit or on your behalf and that in any manner relates to the injury or illness that gives rise to the plan's right of reimbursement or subrogation (notice is required a minimum of five business days before the settlement).
- You and your agent or attorney must agree to keep segregated in its own account any recovery or payment of any kind to your benefit that in any manner relates to the injury or illness giving rise to the plan's right of reimbursement or subrogation, until the plan's right is satisfied or released.
- In the event you or your agent or attorney fails to comply with any of these conditions, any such benefits provided or advanced for any illness or injury may be recovered through legal action to the extent that the settlement or recovery exceeds full compensation to you for the injury or illness that you sustained.
- Any benefits provided or advanced under the plan are provided or advanced solely to assist you. By paying such benefits, the plan is not waiving any right to reimbursement or subrogation.

## Fees and expenses

You may incur attorney fees and costs in connection with obtaining a recovery. We may pay a proportional share of such attorney fees and costs you incur at the time of any settlement or recovery to otherwise reduce the amount of reimbursement paid to the plan to less than the full amount of benefits paid by the plan.

## Services covered by other insurance

The plan does not cover services that are covered by other insurance, including, but not limited to, no fault automobile medical payments (Med-Pay), no fault automobile personal injury protection (PIP), homeowner's no-fault coverage, commercial premises no fault medical coverage, or sports policies, including excess, underinsured or uninsured motorist coverage or similar contract or insurance. You are responsible for any cost-sharing required under the other coverage as allowed by state law. Once you have exhausted benefits (e.g., reached the maximum medical expenses amount of the other insurance policy (-ies), or services are no longer injury-related), the plan will cover services according to this COC.

## Motor vehicle coverage

If you are involved in a motor vehicle accident, whether as a driver, passenger, pedestrian, or other capacity, you may have rights under multiple motor vehicle insurance no fault coverages and also against a third party who may be responsible for the accident. In that case, this right of reimbursement and subrogation provision still applies.

## Future medical expenses

Benefits for otherwise covered services may be excluded as follows:

- When you have received a recovery from another source relating to an illness or injury for services for which we normally would provide benefits. The amount of any exclusions under this provision, however, will not exceed the amount of your recovery.
- Until the total amount excluded equals the third-party recovery.

# General provisions

UMP is administered by Regence BlueShield and ArrayRx under contract with HCA.

## What you need to know: your rights and responsibilities

To make sure UMP offers access to the best possible medical care, we must work together with you and your providers as partners. To achieve this goal, you must know your rights and responsibilities.

As a plan member, you have the right to:

- Ask your provider to submit secondary claims to Medicare, if applicable.
- Be informed by your providers about all appropriate or medically necessary treatment options for your condition, regardless of cost or benefit coverage.
- Be treated with respect.
- Complain about or appeal plan services or decisions, or the care you receive.
- Get a second opinion about your provider's care recommendations.
- Have a translator's assistance, if required, when contacting the plan.
- Keep your medical records and personal information confidential as described in the UMP Notice of Privacy Practices, available online on the HCA website at **hca.wa.gov/ump-privacy**.
- Make decisions with your providers about your health care.
- Make recommendations about member rights and responsibilities.
- On request, receive information from the plan about:
  - How new technology is evaluated for inclusion as a covered service.
  - How the plan reimburses providers.
  - Prior authorization review requirements.
  - Providers you select and their qualifications.
  - Services and treatments that have completed HTCC review and how that affects coverage by UMP.
  - Technologies and treatments currently under review by the HTCC.
  - The plan and preferred providers.

- Your covered expenses, exclusions, reductions, and maximums or limits.

- Receive:
  - A written explanation from the plan about any request to refund an overpayment.
  - All covered services and supplies determined to be medically necessary as described in this COC, subject to the maximums, limits, exclusions, deductibles, copays, and/or coinsurance.
  - Courteous, prompt answers from the plan.
  - Timely, proper medical care without discrimination of any kind — regardless of health status or condition, sex, ethnicity, race, marital status, color, national origin, age, disability, or religion.

As a plan member, you have the responsibility to:

- Comply with requests for information by the date given.
- Confirm provider and facility network status before every visit.
- Contact the plan as soon as possible if you do not understand what is covered, if you have any questions, or if you need information.
- Enroll in Medicare Part A and Part B if you are currently retired, or are in the process of retiring, and become eligible for Medicare. You must notify the PEBB Program when you enroll in Medicare Part A and Part B. When you enroll in Medicare you are no longer eligible to contribute to a health savings account (HSA). You need to change your medical plan or you may be liable for any tax penalties resulting from contributions made when you are no longer eligible. Call 1-800-200-1004 (TRS: 711) to speak with a customer service representative for the PEBB Program.
- Follow your providers' instructions about your health care.
- Give your providers complete information about your health to get the best possible care.
- Keep your mailing address current by reporting changes as follows:
  - **Employees:** To your payroll or benefits office.
  - **Retirees, PEBB Continuation Coverage members, and retired employees of a former employer group:** To the PEBB Program. Send your address changes to:

    Health Care Authority
    PEBB Program
    PO Box 42684
    Olympia, WA 98504-2684
- Know how to access emergency care.
- Not engage in fraud or abuse in dealing with the plan or your providers.
- Participate with your providers in making decisions about your health care.
- Pay your copays, coinsurance, and deductibles promptly.
- Refund promptly any overpayment made to you or for you.
- Report to the plan any outside sources of health care coverage or payment.
- Return your completed Multiple Coverage Inquiry form you receive from the plan in a timely manner to prevent delay in claims payment.
- Understand how to contact the plan for more information and help with any covered service or information described in this COC.

- Understand how UMP coverage coordinates with other insurance coverage you may have, including Medicare.
- Understand your plan benefits, including what is covered, prior authorization and notice requirements, and other information described in this COC.

# Information available to you

We support the goal of giving you and your family the detailed information you need to make the best possible health care decisions. See the Directory pages at the beginning of this booklet for links and contact information.

You may find the following information in this COC:

- Benefit exclusions, reductions, and maximums or limits (see the "What the plan does not cover" section)
- Clear explanation of complaint and appeal procedures (see the "Complaint and appeal procedures" section)
- Definition of terms (see the "Definitions" section)
- List of covered expenses (see the "List of benefits" section)
- Policies regarding prescription drug coverage and how the plan adds and removes drugs from the UMP Preferred Drug List (see "The UMP Preferred Drug List" and the "Guidelines for prescription drugs UMP covers" sections)
- Preventive health care benefits that are covered (see the "Preventive care" benefit)
- Process for prior authorization, notice, or review (see the "Limits on plan coverage" section)

You may find the following on the UMP website or by contacting UMP Customer Service:

- Accreditation information, including measures used to report the plan's performance, such as consumer satisfaction survey results or Healthcare Effectiveness Data and Information Set (HEDIS) measures
- Clinical coverage criteria applicable to health care services and supplies that require prior authorization
- Description and justification for provider compensation programs, including any incentives or penalties intended to encourage providers to withhold services
- General reimbursement or payment arrangements between the plan and preferred providers
- Information on the plan's care management programs
- Notice of privacy practices (includes plan policy for protecting the confidentiality of health information; see "Confidentiality of your health information" on page 121)
- Online directory of preferred providers, including both primary care providers and specialists
- Procedures to follow for consulting with providers
- The Summary of Benefits and Coverage (SBC)
- When the plan may retroactively deny coverage for prior authorized medical services

The following are available through your Regence account or by contacting UMP Customer Service:

- Medical claims history and deductible status
- Online directory of preferred providers, including both primary care providers and specialists

The following are available on the UMP Prescription drug coverage webpage or by contacting ArrayRx Customer Service:

- The UMP Preferred Drug List
- Clinical coverage criteria applicable to prescription drugs that require prior authorization (through your online prescription drug account)
- Prescription drug claims history and deductible status (through your online prescription drug account)

You may also contact UMP Customer Service for an annual accounting of all payments made by the plan that have been counted against medical payment limits, day limits, visit limits, or other limits on your coverage. The plan will provide a written summary of payments within 30 calendar days of your request. Some of this information is also available through your Regence account.

You may contact ArrayRx Customer Service with questions about coverage of and limitations on prescription drugs.

The plan does not prevent or discourage providers from telling you about the care you require, including various treatment options and whether the provider thinks that care is consistent with the plan's coverage criteria. You may, at any time, get health care outside of plan coverage for any reason. However, you must pay for those services and supplies. In addition, the plan does not prevent or discourage you from talking about other health plans with your provider.

# Confidentiality of your health information

The plan follows the UMP Notice of Privacy Practices, available online on the HCA website at **hca.wa.gov/ump-privacy** or by contacting UMP Customer Service. The plan will release member health information only as described in that notice or as required or permitted by law or court order.

## How to designate an authorized representative

> **TIP:** Because of privacy laws, the plan usually cannot share information on appeals or complaints with family or other persons unless the member is a minor, or the plan has received written authorization to release personal health information to the other person.

In most cases, the plan must have written authorization to communicate with anyone but the member. However, a parent or legal guardian may act as a representative for a member under age 13 without written authorization, except for certain sensitive conditions. For members ages 13 to 17, a parent or legal guardian may usually act as a representative, except for certain specially protected types of information, for which the plan must receive written authorization as described below.

You may choose to authorize a representative to:

- Communicate with the plan on your behalf regarding an appeal in process.
- Share your protected health information.
- Talk to the plan about claims or services.

To authorize release of protected health information, you must complete an Authorization to Disclose Protected Health Information form. The forms for medical and prescription drug appeals are different. To get the forms, follow the instructions below:

- Medical benefits: Contact UMP Customer Service or use your Regence account (see Directory for link and contact information).
- Prescription drug benefits: Contact ArrayRx Customer Service or download the UMP (ArrayRx) Prescription drug complaints and appeals form by visiting forms and publications at **hca.wa.gov/ump-forms-pubs**.

Send the form to the address on the form. UMP cannot share information until we receive the completed form. On the form, you must specify:

- What information may be disclosed;
- The purpose of the disclosure (e.g., receiving an outcome of an appeal); and
- Who is designated to receive or release the information.

### Release of information

The plan or HCA may require you to give information when needed to determine eligibility, administer benefits, or process claims. This could include medical and other records. The plan could deny coverage if you do not provide the information when requested.

## Relationship to Blue Cross and Blue Shield Association

HCA, on behalf of itself and its members, hereby expressly acknowledges that this contract constitutes an agreement solely between HCA and Regence BlueShield, an independent corporation operating under a license from the Blue Cross and Blue Shield Association, an association of independent Blue Cross and Blue Shield plans (the "Association"), permitting the contractor Regence BlueShield to use the Washington license for those counties designated in its service area.

## Right to receive and release needed information

Regence BlueShield may need certain facts about your health care coverage or services provided to process your claims correctly. Regence may get these facts from or give them to other organizations or persons without your consent. You must give Regence any facts necessary for processing of claims to get benefits under this plan. See page 121 for more information about the confidentiality of your health information.

## Right of recovery

The plan has the right to a refund of incorrect payments. The plan may recover excess payment from any:

- Person that received an excess payment.
- Person on whose behalf an excess payment was made.
- Other issuers of payment.
- Other plans involved.

## Limitations on liability

In all cases, you have the exclusive right to choose a health care provider. Since neither UMP nor Regence BlueShield provides any health care services, neither may be held liable for any claim or damages connected with injuries you may suffer while receiving health services or supplies provided by professionals who are neither employees nor agents of either UMP or Regence BlueShield. Neither Regence BlueShield nor UMP is responsible for the quality of health care you receive, except as provided by law.

In addition, UMP will not be liable to any person or entity for the inability or failure to procure or provide the benefits of the plan by reason of epidemic, disaster, or other cause or condition beyond UMP's control.

## Governing law

The plan is governed by and construed in accordance with the laws of the United States of America and by applicable laws of Washington State without regard to its conflict of law rules.

## Anti-assignment

Members may not assign this COC, or any rights, interests or obligations contained in this COC, in whole or in part, to a third party (including, but not limited to, providers of medical services), without the plan's written consent. Any attempt to assign any rights, interests or obligations contained in this COC, in whole or in part, to a third party is void and/or invalid, and the plan will not recognize it.

## No waiver

The failure or refusal of either party to demand strict performance of the plan or to enforce any provision will not act as or be construed as a waiver of that party's right to later demand its performance or to enforce that provision. No provision of the plan will be considered waived unless such waiver is in writing and signed by one of HCA's authorized officers.

# Acronyms

ABA – Applied Behavior Analysis [Therapy]

ASC – Ambulatory surgery center

CDC – Centers for Disease Control and Prevention

CDHP – Consumer-Directed Health Plan

CHIP – Children's Health Insurance Program

CMS – Centers for Medicare and Medicaid Services

COB – Coordination of benefits

COBRA – Consolidated Omnibus Budget Reconciliation Act

COC – Certificate of coverage

DME – Durable medical equipment

EOB – Explanation of benefits

EOMB – Explanation of Medicare benefits

ER – Emergency room

ERB – Employee and Retiree Benefits [Division]

FDA – Food and Drug Administration

FMLA – Family Medical Leave Act

FSA – Flexible spending arrangement

HCA – Health Care Authority

HDHP – High-deductible health plan

HIPAA – Health Insurance Portability and Accountability Act

HRA – Health reimbursement arrangement

HRSA – Health Resources and Services Administration

HSA – Health savings account

HTCC – Health Technology Clinical Committee

IRC – Internal Revenue Code

IRO – Independent review organization

IRS – Internal Revenue Service

MSN – Medicare Summary Notice

NMSN – National Medical Support Notice

P&T – Pharmacy and Therapeutics Committee

PCP – Primary care provider

PDP – Prescription Drug Plan

PEBB – Public Employees Benefits Board

PFML – Paid Family and Medical Leave

PHP – Partial Hospitalization Program

PPACA – Patient Protection and Affordable Care Act

PPO – Preferred Provider Organization

PPS – Postal Prescription Services

RCW – Revised Code of Washington

REMS – Risk Evaluation and Mitigation Strategies

SBC – Summary of Benefits and Coverage

SEBB – School Employees Benefits Board

TMJ – Temporomandibular joint

TRS – Telecommunications Relay Service

UMP – Uniform Medical Plan

VSP – Vision Service Plan

WAC – Washington Administrative Code

# Eligibility and enrollment

In these sections, "health plan" is used to refer to a plan offering medical, dental, vision, or any combination of these coverages developed by the Public Employees Benefits Board (PEBB) and provided by a contracted vendor or self-insured plans administered by the Health Care Authority (HCA).

## Eligibility for subscribers and dependents

### Employee eligibility

The employee's state agency will inform the employee in writing whether they are eligible for PEBB benefits upon employment and whenever their eligibility status changes. The written notice will include information about the employee's right to appeal eligibility and enrollment decisions.

An employee of an employer group (such as a county, city, port, water district, etc.) that contracts with HCA for PEBB benefits should contact their payroll or benefits office for eligibility criteria.

Employees have the right to appeal eligibility and enrollment decisions. Information about appeals can be found under "Appeal rights."

### Continuation coverage eligibility

The PEBB Program determines whether subscribers are eligible for PEBB Continuation Coverage (COBRA or Unpaid Leave) upon receipt of their election to enroll. If the subscriber requests to enroll and is not eligible, the PEBB Program will notify them of their right to appeal. Information about appeals can be found under "Appeal rights."

#### Retired employee of a former employer group eligibility

The PEBB Program determines whether a retired employee, a retired school employee, or an eligible survivor of a former employer group is eligible to self-pay coverage in PEBB Continuation Coverage (Employer Group Ended Participation) upon receipt of their election to enroll. If the retired employee, the retired school employee, or the survivor requests to enroll and is not eligible, the PEBB Program will notify them of their right to appeal. Information about appeals can be found under "Appeal rights."

### Dependent eligibility

The following are eligible dependents:

- **Legal spouse**
- **State-registered domestic partner** and substantially equivalent legal unions from jurisdictions as defined in Washington state statute. Individuals in a state-registered domestic partnership are treated the same as a legal spouse except when in conflict with federal law.
- **Children**, through the last day of the month in which their 26th birthday occurred regardless of marital status, student status, or eligibility for coverage under another plan. It also includes children age 26 or older with a disability as described below in "Children of any age with a developmental or physical disability." Children are defined as the subscriber's:
  - **Children based on establishment of a parent-child relationship,** as described in Washington state statutes, except when parental rights have been terminated.
  - **Children of the subscriber's spouse,** based on the spouse's establishment of a parent-child relationship, except when parental rights have been terminated. The stepchild's relationship to the subscriber (and eligibility as a dependent) ends on the same date the marriage with the spouse ends through divorce, annulment, dissolution, termination, or death.
  - **Children for whom the subscriber has assumed a legal obligation** for total or partial support in anticipation of adoption of the child.

- **Children of the subscriber's state-registered domestic partner,** based on the state-registered domestic partner's establishment of a parent-child relationship, except when parental rights have been terminated. The child's relationship to the subscriber (and eligibility as a dependent) ends on the same date the subscriber's legal relationship with the state-registered domestic partner ends through divorce, annulment, dissolution, termination, or death.
- **Children specified in a court order or divorce decree** for whom the subscriber has a legal obligation to provide support or health care coverage.
- **Extended dependents in the legal custody or legal guardianship of the subscriber, the subscriber's spouse, or the subscriber's state-registered domestic partner.** The legal responsibility is demonstrated by a valid court order and the child's official residence with the custodian or guardian. Extended dependent child does not include foster children unless the subscriber, the subscriber's spouse, or the subscriber's state-registered domestic partner has assumed a legal obligation for total or partial support in anticipation of adoption.
- **Children of any age with a developmental or physical disability** that renders them incapable of self-sustaining employment and chiefly dependent upon the subscriber for support and maintenance, provided such condition occurs before the age of 26. The following requirements apply to a dependent child with a disability:
  - The subscriber must provide proof of the disability and dependency within 60 days of the child's attainment of age 26.
  - The subscriber must notify the PEBB Program in writing when the child is no longer eligible under this subsection.
  - A child with a developmental or physical disability who becomes self-supporting is not eligible as of the last day of the month in which they become capable of self-support.
  - A child with a developmental or physical disability age 26 and older who becomes capable of self-support does not regain eligibility if they later become incapable of self-support.
  - The PEBB Program, with input from the medical plan, will periodically verify the eligibility of a dependent child with a disability beginning at age 26, but no more frequently than annually after the two-year period following the child's 26th birthday. Verification will require renewed proof of disability and dependence from the subscriber.

# Enrollment for subscribers and dependents

## For all subscribers and dependents

- To enroll at any time other than during the initial enrollment period, see "Making changes."
- Any dependents enrolled in PEBB medical will be enrolled in the same medical plan as the subscriber. Exceptions may apply if there are both Medicare and non-Medicare enrollees on the same account.

## Employee enrollment

An employee must use Benefits 24/7, the PEBB Program's online enrollment system, or submit a *PEBB Employee Enrollment/Change* form or *PEBB Employee Enrollment/Change form for Medical Only Groups*, and any supporting documents to their employing agency when they become newly eligible or regain eligibility for PEBB benefits. The online enrollment must be completed or the form must be received no later than 31 days after the date the employee becomes eligible or regains eligibility.

If the employee does not enroll online or return the form by the deadline, the employee will be enrolled in Uniform Medical Plan Classic and a tobacco use premium surcharge will be incurred. Consequently,

dependents cannot be enrolled until the PEBB Program's next annual open enrollment or when a qualifying event occurs that creates a special open enrollment for enrolling a dependent.

### Waiving medical enrollment

An eligible employee may waive enrollment in PEBB medical if they are enrolled in other employer-based group medical, a TRICARE plan, or Medicare Part A and Part B. They may not waive enrollment in PEBB medical if they are enrolled in PEBB retiree insurance coverage. The PEBB Program will automatically defer their enrollment or automatically disenroll them from PEBB retiree insurance coverage when they become eligible for the employer contribution toward PEBB benefits.

An employee may waive their enrollment in PEBB medical to enroll in School Employees Benefits Board (SEBB) medical only if they are enrolled in SEBB dental and SEBB vision. An employee who waives enrollment in PEBB medical to enroll in SEBB medical also waives enrollment in PEBB dental and PEBB vision. An employee may not waive PEBB medical, PEBB dental, and PEBB vision to enroll in SEBB medical with a SEBB participating employer group that offers only medical coverage.

If an employee waives enrollment in PEBB medical, the employee cannot enroll eligible dependents. For information on when an eligible employee may waive medical plan enrollment after their initial enrollment period, or to enroll after having waived, see "Making changes."

## Continuation coverage and retired employees of a former employer group enrollment

A subscriber enrolling in PEBB Continuation Coverage (COBRA or Unpaid Leave) or PEBB Continuation Coverage (Employer Group Ended Participation) may enroll by using Benefits 24/7 or by submitting the applicable *PEBB Continuation Coverage Election/Change* form and any supporting documents to the PEBB Program.

For PEBB Continuation Coverage (COBRA or Unpaid Leave), the online enrollment must be completed or the PEBB Program must receive the election form no later than 60 days from the date the enrollee's PEBB health plan coverage ended or from the postmark date on the *PEBB Continuation Coverage Election Notice* sent by the PEBB Program, whichever is later. For PEBB Continuation Coverage (Employer Group Ended Participation), the online enrollment must be completed or the PEBB Program must receive the required form no later than 60 days after the employer group's date of termination.

Premiums and applicable premium surcharges associated with continuing PEBB medical must be made directly to HCA. The first premium payment and applicable premium surcharges are due to HCA no later than 45 days after the election period ends as described above. For more information, see "Options for continuing PEBB medical coverage" and the *PEBB Continuation Coverage Election Notice* sent by the PEBB Program.

## Dependent enrollment

To enroll an eligible dependent, the subscriber must include the dependent's information using Benefits 24/7 or on the applicable enrollment form and provide the required document(s) as proof of the dependent's eligibility. The dependent will not be enrolled in PEBB health plan coverage if the PEBB Program or the employing agency is unable to verify their eligibility within the PEBB Program enrollment timelines.

### National Medical Support Notice (NMSN)

When a National Medical Support Notice (NMSN) requires a subscriber to provide health plan coverage for a dependent child, the following provisions apply:

The subscriber may enroll their dependent child and request changes to their health plan coverage as described under "Changes to health plan coverage or enrollment are allowed as directed by the NMSN" below.

- **An employee** must use Benefits 24/7 or submit the required form(s) to their employing agency.
- **Any other subscriber** must use Benefits 24/7 or submit the required form(s) to the PEBB Program.

If the subscriber fails to request enrollment or health plan coverage changes as directed by the NMSN, the employing agency or the PEBB Program may make enrollment or health plan coverage changes according to "Changes to health plan coverage or enrollment are allowed as directed by the NMSN" below, upon request of:

- The child's other parent.
- A child support enforcement program.

**Changes to health plan coverage or enrollment are allowed as directed by the NMSN:**

H. The dependent will be enrolled under the subscriber's health plan coverage as directed by the NMSN.

I. An employee who has waived PEBB medical will be enrolled in medical as directed by the NMSN, in order to enroll the dependent.

J. The subscriber's selected health plans will be changed if directed by the NMSN.

K. If the dependent is already enrolled under another PEBB subscriber, the dependent will be removed from the other health plan coverage and enrolled as directed by the NMSN.

L. If the dependent is enrolled in both PEBB medical and School Employees Benefits Board (SEBB) medical as a dependent and there is an NMSN in place, enrollment will be in accordance with the NMSN.

M. If the subscriber is eligible for and elects Consolidated Omnibus Budget Reconciliation Act (COBRA) coverage or other continuation coverage, the NMSN will be enforced and the dependent must be covered in accordance with the NMSN.

Changes to health plan coverage or enrollment as described above in A through C will begin the first day of the month following receipt of the NMSN. If the NMSN is received on the first day of the month, the change to health plan coverage or enrollment begins on that day.

A dependent will be removed from the subscriber's health plan coverage, as described above in D, the last day of the month the NMSN is received. If that day is the first of the month, the change in enrollment will be made the last day of the previous month.

When a NMSN requires a spouse, former spouse, or other individual to provide health plan coverage for a dependent who is already enrolled in the subscriber's PEBB coverage, and that health plan coverage is in fact provided, the dependent may be removed from the subscriber's PEBB health plan coverage prospectively.

# Dual enrollment

**A subscriber** and their dependents may each be enrolled in only one PEBB medical plan.

**An employee** or their dependent who is eligible to enroll in both the PEBB Program and the School Employees Benefits Board (SEBB) Program is limited to a single enrollment in either the PEBB or SEBB Program.

For example:

- A child who is an eligible dependent under two parents enrolled in PEBB Program benefits may be enrolled as a dependent under both parents but is limited to a single enrollment in PEBB medical.
- A child who is an eligible dependent of an employee in the PEBB Program and a school employee in the SEBB Program may only be enrolled as a dependent under one parent in either the PEBB or SEBB Program.

# Medicare eligibility and enrollment

## Employee and dependent

**If an employee** or their dependent becomes eligible for Medicare, they should contact the Social Security Administration to ask about the advantages of immediate or deferred Medicare enrollment.

An employee or their dependent are deemed eligible for Medicare when they have the option to receive Medicare Part A benefits. An employee that chooses to enroll in Medicare Part A is no longer eligible to contribute to a health savings account. If an employee continues to do so, they will be liable for any tax penalties resulting from contributions made when they are no longer eligible. If an employee or their dependent chooses to enroll in Medicare Part A, Medicare regulations and guidelines will determine whether Medicare is the primary or secondary payer.

An employee or their dependent who is enrolled in Medicare may remain enrolled in PEBB medical. However, an employee may choose to waive their PEBB medical if they are enrolled in Medicare Part A and Part B or remove their dependent from their PEBB medical and choose Medicare as their primary insurer. If an employee does so, neither the employee nor their dependent can enroll in PEBB medical except during the annual open enrollment or when a specific life event creates a special open enrollment.

In most situations, an employee and their dependent can defer Medicare Part B enrollment without a penalty while enrolled in PEBB medical. When the employee terminates employment, the employee and the dependent can enroll in Medicare Part B during a Special Enrollment Period. If Medicare eligibility is due to a disability, the employee or their dependent must contact the Social Security Administration about deferring enrollment in Medicare Part B.

**Upon retirement,** Medicare will become the primary insurance payer, and the PEBB medical plan will become secondary. See "PEBB retiree insurance coverage."

## Continuation coverage subscriber, a retired employee of a former employer group, or their dependent

**If a continuation coverage subscriber, a retired employee of a former employer group,** or their dependent becomes eligible for Medicare when they are turning age 65 but not getting Social Security benefits, federal regulations allow a seven-month initial enrollment period to enroll in Medicare. If they do not enroll within three months before the month they turn age 65, enrollment in Medicare may be delayed. If enrollment in Medicare does not occur when the subscriber or their dependent is first eligible, a late enrollment penalty may apply.

# When medical coverage begins

## Employees and dependents

**For a newly eligible employee** and their eligible dependents, medical coverage begins the first day of the month following the date the employee becomes eligible. If the employee becomes eligible on the first working day of the month, then coverage begins on that date.

If the eligible employee is a faculty member hired on a quarter-to-quarter or semester-to-semester basis, medical coverage begins the first day of the month following the beginning of the second consecutive

quarter or semester. If the first day of the second consecutive quarter or semester is the first working day of the month, medical coverage begins on that day.

**For an employee regaining eligibility,** including following a period of leave or after being between periods of leave as described in PEBB Program rules, and their eligible dependents, medical coverage begins the first day of the month the employee is in pay status eight or more hours. If the employee is a returning faculty member regaining eligibility no later than the 12th month after the month in which they lost eligibility for the employer contribution toward PEBB benefits, medical coverage begins the first day of the month in which the quarter or semester begins.

**Note:** When an employee who is called to active duty in the uniformed services under the Uniformed Services Employment and Reemployment Rights Act (USERRA) loses eligibility for the employer contribution toward PEBB benefits, they regain eligibility for the employer contribution toward PEBB benefits the day they return from active duty. Medical coverage begins the first day of the month in which the employee returns from active duty.

# Continuation coverage subscribers and dependents

**For a continuation coverage subscriber** and their eligible dependents enrolling when newly eligible due to a qualifying event, medical coverage begins the first day of the month following the day they lost eligibility for PEBB medical.

## Retired employees of a former employer group and dependents

**For a retired employee of a former employer group** and their eligible dependents enrolling when newly eligible, medical coverage begins the first day of the month following the day they lost eligibility for PEBB retiree insurance coverage.

# All subscribers and dependents

**For a subscriber or their eligible dependents enrolling during the PEBB Program's annual open enrollment,** medical coverage begins January 1 of the following year.

**For a subscriber or their eligible dependents enrolling during a special open enrollment,** medical coverage begins the first day of the month following the event date or the date the online enrollment election using Benefits 24/7 or the required form is received, whichever is later. If that day is the first of the month, medical coverage begins on that day.

If the special open enrollment is due to the **birth or adoption of a child**, or when the subscriber has assumed a legal obligation for total or partial support in anticipation of adoption of a child, medical coverage will begin as follows:

- **For an employee**, medical coverage will begin the first day of the month in which the event occurs.
- **For a newly born child**, medical coverage will begin the date of birth.
- **For a newly adopted child**, medical coverage will begin on the date of placement or the date a legal obligation is assumed in anticipation of adoption, whichever is earlier.
- **For a spouse or state-registered domestic partner** of a subscriber, medical coverage will begin the first day of the month in which the event occurs.

If the special open enrollment is due to the enrollment of an **extended dependent or a dependent child with a disability**, medical coverage will begin the first day of the month following the event date or eligibility certification, whichever is later.

## Removing a dependent who is no longer eligible

A subscriber must provide notice to remove a dependent who is no longer eligible due to divorce, annulment, dissolution, or a qualifying event of a dependent ceasing to be eligible as a dependent child as described under "Dependent eligibility." The notice must be received within 60 days of the last day of the month the dependent no longer meets the eligibility criteria.

- **An employee** must provide notice online using Benefits 24/7 or by submitting a written request to their employing agency.
- **Any other subscriber** must provide notice online using Benefits 24/7 or by submitting a written request to the PEBB Program.

Consequences for not submitting notice within the required 60 days may include, but are not limited to:

- The dependent may lose eligibility to continue PEBB medical under one of the continuation coverage options described in "Options for continuing PEBB medical coverage."
- The subscriber may be billed for claims paid by the medical plan for services that were rendered after the dependent lost eligibility.
- The subscriber may not be able to recover subscriber-paid insurance premiums for the dependent that lost eligibility.
- The subscriber may be responsible for premiums paid by the state for the dependent's medical plan coverage after the dependent lost eligibility.

## Voluntary termination for continuation coverage subscribers or retired employees of a former employer group

A continuation coverage subscriber or a retired employee of a former employer group may voluntarily terminate enrollment in a medical plan at any time by submitting a request online using Benefits 24/7 or in writing to the PEBB Program. Enrollment in the medical plan will be terminated the last day of the month in which the request was received online or by the PEBB Program, or on the last day of the month specified in the termination request, whichever is later. If the request is received on the first day of the month, medical plan enrollment will be terminated on the last day of the previous month.

## Making changes during annual open enrollment and special open enrollment

A subscriber may make certain changes to their enrollment during the annual open enrollment or when a specific life event creates a special open enrollment period.

### Annual open enrollment changes

**An employee** may make the following changes to their enrollment during the PEBB Program's annual open enrollment period:

- Change their medical plan
- Waive their medical plan enrollment
- Enroll after waiving medical plan enrollment
- Enroll or remove eligible dependents

**An employee** must submit the election change online using Benefits 24/7 or submit the required *PEBB Employee Enrollment/Change* form and any supporting documents to their employing agency. The change

must be completed online, or the forms received, no later than the last day of the annual open enrollment period.

**Any other subscriber** may make the following changes to their enrollment during the PEBB Program's annual open enrollment period:

- Enroll in or terminate enrollment in a medical plan
- Change their medical plan
- Enroll or remove eligible dependents

They must submit the election change online using Benefits 24/7 or submit the required *PEBB Continuation Coverage Election/Change* form (as appropriate) and any supporting documents to the PEBB Program. The change must be completed online, or the forms received, no later than the last day of the annual open enrollment period.

The change will be effective January 1 of the following year, except when a subscriber chooses to terminate enrollment or remove an eligible dependent, then the change will be effective on December 31.

## Special open enrollment changes

A subscriber may change their enrollment outside of the annual open enrollment period if a qualifying event creates a special open enrollment period. However, the change in enrollment must be allowable under Internal Revenue Code (IRC) and Treasury Regulations and correspond to and be consistent with the event that creates the special open enrollment for the subscriber, their dependent, or both.

A special open enrollment event must be other than an employee gaining initial eligibility or regaining eligibility for PEBB benefits.

A special open enrollment may allow a subscriber to make the following changes:

- Enroll in or change their medical plan
- Waive their medical plan enrollment
- Enroll after waiving medical plan enrollment
- Enroll or remove eligible dependents

To request a special open enrollment:

- **An employee** must make the change online using Benefits 24/7 or submit the required *PEBB Employee Enrollment/Change* form and any supporting documents to their employing agency.
- **Any other subscriber** must make the change online using Benefits 24/7 or submit the required *PEBB Continuation Coverage Election/Change* form (as appropriate) and any supporting documents to the PEBB Program.

The change must be completed online, or the forms must be received, no later than 60 days after the event that creates the special open enrollment. In many instances, the date the change is received online or the date the form is received affects the effective date of the enrollment or change in enrollment. Submitting the requested change sooner may avoid a delay in the enrollment or change. In addition, the PEBB Program or the employing agency will require the subscriber to provide proof of a dependent's eligibility, evidence of the event that created the special open enrollment, or both.

**Exceptions:** If a subscriber wants to enroll a newborn or child whom the subscriber has adopted or has assumed a legal obligation for total or partial support in anticipation of adoption, the subscriber should complete the request online or notify their employing agency or the PEBB Program by submitting the required forms as soon as possible to ensure timely payment of claims. If adding the child increases the premium, the request must be received online, or the required forms must be received, no later than 60

days after the date of the birth, adoption, or the date the legal obligation is assumed for total or partial support in anticipation of adoption.

**A continuation coverage subscriber or a retired employee of a former employer group:**

- Has six months from the date of their or their dependent's enrollment in Medicare Part B to change their enrollment to a PEBB Medicare Supplement Plan. The change must be made online using Benefits 24/7 or the PEBB Program must receive the required form(s) no later than six months after the enrollment in Medicare Part B for either the subscriber or their dependent.
- Has seven months to enroll in a Medicare Advantage Prescription Drug (MAPD) plan or Uniform Medical Plan (UMP) Classic Medicare with Part D (PDP) that begins three months before they or their dependent first enrolled in both Medicare Part A and Part B and ends three months after the month of Medicare eligibility. They may also enroll themselves or their dependent in a MAPD plan or UMP Classic Medicare with Part D (PDP) before their last day of the Medicare Part B initial enrollment period. The change must be made online using Benefits 24/7 or the forms must be received by the PEBB Program, no later than the last day of the month prior to the month they or their dependent enrolls in the MAPD plan or UMP Classic Medicare with Part D (PDP).
- Has three months to enroll in a MAPD plan, Medicare Supplement plan, or the UMP Classic Medicare with Part D (PDP) if eligible, after they or their dependent gain, lose, or have a change in low-income subsidy (LIS) eligibility, or notification of such a change, whichever is later. The change must be made online using Benefits 24/7 or the forms must be received by the PEBB Program, no later than three months after they or their dependent gain, lose, or have a change in LIS eligibility, or notification of such a change, whichever is later.

## Special open enrollment events that allow for a change in health plans

A subscriber may not change their health plan during a special open enrollment if their state-registered domestic partner or state-registered domestic partner's child is not a tax dependent.

Any of the following events may create a special open enrollment:

- Subscriber gains a new dependent due to:
  - Marriage or registering a state-registered domestic partnership.
  - Birth, adoption, or assuming a legal obligation for total or partial support in anticipation of adoption.
  - A child becoming eligible as an extended dependent through legal custody or legal guardianship.
- Subscriber or their dependent loses other coverage under a group health plan or through health insurance coverage, as defined by the Health Insurance Portability and Accountability Act (HIPAA).
- Subscriber has a change in employment status that affects their eligibility for the employer contribution toward their employer-based group health plan.
- Subscriber's dependent has a change in their own employment status that affects their eligibility or their dependent's eligibility for the employer contribution under their employer-based group health plan. "Employer contribution" means contributions made by the dependent's current or former employer toward health coverage as described in the Treasury Regulation.
- Subscriber or their dependent has a change in residence that affects health plan availability. If the subscriber has a change in residence and their current medical plan is no longer available, the subscriber must select a new medical plan. If the subscriber does not elect a new medical plan as required, they will be enrolled in a PEBB medical plan designated by the director of HCA or their designee.

- A court order requires the subscriber or any other individual to provide insurance coverage for an eligible dependent of the subscriber (a former spouse or former state-registered domestic partner is not an eligible dependent).
- Subscriber or their dependent enrolls in coverage under Medicaid or a state Children's Health Insurance Program (CHIP), or the subscriber or their dependent loses eligibility for coverage under Medicaid or CHIP.
- Subscriber or their dependent becomes eligible for state premium assistance subsidy for PEBB health plan coverage from Medicaid or CHIP.
- Subscriber or their dependent enrolls in coverage under Medicare, or the subscriber or their dependent loses eligibility for coverage under Medicare or enrolls in or terminates enrollment in a Medicare Advantage Prescription Drug or a Part D plan. If the subscriber's current medical plan becomes unavailable due to the subscriber or their dependent's enrollment in Medicare, the subscriber must select a new medical plan.
- Subscriber or their dependent gains, loses, or has a change in low-income subsidy eligibility.
- Subscriber or their dependent's current medical plan becomes unavailable because the subscriber or enrolled dependent is no longer eligible for a health savings account (HSA).
- Subscriber or their dependent experiences a disruption of care for active and ongoing treatment that could function as a reduction in benefits for the subscriber or their dependent. The subscriber may not change their health plan election because the subscriber or dependent's physician stops participation with the subscriber's health plan unless the PEBB Program determines that a continuity of care issue exists. The PEBB Program will consider but not limit its consideration to the following:
  - Active cancer treatment, such as chemotherapy or radiation therapy
  - Treatment following a recent organ transplant
  - A scheduled surgery
  - Recent major surgery still within the postoperative period
  - Treatment for a high-risk pregnancy
- Subscriber or their dependent becomes eligible and enrolls in a TRICARE plan or loses eligibility for a TRICARE plan.
- The PEBB Program determines that there has been a substantial decrease in the providers available under a PEBB medical plan.
- The PEBB Program determines there has been a substantial increase in the employee's premiums when the employee transfers from one employing agency to another employing agency.

**Note:** The plan cannot guarantee that any physician, hospital, or other provider will be available or remain under contract with the plan. An enrollee may not change medical plans simply because their provider or health care facility discontinues participation with this medical plan until the PEBB Program's next annual open enrollment or when another qualifying event creates a special open enrollment for changing health plans, unless the PEBB Program determines that a continuity of care issue exists or there has been a substantial decrease in the providers available under the plan.

## Special open enrollment events that allow adding or removing a dependent

Any of the following events may create a special open enrollment:

- Subscriber gains a new dependent due to:
  - Marriage or registering a state-registered domestic partnership.

    - Birth, adoption, or when the subscriber has assumed a legal obligation for total or partial support in anticipation of adoption.
    - A child becoming eligible as an extended dependent through legal custody or legal guardianship.
- Subscriber or their dependent loses other coverage under a group health plan or through health insurance coverage, as defined by the Health Insurance Portability and Accountability Act (HIPAA).
- Subscriber has a change in employment status that affects their eligibility for the employer contribution toward their employer-based group health plan.
- Subscriber's dependent has a change in their own employment status that affects their eligibility or their dependent's eligibility for the employer contribution under their employer-based group health plan. "Employer contribution" means contributions made by the dependent's current or former employer toward health coverage as described in the Treasury Regulation.
- Subscriber or their dependent has a change in enrollment under an employer-based group health plan during its annual open enrollment that does not align with the PEBB Program's annual open enrollment.
- Subscriber's dependent has a change in residence from outside of the United States to within the United States, or from within the United States to outside of the United States and that change in residence resulted in the dependent losing their health insurance.
- A court order requires the subscriber or any other individual to provide insurance coverage for an eligible dependent of the subscriber (a former spouse or former state-registered domestic partner is not an eligible dependent).
- Subscriber or their dependent enrolls in coverage under Medicaid or a state Children's Health Insurance Program (CHIP), or the subscriber or their dependent loses eligibility for coverage under Medicaid or CHIP.
- Subscriber or their dependent becomes eligible for a state premium assistance subsidy for PEBB health plan coverage from Medicaid or CHIP.
- Subscriber's dependent enrolls in Medicare or loses eligibility for Medicare.
- Subscriber or their dependent enrolled in a Medicare Advantage Prescription Drug plan, Medicare Supplement plan, or the Uniform Medical Plan Classic Medicare with Part D (PDP) who gains, loses, or has a change to their low-income subsidy eligibility.
- Subscriber or their dependent becomes eligible and enrolls in a TRICARE plan or loses eligibility for a TRICARE plan.

## Special open enrollment events that allow waiving medical enrollment and enrolling after waiving

An employee may waive PEBB medical during a special open enrollment if they are enrolled in other employer-based group medical, a TRICARE plan, or Medicare Part A and Part B. An employee may not waive enrollment in PEBB medical if they are enrolled in PEBB retiree insurance coverage. Their enrollment in PEBB retiree insurance coverage will be automatically deferred or automatically disenrolled by the PEBB Program.

Any of the following events may create a special open enrollment:

- Employee gains a new dependent due to:
    - Marriage or registering a state-registered domestic partnership.

  - Birth, adoption, or when the employee has assumed a legal obligation for total or partial support in anticipation of adoption.
  - A child becoming eligible as an extended dependent through legal custody or legal guardianship.
- Employee or their dependent loses other coverage under a group health plan or through health insurance coverage, as defined by the HIPAA.
- Employee has a change in employment status that affects their eligibility for the employer contribution toward their employer-based group medical.
- Employee's dependent has a change in their own employment status that affects their eligibility or their dependent's eligibility for the employer contribution under their employer-based group medical. "Employer contribution" means contributions made by the dependent's current or former employer toward health coverage as described in the Treasury Regulation.
- Employee or their dependent has a change in enrollment under an employer-based group medical plan during its annual open enrollment that does not align with the PEBB Program's annual open enrollment.
- Employee's dependent has a change in residence from outside of the United States to within the United States, or from within the United States to outside of the United States and the change in residence resulted in the dependent losing their health insurance.
- A court order requires the employee or any other individual to provide a health plan for an eligible dependent of the employee (a former spouse or former state-registered domestic partner is not an eligible dependent).
- Employee or their dependent enrolls in coverage under Medicaid or a state Children's Health Insurance Program (CHIP), or the employee or their dependent loses eligibility for coverage under Medicaid or CHIP. **Note:** An employee may only return from having waived PEBB medical for the events described in this paragraph. An employee may not waive their PEBB medical for the events described in this paragraph.
- Employee or their dependent becomes eligible for a state premium assistance subsidy for PEBB health plan coverage from Medicaid or CHIP.
- Employee or their dependent becomes eligible and enrolls in a TRICARE plan or loses eligibility for a TRICARE plan.
- Employee becomes eligible and enrolls in Medicare or loses eligibility for Medicare.

# When medical coverage ends

## Termination dates

Medical coverage ends on the following dates:

- On the last day of the month when any enrollee ceases to be eligible.
- On the date a medical plan terminates due to a change in contracted service area or when the group policy ends. If that should occur, the subscriber will have the opportunity to enroll in another PEBB medical plan.
- **For an employee** and their dependents, on the last day of the month the employment relationship is terminated. The employment relationship is considered terminated:
  - On the date specified in an employee's letter of resignation.
  - On the date specified in any contract or hire letter.

- On the effective date of an employer-initiated termination notice.

  **Note:** If the employing agency deducted the employee's premium for PEBB insurance coverage after the employee was no longer eligible for the employer contribution, medical coverage ends the last day of the month that employee premiums were deducted.

- **For a continuation coverage subscriber or a retired employee of a former employer group** who submits a request to terminate medical coverage, enrollment in medical coverage will be terminated the last day of the month in which the request was received online using Benefits 24/7 or by the PEBB Program, or on the last day of the month specified in the termination request, whichever is later. If the request is received on the first day of the month, medical coverage will be terminated on the last day of the previous month.

A subscriber will be responsible for payment of any services received after the date medical coverage ends as described above.

# Final premium payments

Premium payments and applicable premium surcharges are not prorated during any month, for any reason, even if an enrollee dies or asks to terminate their medical plan before the end of the month.

If the monthly premium or applicable premium surcharges remain unpaid for 30 days, the account will be considered delinquent. A subscriber is allowed a grace period of 30 days from the date the monthly premiums or applicable premium surcharges become delinquent to pay the unpaid premium balance and applicable premium surcharges. If the subscriber's premium balance or applicable premium surcharges remain unpaid for 60 days from the original due date, the subscriber's medical coverage (including enrolled dependents) will be terminated retroactive to the last day of the month for which the monthly premiums and any applicable premium surcharges were paid. When a subscriber is terminated for non-payment of premiums and applicable premium surcharges, they may not enroll again unless they regain eligibility.

## If an enrollee is hospitalized

An enrollee who is receiving covered services in a hospital on the date medical coverage ends will continue to be eligible for covered services while an inpatient for the condition which the enrollee was hospitalized, until one of the following events occur:

- According to this plan's clinical criteria, it is no longer medically necessary for the enrollee to be an inpatient at the facility.
- The remaining benefits available for the hospitalization are exhausted, regardless of whether a new calendar year begins.
- The enrollee becomes covered under another agreement with a group health plan that provides benefits for the hospitalization.
- The enrollee becomes enrolled under an agreement with another carrier that provides benefits for the hospitalization.

This provision will not apply if the enrollee is covered under another agreement that provides benefits for the hospitalization at the time medical coverage ends, except as set forth in this section, or if the enrollee is eligible for PEBB Continuation Coverage as described in "Options for continuing PEBB medical coverage."

# Options for continuing PEBB medical coverage

When medical coverage ends, the subscriber and their dependents covered by this medical plan may be eligible to continue PEBB medical coverage during temporary or permanent loss of eligibility.

There are three options the subscriber and their dependents may qualify for when coverage ends.

- PEBB Continuation Coverage (COBRA)
- PEBB Continuation Coverage (Unpaid Leave)
- PEBB retiree insurance coverage, this may include employees separating from employment

A subscriber also has the right to convert to individual medical insurance coverage with the plan when continuation of group medical insurance coverage is no longer possible.

### PEBB Continuation Coverage

The PEBB Program administers the following continuation coverage options to temporarily extend group insurance coverage when the enrollee's PEBB medical plan coverage ends due to a qualifying event:

- **PEBB Continuation Coverage (COBRA)** includes eligibility and administrative requirements under federal COBRA laws and regulations. Some enrollees who are not qualified beneficiaries under federal COBRA, may also qualify for PEBB Continuation Coverage (COBRA).
- **PEBB Continuation Coverage (Unpaid Leave)** is an option created by the PEBB Program with wider eligibility criteria and qualifying event types than COBRA.

An enrollee who qualifies for both types of PEBB Continuation Coverage (COBRA and Unpaid Leave) may enroll in only one of these options. See "Continuation coverage enrollment" and the *PEBB Continuation Coverage Election Notice* sent by the PEBB Program.

**Premium payments for PEBB Continuation Coverage**

If a subscriber enrolls in continuation coverage, the subscriber is responsible for timely payment of premiums and applicable premium surcharges.

### PEBB retiree insurance coverage

The following are eligible to continue enrollment or defer (postpone) enrollment in PEBB retiree insurance coverage if they meet procedural and substantive eligibility requirements:

- Retiring employee or retiring school employee
- Separating employee or separating school employee who is at least age 55 and meets specific retirement plan and years of service requirements and who will not yet receive a monthly retirement plan payment
- Eligible elected or full-time appointed official of the legislative or executive branch of state government leaving public office
- Dependent becoming eligible as a survivor
- Retiree or survivor enrolled in PEBB retiree insurance coverage

For details, see the *PEBB Retiree Enrollment Guide*.

## Family and Medical Leave Act of 1993

An employee on approved leave under the federal Family and Medical Leave Act (FMLA) may continue to receive the employer contribution toward PEBB benefits in accordance with the federal FMLA.

The employing agency determines if the employee is eligible for leave and the duration of the leave under FMLA. The employee must continue to pay their monthly premium contribution and applicable premium surcharges during this period.

If an employee exhausts the period of leave approved under FMLA, they may continue PEBB insurance coverage by self-paying the monthly premium and applicable premium surcharges set by HCA, with no contribution from the employing agency. See "Options for continuing PEBB medical coverage."

## Paid Family and Medical Leave Act

An employee on approved leave under the Washington state Paid Family and Medical Leave (PFML) Program may continue to receive the employer contribution toward PEBB benefits.

The Employment Security Department determines if the employee is eligible for leave under PFML. The employee must continue to pay their monthly premium contribution and applicable premium surcharges during this period.

If an employee exhausts the period of leave approved under PFML, they may continue PEBB insurance coverage by self-paying the monthly premium and applicable premium surcharges set by HCA, with no contribution from the employing agency. See "Options for continuing PEBB medical coverage."

### Availability of other coverage

An enrollee (including a dependent of a subscriber terminated for cause) has the right to switch from PEBB group medical to an individual plan when they are no longer eligible to continue the PEBB group medical plan and are not eligible for Medicare or covered under another group insurance coverage that provides benefits for hospital or medical care.

To receive assistance in finding an individual plan that will meet your needs, call us at 1-833-458-0656.

# General provisions for eligibility and enrollment

## Payment of premiums during a labor dispute

Any employee or dependent whose monthly premiums are paid in full or in part by the employing agency may pay premiums directly to HCA if the employee's compensation is suspended or terminated directly or indirectly because of a strike, lockout, or any other labor dispute, for a period not to exceed six months.

When the employee's compensation is suspended or terminated, HCA will notify the employee immediately, by mail at the last address of record, that the employee may pay premiums as they become due.

If coverage is no longer available to the employee under this certificate of coverage, then the employee may be eligible to purchase an individual medical plan from this plan consistent with premium rates filed with the Washington State Office of the Insurance Commissioner.

## Termination for just cause

The purpose of this provision is to allow for a fair and consistent method to process the plan-designated provider's request to terminate coverage from this plan for just cause.

An eligible dependent may have coverage terminated by HCA for the following reasons:

- Failure to comply with the PEBB Program's procedural requirements, including failure to provide information or documentation requested by the due date in written requests from the PEBB Program
- Knowingly providing false information
- Failure to pay the monthly premium and applicable premium surcharges when due
- Misconduct. Examples of such termination include, but are not limited to the following:
  - Fraud, intentional misrepresentation or withholding of information the subscriber knew or should have known was material or necessary to accurately determine eligibility or the correct premium
  - Abusive or threatening conduct repeatedly directed to an HCA employee, a health plan, or other HCA-contracted vendor providing PEBB insurance coverage on behalf of HCA, its employees, or other persons

The PEBB Program will enroll an employee and their eligible dependents in another PEBB medical plan upon termination from this plan.

## Appeal rights

**Any current or former employee of a state agency** or their dependent may appeal a decision made by the state agency regarding PEBB eligibility, enrollment, or premium surcharges to the state agency.

**Any current or former employee of an employer group**, such as a county, city, port, water district, etc., that contracts with HCA for PEBB benefits, or their dependent may appeal a decision made by an employer group regarding PEBB eligibility, enrollment, or premium surcharges to the employer group.

**Any enrollee** may appeal a decision made by the PEBB Program regarding PEBB eligibility, enrollment, premium payments, or premium surcharges to the PEBB Appeals Unit.

**Any enrollee** may appeal a decision regarding the administration of a PEBB medical plan by following the appeal provisions of the plan, except when regarding eligibility, enrollment, and premium payment decisions.

Learn more at **hca.wa.gov/pebb-appeals**.

## Relationship to law and regulations

Any provision of this certificate of coverage that is in conflict with any governing law or regulation of Washington state is hereby amended to comply with the minimum requirements of such law or regulation.

# Eligibility and enrollment for a retiree or survivor

In these sections, the term "retiree" or "retiring employee" includes a retiring or eligible separating employee from a Public Employees Benefits Board (PEBB) employing agency or employer group, and an elected or full-time appointed official of the legislative and executive branch of state government. The term "retiree" or "retiring school employee" includes a retiring or eligible separating school employee from a School Employees Benefits Board (SEBB) organization or employer group. Additionally, "health plan" is used to refer to a plan offering medical, dental, vision, or any combination of these coverages developed by PEBB and provided by a contracted vendor or self-insured plans administered by the Health Care Authority (HCA).

## Eligibility for subscribers and dependents

### Retiree eligibility

The PEBB Program determines if a retiring employee or retiring school employee is eligible to enroll in PEBB retiree insurance coverage upon receipt of their election to enroll. If the retiring employee or retiring school employee does not have substantive eligibility or does not meet the procedural requirements for enrollment in PEBB retiree insurance coverage, the PEBB Program will notify them of their right to appeal eligibility decisions. Information about appeals can be found under "Appeal rights."

### Survivor eligibility

The PEBB Program determines whether a dependent is eligible to enroll or continue enrollment in PEBB retiree insurance coverage as a survivor. If an election to enroll is required, eligibility will be determined upon receipt of their election to enroll. If the survivor does not meet the eligibility and procedural requirements for enrollment in PEBB retiree insurance coverage, the PEBB Program will notify them of their right to appeal. Information about appeals can be found under "Appeal rights."

# Dependent eligibility

The following are eligible dependents:

- **Legal spouse**
- **State-registered domestic partner** and substantially equivalent legal unions from jurisdictions as defined in Washington state statute. Individuals in a state-registered domestic partnership are treated the same as a legal spouse except when in conflict with federal law.
- **Children**, through the last day of the month in which their 26th birthday occurred regardless of marital status, student status, or eligibility for coverage under another plan. It also includes children age 26 or older with a disability as described below in "Children of any age with a developmental or physical disability." Children are defined as the subscriber's:
  - **Children based on establishment of a parent-child relationship**, as described in Washington state statutes, except when parental rights have been terminated.
  - **Children of the subscriber's spouse**, based on the spouse's establishment of a parent-child relationship, except when parental rights have been terminated. The stepchild's relationship to the subscriber (and eligibility as a dependent) ends on the same date the marriage with the spouse ends through divorce, annulment, dissolution, termination, or death.
  - **Children for whom the subscriber has assumed a legal obligation** for total or partial support in anticipation of adoption of the child.
  - **Children of the subscriber's state-registered domestic partner,** based on the state-registered domestic partner's establishment of a parent-child relationship, except when parental rights have been terminated. The child's relationship to the subscriber (and eligibility as a dependent) ends on the same date the subscriber's legal relationship with the state-registered domestic partner ends through divorce, annulment, dissolution, termination, or death.
  - **Children specified in a court order or divorce decree** for whom the subscriber has a legal obligation to provide support or health care coverage.
  - **Extended dependents in the legal custody or legal guardianship of the subscriber, the subscriber's spouse, or the subscriber's state-registered domestic partner.** The legal responsibility is demonstrated by a valid court order and the child's official residence with the custodian or guardian. Extended dependent child does not include foster children unless the subscriber, the subscriber's spouse, or the subscriber's state-registered domestic partner has assumed a legal obligation for total or partial support in anticipation of adoption.
  - **Children of any age with a developmental or physical disability** that renders them incapable of self-sustaining employment and chiefly dependent upon the subscriber for support and maintenance, provided such condition occurs before the age of 26. The following requirements apply to a dependent child with a disability:
    - The subscriber must provide proof of the disability and dependency within 60 days of the child's attainment of age 26.
    - The subscriber must notify the PEBB Program in writing when the child is no longer eligible under this subsection.
    - A child with a developmental or physical disability who becomes self-supporting is not eligible as of the last day of the month in which they become capable of self-support.
    - A child with a developmental or physical disability age 26 and older who becomes capable of self-support does not regain eligibility if they later become incapable of self-support.

- The PEBB Program, with input from the medical plan, will periodically verify the eligibility of a dependent child with a disability beginning at age 26, but no more frequently than annually after the two-year period following the child's 26th birthday. Verification will require renewed proof of disability and dependence from the subscriber.

**A retiree, a survivor, or their enrolled dependents are required to enroll in Medicare Part A and Part B, if eligible.** This is a requirement for enrollment in a PEBB retiree health plan. A subscriber must provide a copy of their or their dependent's Medicare card or Medicare benefit verification letter with Medicare Part A and Part B effective dates by uploading it online using Benefits 24/7, the PEBB Program's online enrollment system, or providing it to the PEBB Program. If a subscriber or their dependent is not enrolled in either Medicare Part A or Part B on their 65th birthday, the subscriber must upload or provide the PEBB Program with a copy of the denial letter from the Social Security Administration. The only exception to this rule is for an employee or school employee who retired on or before July 1, 1991.

# Enrollment for subscribers and dependents

## Deferring enrollment

A retiring employee, a retiring school employee, or a dependent becoming eligible as a survivor may defer (postpone) enrollment in PEBB retiree insurance coverage if they meet the substantive eligibility requirements to enroll and also meet the procedural requirement by electing to defer enrollment using Benefits 24/7 or by submitting a *PEBB Retiree Election Form (form A)* to the PEBB Program within the enrollment timelines.

If a retiring employee, a retiring school employee, or a dependent becoming eligible as a survivor chooses to defer enrollment in PEBB medical, enrollment in PEBB dental and PEBB vision will also be deferred. Deferring enrollment in PEBB retiree insurance coverage will terminate enrollment for all eligible dependents, except as described below.

A retiring employee, a retiring school employee, or a dependent becoming eligible as a survivor who does not enroll in PEBB retiree insurance coverage is only eligible to enroll later if they have deferred enrollment. Enrollment may be deferred as follows:

- Beginning January 1, 2001, enrollment in PEBB retiree insurance coverage may be deferred when the subscriber is enrolled in employer-based group medical insurance as an employee or the dependent of an employee, or such medical insurance continued under Consolidated Omnibus Budget Reconciliation Act (COBRA) coverage or continuation coverage.
- Beginning January 1, 2001, enrollment in PEBB retiree insurance coverage may be deferred when the subscriber is enrolled as a retiree or the dependent of a retiree in a federal retiree medical plan.
- Beginning January 1, 2006, enrollment in PEBB retiree insurance coverage may be deferred when the subscriber is enrolled in Medicare Parts A and B and a Medicaid program that provides creditable coverage. Eligible dependents who are not enrolled in Medicaid coverage that provides creditable coverage may be enrolled.
- Beginning January 1, 2014, subscribers who are not eligible for Medicare Part A and Part B may defer enrollment in PEBB retiree insurance coverage when the subscriber is enrolled in qualified health plan coverage through a health benefit exchange developed under the Affordable Care Act.
- Beginning July 17, 2018, enrollment in PEBB retiree insurance coverage may be deferred when the subscriber is enrolled in the Civilian Health and Medical Program of the Department of Veterans Affairs (CHAMPVA).
- Beginning January 1, 2025, subscribers who are enrolled in Medicare may defer enrollment in PEBB retiree insurance coverage when they permanently reside outside of the United States.

- Beginning January 1, 2026, subscribers who are enrolled in Medicare may defer enrollment in PEBB retiree insurance coverage when they have a temporary absence from the United States that exceeds the permitted time allowed under the federal regulations and guidance of the Centers for Medicare and Medicaid Services (CMS).

**Exception:** A retiree may defer enrollment in PEBB retiree insurance coverage during the period of time they are enrolled as a dependent in a medical plan sponsored by PEBB or School Employees Benefits Board (SEBB), including such coverage under COBRA or continuation coverage. They do not need to elect to defer enrollment online or submit a *PEBB Retiree Election Form*.

The PEBB Program will automatically defer enrollment in PEBB retiree insurance coverage:

- If a retiree or a survivor becomes eligible for the employer contribution toward PEBB or SEBB benefits.
- If a retiree or a survivor who is not on Medicare and permanently resides outside of the United States, becomes eligible for Medicare Parts A and B, and does not respond to HCA within the required timeframe.
- If a retiree or a survivor must be disenrolled by a Medicare Advantage plan, Medicare Advantage Prescription Drug (MAPD) plan, or a Medicare Part D plan as required by federal law, due to permanently residing outside of the United States or due to a temporary absence from the United States that exceeds the permitted time allowed under the federal regulations and guidance of CMS.

A retiree or a survivor whose enrollment in PEBB retiree insurance coverage is deferred, may enroll as described in the section titled "Enrollment following deferral."

# Retiree and survivor enrollment

**An eligible retiree, a survivor, or their dependent** can enroll in only one PEBB medical plan, even if eligibility criteria is met under two or more subscribers.

**An eligible retiring employee or a retiring school employee** must enroll using Benefits 24/7 or by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program. The online enrollment must be completed or the forms must be received no later than 60 days after the employee's or the school employee's own employer-paid coverage, COBRA coverage, or continuation coverage ends. The first premium payment and applicable premium surcharges are due to HCA no later than 45 days after the election period ends.

**An eligible elected or full-time appointed official** must enroll using Benefits 24/7 or by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program. The online enrollment must be completed or the forms must be received no later than 60 days after the official leaves public office. The first premium payment and applicable premium surcharges are due to HCA no later than 45 days after the election period ends.

**An eligible survivor of a retiree** who is enrolled at the time of the retiree's death will be enrolled under their own account with no gap in coverage in the same PEBB health plan coverage they were enrolled in. To make changes to their PEBB health plan coverage, they must submit a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program. They must be received no later than 60 days after the death of the retiree. An eligible survivor of a retiree who is not enrolled at the time of the retiree's death, must enroll by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program. They must be received no later than 60 days after the death of the retiree.

**An eligible survivor of an employee or school employee** must enroll by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program. They must be received no later than 60 days after the later of the date of the employee's or the school employee's death, or the date the survivor's PEBB insurance coverage or SEBB insurance coverage

ends. The first premium payment and applicable premium surcharges are due to HCA no later than 45 days after the election period ends.

**An eligible employee or school employee determined to be retroactively eligible for disability retirement** must enroll using Benefits 24/7 or by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms, supporting documents, and their formal determination letter to the PEBB Program. The online enrollment must be completed or the forms must be received no later than 60 days after the date on the determination letter. The first premium payment and applicable premium surcharges are due to HCA no later than 45 days after the election period ends.

**An eligible survivor of an emergency service personnel killed in the line of duty** must enroll by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program. They must be received no later than 180 days after the later of:

- The date on the letter from the Department of Retirement Systems or the Board for Volunteer Firefighters and Reserve Officers that informs the survivor that they are determined to be an eligible survivor;
- The date of the emergency service worker's death; or
- The last day the survivor was covered under any health plan through the emergency service worker's employer or COBRA coverage from the emergency service worker's employer.

**A retiree or a survivor whose enrollment is deferred** and is enrolling in a PEBB retiree health plan must enroll using Benefits 24/7 or by submitting a *PEBB Retiree Election Form (form A)* along with any other required forms and supporting documents to the PEBB Program.

A retiree or a survivor whose enrollment is deferred while enrolled in other qualifying coverage must also submit evidence of continuous enrollment. The online enrollment must be completed or the forms must be received no later than 60 days after a loss of other qualifying coverage.

A retiree or a survivor enrolled in Medicare whose enrollment is deferred while permanently residing outside of the United States or due to a temporary absence from the United States and who returns to the United States, must also submit proof of enrollment in Medicare Parts A and B, proof of permanent residence in the United States, and proof they currently reside in the United States; evidence of continuous enrollment in qualified coverage is not required. The online enrollment must be completed or the forms must be received no later than 60 days after the date the permanent move begins or 60 days following the month of the permanent move, whichever is later.

The first premium payment and applicable premium surcharges are due to HCA no later than 45 days after the election period ends.

See "Enrollment following deferral" for additional information and enrollment timelines.

# Dependent enrollment

To enroll an eligible dependent, the subscriber must include the dependent's information using Benefits 24/7 or on the applicable enrollment form and provide the required document(s) as proof of the dependent's eligibility. The dependent will not be enrolled in PEBB health plan coverage if the PEBB Program is unable to verify their eligibility within the PEBB Program enrollment timelines.

Dependents who are enrolled in PEBB medical must be enrolled in the same PEBB medical plan as the retiree or survivor.

A retiree or a survivor may also enroll an eligible dependent during the PEBB Program's annual open enrollment or when a specific life event creates a special open enrollment. See "Making changes."

# Medicare eligibility and enrollment

## Medicare Part A and Part B

**If a subscriber or their enrolled dependent** becomes eligible for Medicare, they should contact the Social Security Administration to ask about Medicare enrollment.

**Any enrollee who is eligible for Medicare must enroll and stay enrolled in Medicare Part A and Part B.**

In most cases, Medicare will become the primary insurance coverage and the PEBB retiree medical plan will become the secondary insurance coverage.

**A subscriber** must provide a copy of their or their dependent's Medicare card or Medicare benefit verification letter from the Social Security Administration with effective dates by uploading it online using Benefits 24/7 or providing it to the PEBB Program. If a subscriber or their dependent is not enrolled in either Medicare Part A or Part B on their 65th birthday, the subscriber must upload or provide a copy of the denial letter from the Social Security Administration. If this procedural requirement is not met, eligibility will end as described in the termination notice sent by the PEBB Program. The only exception to this rule is for an employee or school employee who retired on or before July 1, 1991.

## Medicare Part D

The PEBB Program has determined that this medical plan has prescription drug coverage that is, on average, as good as or better than the standard Medicare Part D prescription drug coverage (it is "creditable coverage"). Therefore, a subscriber or their enrolled dependent cannot enroll in a Medicare Part D plan and stay in this medical plan.

If the subscriber terminates this medical plan, they may contact the PEBB Program to request a certificate of creditable coverage. If creditable prescription drug coverage is not maintained, Medicare Part D premiums may be higher in the future.

If a subscriber, or their enrolled dependent chooses to enroll in a stand-alone Medicare Part D plan, PEBB retiree insurance coverage may only be continued by enrolling in the PEBB-sponsored Medicare supplement plan.

# When medical coverage begins

**For an eligible retiring employee or retiring school employee** and their eligible dependents, medical coverage begins the first day of the month after the retiring employee's or retiring school employee's own employer-paid coverage, COBRA coverage, or continuation coverage ends.

**For an eligible employee or school employee determined to be retroactively eligible for disability retirement** and their eligible dependents, medical coverage begins on the date chosen by the employee or school employee as allowed under PEBB Program rules.

**For an eligible elected or full-time appointed official** and their eligible dependents, medical coverage begins the first day of the month following the date the official leaves public office.

**For an eligible survivor of a retiree** and their eligible dependents, medical coverage will be continued without a gap, subject to payment of premiums and applicable premium surcharges. If the eligible survivor is not enrolled at the time of the retiree's death, medical coverage will begin the first day of the month following the retiree's death.

**For an eligible survivor of an employee or school employee** and their eligible dependents, medical coverage begins the first day of the month following the later of the date of the employee's or the school employee's death or the date the survivor's PEBB insurance coverage or SEBB insurance coverage ends. This does not include emergency service personnel killed in the line of duty.

**For an eligible survivor of an emergency service personnel killed in the line of duty** and their eligible dependents, medical coverage begins on the date chosen, as allowed under PEBB Program rules.

**For a retiree or a survivor whose enrollment is deferred** and is enrolling in a PEBB retiree health plan following loss of other qualifying coverage, medical coverage for the retiree or the survivor and their eligible dependents begins the first day of the month after the loss of the other qualifying coverage. For a retiree or a survivor enrolled in Medicare whose enrollment was deferred while permanently residing outside of the United States or due to a temporary absence from the United States, and who returns to the United States, medical coverage for the retiree or the survivor and their eligible dependents begins the first day of the month after the permanent move.

**For a retiree, a survivor, or their eligible dependents enrolling during the PEBB Program's annual open enrollment,** medical coverage begins January 1 of the following year.

**For a retiree, a survivor, or their eligible dependents enrolling during a special open enrollment,** medical coverage begins the first day of the month following the event date or the date the online enrollment election using Benefits 24/7 or the required form is received, whichever is later. If that day is the first of the month, medical coverage begins on that day.

If the special open enrollment is **due to the birth or adoption of a child,** or when the subscriber has assumed a legal obligation for total or partial support in anticipation of adoption of a child, medical coverage will begin as follows:

- **For a newly born child,** medical coverage will begin the date of birth;
- **For a newly adopted child,** medical coverage will begin on the date of placement or the date a legal obligation is assumed in anticipation of adoption, whichever is earlier; or
- **For a spouse or state-registered domestic partner** of a subscriber, medical coverage will begin the first day of the month in which the event occurs.

If the special open enrollment is due to the enrollment of an **extended dependent or a dependent child with a disability**, medical coverage will begin the first day of the month following the later of the event date or eligibility certification, whichever is later.

# Making changes

## Removing a dependent who is no longer eligible

A subscriber must provide notice to remove a dependent who is no longer eligible due to divorce, annulment, dissolution, or a qualifying event of a dependent ceasing to be eligible as a dependent child, as described under "Dependent eligibility." The notice must be received online using Benefits 24/7 or by written request to the PEBB Program within 60 days of the last day of the month the dependent no longer meets the eligibility criteria.

Consequences for not submitting notice within the required 60 days may include, but are not limited to:

- The dependent may lose eligibility to continue PEBB medical under one of the continuation coverage options described in "Options for continuing PEBB medical coverage."
- The subscriber may be billed for claims paid by the medical plan for services that were rendered after the dependent lost eligibility.
- The subscriber may not be able to recover subscriber-paid insurance premiums for the dependent that lost eligibility.
- The subscriber may be responsible for premiums paid by the state for the dependent's medical plan coverage after the dependent lost eligibility.

# Voluntary termination

An enrolled retiree or survivor may voluntarily terminate enrollment in a medical plan at any time by submitting a request online using Benefits 24/7 or in writing to the PEBB Program. Enrollment in the medical plan will be terminated the last day of the month in which the request is received online or by the PEBB Program, or on the last day of the month specified in the termination request, whichever is later. If the request is received on the first day of the month, medical plan enrollment will be terminated on the last day of the previous month.

A retiree or a survivor who voluntarily terminates their enrollment in a medical plan also terminates all other health plan enrollment and enrollment for all eligible dependents. Once coverage is terminated, a retiree or a survivor may not enroll again in the future unless they reestablish eligibility for PEBB insurance coverage by becoming newly eligible.

# Deferring enrollment

An enrolled retiree or survivor may defer enrollment in PEBB retiree insurance coverage at any time by submitting the request online using Benefits 24/7 or the *PEBB Retiree Change Form (form E),* along with any other required forms and supporting documents, to the PEBB Program. Enrollment in PEBB retiree insurance coverage will be deferred effective the first day of the month following the date the request is received online or by the PEBB Program. If the request is received on the first day of the month, enrollment will be deferred effective that day. A retiree or a survivor who deferred their enrollment may enroll as described in "Enrollment following deferral."

# Enrollment following deferral

A retiree or a survivor whose enrollment in PEBB retiree insurance coverage is deferred:

- **While enrolled in employer-based group medical or such medical coverage continued under COBRA coverage or continuation coverage** may enroll in a PEBB medical plan during the PEBB Program's annual open enrollment period, or no later than 60 days after the date their enrollment in employer-based group medical coverage or such coverage under COBRA coverage or continuation coverage ends.
- **While enrolled in a federal retiree medical plan as a retiree or dependent** will have a one-time opportunity to enroll in a PEBB medical plan during the PEBB Program's annual open enrollment period, or no later than 60 days after their enrollment in a federal retiree medical plan ends.
- **While enrolled in Medicare Parts A and B and a Medicaid program that provides creditable coverage** may enroll in a PEBB medical plan during the PEBB Program's annual open enrollment period, or no later than 60 days after their Medicaid coverage ends, or no later than the end of the calendar year when their Medicaid coverage ends if they were also enrolled in a subsidized Medicare Part D plan.
- **While enrolled in qualified health plan coverage through a health benefit exchange** developed under the Affordable Care Act will have a one-time opportunity to enroll or reenroll in a PEBB medical plan during the PEBB Program's annual open enrollment period, or no later than 60 days after exchange coverage ends.
- **While enrolled in CHAMPVA** will have a one-time opportunity to enroll in a PEBB medical plan during the PEBB Program's annual open enrollment period, or no later than 60 days after their enrollment in a CHAMPVA medical plan ends.
- **While enrolled in Medicare and permanently residing outside of the United States, or due to a temporary absence from the United States,** and who returns to the United States, may enroll in a PEBB medical plan no later than 60 days after the date the permanent move begins or 60 days following the month of the permanent move, whichever is later.

- **While enrolled as a dependent** in a medical plan sponsored by PEBB or SEBB, including coverage under COBRA or continuation coverage, may enroll in a PEBB medical plan during the PEBB Program's annual open enrollment period, or no later than 60 days after the enrollment in a medical plan sponsored by PEBB or SEBB ends, or such coverage under COBRA or continuation coverage ends. The evidence of continuous enrollment required to enroll as described below may include a medical plan sponsored by a Washington state educational service district if enrollment was deferred prior to January 1, 2024.

For a retiree or a survivor to enroll in a PEBB medical plan, the enrollment must be completed online using Benefits 24/7 or the PEBB Program must receive a *PEBB Retiree Election Form (form A)* and any other required forms and supporting documents during the timelines described in this section.

A retiree or a survivor whose enrollment is deferred while enrolled in other qualifying coverage, must also submit evidence of continuous enrollment in one or more qualifying coverages. A gap in coverage of 31 days or less is allowed between the date PEBB retiree insurance coverage is deferred and the start date of a qualifying coverage, and between each period of enrollment in qualifying coverages during the deferral period.

A retiree or a survivor enrolled in Medicare whose enrollment is deferred while permanently residing outside of the United States or due to a temporary absence from the United States, and who returns to the United States, must submit proof of enrollment in Medicare Parts A and B, proof of permanent residence in the United States, and proof they currently reside in the United States; evidence of continuous enrollment in qualified coverage is not required while the retiree or survivor enrolled in Medicare lives outside of the United States.

A retiree or a survivor whose enrollment in PEBB retiree insurance coverage is deferred as described above may also enroll in a PEBB medical plan if they receive formal notice that HCA has determined it is more cost-effective to enroll in a PEBB medical plan than a medical assistance program.

A retiree or a survivor should contact the PEBB Program or visit **hca.wa.gov/pebb-retirees** to get the required forms, information on premiums, and a list of available medical plans.

# Making changes during annual open enrollment and special open enrollment

A subscriber may make certain changes to their enrollment during the annual open enrollment or when a specific life event creates a special open enrollment period.

## Annual open enrollment changes

A subscriber may make the following changes to their enrollment during the PEBB Program's annual open enrollment period:

- Enroll in a medical plan following a deferral
- Defer or terminate their enrollment in a medical plan
- Enroll or remove eligible dependents
- Change their medical plan

A subscriber must submit the election change online using Benefits 24/7 or submit the required *PEBB Retiree Open Enrollment Election/Change Form (form A-OE)* along with any other required forms, and any supporting documents to the PEBB Program. The change must be completed online, or the forms must be received, no later than the last day of the annual open enrollment period. The change will be effective January 1 of the following year, except when a subscriber chooses to terminate enrollment or remove an eligible dependent, then the change will be effective on December 31.

## Special open enrollment changes

A subscriber may change their enrollment outside of the annual open enrollment period if a qualifying event creates a special open enrollment period. However, the change in enrollment must be allowable under Internal Revenue Code (IRC) and Treasury Regulations and correspond to and be consistent with the event that creates the special open enrollment for the subscriber, their dependent, or both.

To make an enrollment change, the subscriber must submit the change online using Benefits 24/7 or submit the required *PEBB Retiree Change Form (form E)* along with any other required forms to the PEBB Program. The change must be completed online, or the PEBB Program must receive the forms no later than 60 days after the event that creates the special open enrollment. In many instances, the date the change is received online or the date the form is received affects the effective date of the enrollment or change in enrollment. Submitting the requested change sooner may avoid a delay in the enrollment or change. In addition, the PEBB Program will require the subscriber to provide proof of the dependent's eligibility, evidence of the event that created the special open enrollment, or both.

**Exceptions:**

- A subscriber enrolled in PEBB retiree insurance coverage has six months from the date of their or their dependent's enrollment in Medicare Part B to change their enrollment to a PEBB Medicare Supplement Plan. The change must be made online or the PEBB Program must receive the required form(s) no later than six months after the enrollment in Medicare Part B for either the subscriber or their dependent.
- A subscriber enrolled in PEBB retiree insurance coverage has seven months to enroll in a Medicare Advantage Prescription Drug (MAPD) plan or Uniform Medical Plan (UMP) Classic Medicare with Part D (PDP) that begins three months before they or their dependent first enrolled in both Medicare Part A and Part B and ends three months after the month of Medicare eligibility. A subscriber may also enroll themselves or their dependent in a MAPD plan or UMP Classic Medicare with Part D (PDP) before their last day of the Medicare Part B initial enrollment period. The change must be made online using Benefits 24/7 or the forms must be received by the PEBB Program, no later than the last day of the month prior to the month the subscriber or their dependent enrolls in the MAPD plan or UMP Classic Medicare with Part D (PDP).
- A subscriber enrolled in PEBB retiree insurance coverage has three months to enroll in a MAPD plan, Medicare Supplement plan, or the UMP Classic Medicare with Part D (PDP), after they or their dependent gain, lose, or have a change in low-income subsidy (LIS) eligibility, or notification of such a change, whichever is later. The change must be made online using Benefits 24/7 or the forms must be received by the PEBB Program, no later than three months after they or their dependent gain, lose, or have a change in LIS eligibility, or notification of such a change, whichever is later.
- If a subscriber wants to enroll a newborn or child whom the subscriber has adopted or has assumed a legal obligation for total or partial support in anticipation of adoption, the subscriber should complete the request online or notify the PEBB Program by submitting the required form(s) as soon as possible to ensure timely payment of claims. If adding the child increases the premium, the request must be received online, or the required form(s) must be received no later than 60 days after the date of the birth, adoption, or the date the legal obligation is assumed for total or partial support in anticipation of adoption.

## Special open enrollment events that allow for a change in health plans

A subscriber may not change their health plan during a special open enrollment if their state-registered domestic partner or state-registered domestic partner's child is not a tax dependent.

Any of the following events may create a special open enrollment:

- Subscriber gains a new dependent due to:
  - Marriage or registering a state-registered domestic partnership.

  - Birth, adoption, or assuming a legal obligation for total or partial support in anticipation of adoption.
  - A child becoming eligible as an extended dependent through legal custody or legal guardianship.
- Subscriber or their dependent loses other coverage under a group health plan or through health insurance coverage, as defined by the Health Insurance Portability and Accountability Act (HIPAA).
- Subscriber has a change in employment status that affects their eligibility for the employer contribution toward their employer-based group health plan.
- Subscriber's dependent has a change in their own employment status that affects their eligibility or their dependent's eligibility for the employer contribution under their employer-based group health plan. "Employer contribution" means contributions made by the dependent's current or former employer toward health coverage as described in the Treasury Regulation.
- Subscriber or their dependent has a change in residence that affects health plan availability. If the subscriber has a change in residence and their current medical plan is no longer available, the subscriber must select a new medical plan. If the subscriber does not elect a new medical plan as required, they will be enrolled in a PEBB medical plan designated by the director of HCA or their designee.
- A court order requires the subscriber or any other individual to provide insurance coverage for an eligible dependent of the subscriber (a former spouse or former state-registered domestic partner is not an eligible dependent).
- Subscriber or their dependent enrolls in coverage under Medicaid or a state Children's Health Insurance Program (CHIP), or the subscriber or their dependent loses eligibility for coverage under Medicaid or CHIP.
- Subscriber or their dependent becomes eligible for state premium assistance subsidy for PEBB health plan coverage from Medicaid or CHIP.
- Subscriber or their dependent enrolls in coverage under Medicare, or the subscriber or their dependent loses eligibility for coverage under Medicare or enrolls in or terminates enrollment in a Medicare Advantage Prescription Drug or a Part D plan. If the subscriber's current medical plan becomes unavailable due to the subscriber or their dependent's enrollment in Medicare, the subscriber must select a new medical plan.
- Subscriber or their dependent gains, loses, or has a change in low-income subsidy eligibility.
- Subscriber or their dependent's current medical plan becomes unavailable because the subscriber or enrolled dependent is no longer eligible for a health savings account (HSA).
- Subscriber or their dependent experiences a disruption of care for active and ongoing treatment that could function as a reduction in benefits for the subscriber or their dependent. The subscriber may not change their health plan election because the subscriber or dependent's physician stops participation with the subscriber's health plan unless the PEBB Program determines that a continuity of care issue exists. The PEBB Program will consider but not limit its consideration to the following:
  - Active cancer treatment, such as chemotherapy or radiation therapy
  - Treatment following a recent organ transplant
  - A scheduled surgery
  - Recent major surgery still within the postoperative period
  - Treatment for a high-risk pregnancy

- Subscriber or their dependent becomes eligible and enrolls in a TRICARE plan or loses eligibility for a TRICARE plan.
- The PEBB Program determines that there has been a substantial decrease in the providers available under a PEBB medical plan.

**Note:** The plan cannot guarantee that any physician, hospital, or other provider will be available or remain under contract with the plan. An enrollee may not change medical plans simply because their provider or health care facility discontinues participation with this medical plan until the PEBB Program's next annual open enrollment or when another qualifying event creates a special open enrollment for changing health plans, unless the PEBB Program determines that a continuity of care issue exists or there has been a substantial decrease in the providers available under the plan.

## Special open enrollment events that allow adding or removing a dependent

Any of the following events may create a special open enrollment:

- Subscriber gains a new dependent due to:
  - Marriage or registering a state-registered domestic partnership.
  - Birth, adoption, or when the subscriber has assumed a legal obligation for total or partial support in anticipation of adoption.
  - A child becoming eligible as an extended dependent through legal custody or legal guardianship.
- Subscriber or their dependent loses other coverage under a group health plan or through health insurance coverage, as defined by the Health Insurance Portability and Accountability Act (HIPAA).
- Subscriber has a change in employment status that affects their eligibility for the employer contribution toward their employer-based group health plan.
- Subscriber's dependent has a change in their own employment status that affects their eligibility or their dependent's eligibility for the employer contribution under their employer-based group health plan. "Employer contribution" means contributions made by the dependent's current or former employer toward health coverage as described in the Treasury Regulation.
- Subscriber or their dependent has a change in enrollment under an employer-based group health plan during its annual open enrollment that does not align with the PEBB Program's annual open enrollment.
- Subscriber's dependent has a change in residence from outside of the United States to within the United States, or from within the United States to outside of the United States and that change in residence resulted in the dependent losing their health insurance.
- A court order requires the subscriber or any other individual to provide insurance coverage for an eligible dependent of the subscriber (a former spouse or former state-registered domestic partner is not an eligible dependent).
- Subscriber or their dependent enrolls in coverage under Medicaid or a state Children's Health Insurance Program (CHIP) or the subscriber or their dependent loses eligibility for coverage under Medicaid or CHIP.
- Subscriber or their dependent becomes eligible for a state premium assistance subsidy for PEBB health plan coverage from Medicaid or CHIP.
- Subscriber's dependent enrolls in Medicare or loses eligibility for Medicare.
- Subscriber or their dependent enrolled in a Medicare Advantage Prescription Drug plan, Medicare Supplement plan, or the Uniform Medical Plan Classic Medicare with Part D (PDP) who gains, loses, or has a change to their low-income subsidy eligibility.

- Subscriber or their dependent becomes eligible and enrolls in a TRICARE plan or loses eligibility for a TRICARE plan.

# When medical coverage ends

## Termination dates

Medical coverage ends on the following dates:

- On the last day of the month when any enrollee ceases to be eligible.
- On the date a medical plan terminates due to a change in contracted service area or when the group policy ends. If that should occur, the subscriber will have the opportunity to enroll in another PEBB medical plan.
- On the last day of the month in which the monthly premium and applicable premium surcharges were paid.
- The first day of the month following the date the election is received online using Benefits 24/7 or the required forms are received by the PEBB Program when an enrolled retiree or survivor requests to defer enrollment in PEBB retiree insurance coverage. If the election is received on the first day of the month, enrollment will be deferred effective that day.
- The last day of the month in which the request is received online using Benefits 24/7 or the PEBB Program receives a written request and all required forms requesting to voluntarily terminate enrollment in a medical plan. If a future date is specified, medical coverage terminates the last day of the month specified. If the termination request is received on the first day of the month, medical plan enrollment will be terminated on the last day of the previous month.

A subscriber will be responsible for payment of any services received after the date medical coverage ends, as described above.

## Final premium payments

The subscriber is responsible for timely payment of premiums and applicable premium surcharges.

Premium payments and applicable premium surcharges are not prorated during any month, for any reason, even if an enrollee dies or asks to terminate their medical plan before the end of the month.

An exception occurs when an enrolled retiree dies, in which case, HCA will waive the premium payment for medical, dental, vision, and any applicable premium surcharges for the retiree for the month in which the death occurred.

If the monthly premium or applicable premium surcharges remain unpaid for 30 days, it will be considered delinquent. A subscriber is allowed a grace period of 30 days from the date the monthly premium or applicable premium surcharges become delinquent to pay the unpaid premium balance or applicable premium surcharges. If the subscriber's premium balance or applicable premium surcharges remain unpaid for 60 days from the original due date, coverage will be terminated retroactive to the last day of the month for which the monthly premium and any applicable premium surcharges were paid. When a subscriber is terminated for non-payment of premiums and applicable premium surcharges, they may not enroll again unless they regain eligibility.

## If an enrollee is hospitalized

An enrollee who is receiving covered services in a hospital on the date medical coverage ends will continue to be eligible for covered services while an inpatient for the condition which the enrollee was hospitalized, until one of the following events occur:

- According to this plan's clinical criteria, it is no longer medically necessary for the enrollee to be an inpatient at the facility.
- The remaining benefits available for the hospitalization are exhausted, regardless of whether a new calendar year begins.
- The enrollee becomes covered under another agreement with a group health plan that provides benefits for the hospitalization.
- The enrollee becomes enrolled under an agreement with another carrier that provides benefits for the hospitalization.

This provision will not apply if the enrollee is covered under another agreement that provides benefits for the hospitalization at the time medical coverage ends, except as set forth in this section, or if the enrollee is eligible for PEBB Continuation Coverage as described in "Options for continuing PEBB medical coverage."

## Options for continuing PEBB medical coverage

A subscriber and their dependents covered by this medical plan may be eligible to continue enrollment under PEBB Continuation Coverage (COBRA) if they lose eligibility. PEBB Continuation Coverage (COBRA) temporarily extends group insurance coverage if certain circumstances occur that would otherwise end the subscriber's or their dependent's PEBB medical coverage. PEBB Continuation Coverage (COBRA) includes eligibility and administrative requirements under federal COBRA laws and regulations and also includes coverage for some enrollees who are not qualified beneficiaries under federal COBRA continuation coverage. Refer to the *PEBB Continuation Coverage Election Notice* for details.

A subscriber and their dependents covered by this medical plan who lose eligibility for PEBB retiree insurance coverage when their employer group ends participation with the Health Care Authority, may be eligible to continue their enrollment under PEBB Continuation Coverage (Employer Group Ended Participation).

The PEBB Program administers these coverages. Call the PEBB Program at 1-800-200-1004 (TRS: 711) for details.

## Options for continuing coverage under PEBB retiree insurance coverage

A dependent becoming eligible as a survivor of a retiree is eligible to continue enrollment or defer (postpone) enrollment in PEBB retiree insurance coverage if they meet procedural and substantive eligibility requirements. See the *PEBB Retiree Enrollment Guide* for details.

### Availability of other coverage

An enrollee (including a dependent of a subscriber terminated for cause) has the right to switch from PEBB group medical to an individual plan when they are no longer eligible to continue the PEBB group medical plan and are not eligible for Medicare or covered under another group insurance coverage that provides benefits for hospital or medical care.

To receive assistance in finding an individual plan that will meet your needs, call us at 1-833-458-0656.

# General provisions for eligibility and enrollment

## Termination for just cause

The purpose of this provision is to allow for a fair and consistent method to process the plan-designated provider's request to terminate an enrollee's coverage from this plan for just cause.

A retiree or an eligible dependent may have coverage terminated by HCA for the following reasons:

- Failure to comply with the PEBB Program's procedural requirements, including failure to provide information or documentation requested by the due date in written requests from the PEBB Program
- Knowingly providing false information
- Failure to pay the monthly premium and applicable premium surcharges when due
- Misconduct. Examples of such termination include, but are not limited to the following:
  - Fraud, intentional misrepresentation or withholding of information the subscriber knew or should have known was material or necessary to accurately determine eligibility or the correct premium
  - Abusive or threatening conduct repeatedly directed to an HCA employee, a health plan, or other HCA-contracted vendor providing PEBB insurance coverage on behalf of HCA, its employees, or other persons

If a retiree's PEBB insurance coverage is terminated by HCA for the above reasons, PEBB insurance coverage for all of the retiree's eligible dependents is also terminated.

# Appeal rights

**Any enrollee** may appeal a decision made by the PEBB Program regarding PEBB eligibility, enrollment, premium payments, or premium surcharges to the PEBB Appeals Unit.

Learn more at **hca.wa.gov/pebb-appeals**.

**Fax:** 360-763-4709

**Mail:** Health Care Authority
Attn: PEBB Appeals Unit
PO Box 45504
Olympia, WA 98504-5504

**Hand deliver:** Health Care Authority
626 8th Avenue SE
Olympia, WA 98501

**Any enrollee** may appeal a decision regarding the administration of a PEBB medical plan by following the appeal provisions of the plan, except when regarding eligibility, enrollment, and premium payment decisions.

# Relationship to law and regulations

Any provision of this certificate of coverage that is in conflict with any governing law or regulation of Washington state is hereby amended to comply with the minimum requirements of such law or regulation.

# PEBB customer service

For questions about PEBB retiree eligibility and enrollment, please call the PEBB Program at 1-800-200-1004 (TRS:711) or visit **hca.wa.gov/pebb-retirees**.

For questions about Medicare, please call the Centers for Medicare and Medicaid Services (CMS) at 1-800-MEDICARE or visit **medicare.gov**.

# Definitions

## Allowed amount, medical services

**Allowed amount for medical services** is the most the plan pays for a specific covered service or supply. The allowed amount is determined as follows:

- **For preferred providers** that are within the Regence BlueShield service area, the Preferred Network contract with Regence BlueShield is the relevant contract that determines the allowed amount. For preferred providers that are outside the Regence BlueShield service area, the contract with another Blue Cross or Blue Shield organization in the BlueCard Program for its PPO network is the relevant contract that determines the allowed amount.
- **For participating providers** that are within the Regence BlueShield service area, the participating provider contract with Regence BlueShield is the relevant contract that determines the allowed amount. For participating providers that are outside the Regence BlueShield service area, the contract with another Blue Cross or Blue Shield organization in the BlueCard Program is the relevant contract that determines the allowed amount.
- **For out-of-network providers who are within the Regence BlueShield service area and not contracted with Regence BlueShield,** the amount Regence has determined to be reasonable charges for covered services and supplies is the allowed amount.

  The allowed amount may be based upon the billed charges for some services, as determined by Regence or as otherwise required by law. If a provider is not in the network, as described above, but has a contract with Regence, the allowed amount is based on the negotiated rate.
- **For out-of-network providers accessed through the BlueCard Program**, the allowed amount is the lower of the provider's billed charges and the amount that the other Blue plan identifies as the amount on which it would base a payment to that provider.

  Under the BlueCard Program, when you access covered services within the geographic area served by a Host Blue, Regence BlueShield will remain responsible for fulfilling contractual obligations. However, the Host Blue is responsible for contracting with and generally handling all interactions with its network providers.

Whenever you access covered services outside Regence BlueShield's service area and the claim is processed through the BlueCard Program, the amount you pay for covered services is calculated based on the lower of:

- The covered billed charges for your covered services; or
- The negotiated price that the Host Blue makes available to Regence.

Often, this negotiated price will be a simple discount that reflects an actual price that the Host Blue pays to your health care provider. Sometimes, it is an estimated price that considers special arrangements with your health care provider or provider group that may include types of settlements, incentive payments, and/or other credits or charges. Occasionally, it may be an average price, based on a discount that results in expected average savings for similar types of health care providers after considering the same types of transactions as with an estimated price.

Estimated pricing and average pricing, going forward, also consider adjustments to correct for over- or underestimation of modifications of past pricing for the types of transaction modifications noted above. However, such adjustments will not affect the price Regence uses for your claim because they will not be applied retroactively to claims already paid.

Laws in a small number of states may require the Host Blue to add a surcharge to your calculation. If any state laws mandate other liability calculation methods, including a surcharge, Regence BlueShield would then calculate your liability for any covered services according to applicable law.

Charges more than the allowed amount are not reimbursable. For questions regarding the basis for determination of the allowed amount, contact UMP Customer Service.

## Allowed amount, prescription drugs

The **allowed amount for prescription drugs** is based on ArrayRx's contractually agreed reimbursement with network pharmacies, unless other contractual arrangements or terms apply. All covered prescription drug claims at network pharmacies are paid based on this allowed amount.

## Ambulatory surgery center (ASC)

An **ambulatory surgery center (ASC)** is a health care facility that specializes in providing surgery, pain management, and certain diagnostic services in an outpatient setting. ASC-qualified procedures are typically more complex than those done in a doctor's office but not so complex as to require an overnight stay. Procedures commonly performed in these centers include colonoscopies, endoscopies, cataract surgery, orthopedic, and ENT (ear, nose, and throat) procedures. An ASC may also be known as an outpatient surgery center or same-day surgery center.

## Annual open enrollment

**Annual open enrollment** is a period of time defined by HCA when a subscriber may change to another health plan offered by the PEBB Program and make certain other account changes for an effective date beginning January 1 of the following year.

## Appeal

See the "Complaint and appeal procedures" section for an explanation of appeals and how the process works. For appeals related to PEBB Program eligibility or enrollment see "Appeal rights" in the "Eligibility and enrollment" section and in the "Eligibility and enrollment for a retiree or survivor" section for more information.

## Authorized representative

An **authorized representative** is someone you have designated in writing to communicate with the plan on your behalf. See page 121 for how this works.

## Balance billing

**Balance billing** is a provider billing you for the difference between the billed amount and the allowed amount. Preferred and participating providers cannot balance bill you for covered services above the allowed amount. See an example of how this works in the "Sample payments to different provider network status" section.

**In Washington State:**

When you receive nonemergency services at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

At certain out-of-network hospitals in Washington State, when you receive nonemergency inpatient or outpatient services you pay the out-of-network cost-sharing amount and cannot be balance billed.

Contact UMP Customer Service if you have any questions about the status of a hospital in Washington State.

At an out-of-network facility, when you receive emergency services you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

When you receive covered ground or air ambulance services in Washington State, you pay the network rate and cannot be balance billed regardless of the network status of the provider.

**Outside of Washington State:**

For nonemergency services performed at a network hospital, network hospital outpatient department, network critical access hospital, or network ambulatory surgical center outside of Washington State, you still pay the network rate, but in some states, an out-of-network provider may be allowed to ask you to waive some of your balance billing protections.

At an out-of-network facility, when you receive emergency services you pay the network cost-sharing amount regardless of the network status of the provider or facility and cannot be balance billed.

# Birthday rule

In some instances, the **birthday rule** is used to determine which group health plan will pay first for the dependent children of married, living together and not married, legally separated, or divorced parents. This rule looks at only the month and day, not the year, of the parents' birthdays. For example, the plan of a parent born April 14 is primary over the plan of a parent born August 21. If both parents have the same birthday, the plan that has covered either parent the longest is primary.

# Biosimilar

A **biosimilar**, or biosimilar drug, is a medicine approved by the US Food and Drug Administration that is highly similar in structure to and has no clinical difference to a reference biologic medicine.

# Brand-name drug

A **brand-name drug** is a prescription drug sold under the proprietary name or trade name selected by the manufacturer.

# Business day

**Business days** are Mondays through Fridays, except for legal holidays observed by Washington State.

# Calendar day

A **calendar day** is any day of the week regardless of whether it is observed as a legal holiday by Washington State.

# Calendar year

A **calendar year** is January 1 through December 31.

# Chronic migraine

A **chronic migraine** is having a headache on 15 or more days per month of which eight or more days are a migraine.

## Clinical review

**Clinical review** is when the plan has a clinical professional review medical records related to treatment to determine if treatment is medically necessary.

## Coinsurance

**Coinsurance** is the percentage of the allowed amount you must pay the provider on claims for which the plan pays less than 100 percent of the allowed amount. This includes most medical services and prescription drugs.

## Continuation Coverage

Continuation Coverage means the temporary continuation of PEBB benefits available to enrollees under the Consolidated Omnibus Budget Reconciliation Act (COBRA), the Uniformed Services Employment and Reemployment Rights Act (USERRA), or PEB Board policies.

## Coordination of benefits (COB)

For members covered by more than one group health plan, **coordination of benefits (COB)** is the method the plan uses to determine which plan pays first, which pays second, and the amount paid by each plan. See description and examples in the "If you have other HDHP coverage" section.

## Copay

**Copay** is a set dollar amount you pay when receiving specific services, treatments, or supplies, such as chiropractic, acupuncture, or massage therapy.

## Cost-share

**Cost-share** means the amount you pay for a service, supply, or prescription drug. This may be a deductible, coinsurance, copay, or amounts not covered by the plan.

## Custodial care

**Custodial care** is care primarily to assist in activities of daily living, including institutional care primarily to support self-care and provide room and board. Custodial care includes, but is not limited to, help in walking, getting into and out of bed, bathing, dressing, feeding and preparing special diets, and supervising prescription drugs that are ordinarily self-administered.

## Deductible

Your **deductible** is a fixed dollar amount you must meet each calendar year for health care expenses before the plan starts paying for covered services. For this plan, you must meet your deductible for medical and prescription drug expenses before the plan starts paying for covered medical services and covered prescription drugs. You pay the first $1,750 in health care expenses for one member enrolled; you pay the first $3,500 in health care expenses for two or more members enrolled. Only expenses covered by the plan apply to your deductible. For example, if you receive LASIK surgery (see exclusion on page 94), the plan does not apply this payment to your deductible. Some services are not subject to this deductible (see the "Summary of benefits" section). See the "What you pay for services" section for details on how your deductible works.

# Dependent

A **dependent** is an eligible spouse, state-registered domestic partner, child, or other eligible family member as described in "Dependent eligibility" (see the "Eligibility for subscribers and dependents" section) that is either covered by or eligible to be covered by the plan under the subscriber's account.

# Detoxification

**Detoxification** is a medically supervised treatment program for individuals with alcohol or drug intoxication, designed to rid the body of toxic substances and manage withdrawal symptoms.

# Developmental delay

**Developmental delay** is a significant lag in reaching developmental milestones as expected during infancy and early childhood. The cause may be present at birth or acquired after birth from a disease or disorder of the body, an injury, a disorder of the mind or emotions, or harmful effects of the surrounding environment. Only a physician or other provider may diagnose a developmental delay.

# Durable medical equipment (DME)

**Durable medical equipment (DME)** is:

- Designed for prolonged use.
- For a specific therapeutic or clinical purpose, or to assist in the treatment of an injury or illness.
- Medically necessary (meeting all plan medical necessity criteria).
- Primarily and customarily used only for a medical purpose.

See page 93 for examples of equipment that is not covered.

# Effectiveness

**Effectiveness** means the extent to which a specific intervention, procedure, service, level of service, supply, prescription drug, or drug dose may reasonably be expected to produce the intended results and to have expected benefits that outweigh potential harmful effects under real-world circumstances.

# Efficacy

**Efficacy** is the extent to which a specific intervention, procedure, service, supply, or prescription drug produces the desired effect under ideal conditions (in a controlled environment under lab circumstances).

# Emergency

See the "Medical emergency" definition.

# Emergency fill

**Emergency fill** is a process where the plan covers a limited amount of a prescription drug on an emergency basis while the plan processes your drug prior authorization request.

# Employer group

**Employer group** for the Public Employees Benefits Board (PEBB) Program means those counties, municipalities, political subdivisions, the Washington health benefits exchange, tribal governments, and employee organizations representing state civil service employees obtaining employee benefits through a

contractual agreement with the Health Care Authority (HCA) to participate in benefit plans developed by the PEB board.

**Employer group** for the School Employees Benefits Board (SEBB) Program means an employee organization representing school employees and a tribal school as defined in RCW 28A.715.010, obtaining school employee benefits through a contractual agreement with the Health Care Authority (HCA) to participate in benefit plans developed by the SEB board.

# Employing agency

**Employing agency** means a division, department, or separate agency of state government, including an institution of higher education; a county, municipality, or other political subdivision; and a tribal government covered by chapter 41.05 RCW.

# Enrollee

**Enrollee** means a person who meets all eligibility requirements and is enrolled in PEBB benefits, and for whom applicable premium payments have been made.

# Excluded pharmacy

**Excluded pharmacies** are pharmacies or pharmacy chains that are prohibited from contracting with the Pharmacy Network, and that are not eligible to be reimbursed for Prescription Drug Claims. Prescriptions filled at excluded pharmacies are not covered. Locate a network pharmacy by visiting the UMP Prescription drug coverage webpage (see Directory for link).

# Experimental or investigational

**Experimental or investigational** means any treatment that is not recognized by the plan as conforming to standard medical care for the condition, disease, illness, or injury being treated. "Treatment" in this setting may include any intervention, therapy, procedure, facility, equipment, drug usage, device, service, supply, intervention, biologic product or drug (prescription or nonprescription). Experimental and investigational treatments are not covered, even if the treatment is considered medically necessary. The plan will review scientific evidence from well-designed clinical studies found in peer-reviewed medical literature, if available, and information obtained from the treating provider regarding the treatment to determine if it is experimental or investigational.

A treatment meeting any of the following criteria is considered experimental or investigational:

- Approval of the treatment or one of its components by one or more government agencies (e.g., FDA) is required but has not been obtained at the time the treatment is requested or administered.
- The improvement has not been shown to be attainable outside the laboratory or clinical research setting.
- The scientific evidence does not permit conclusions concerning the effect of the treatment on health outcomes, which include the disease process, injury or illness, length of life, ability to function, and quality of life.
- The scientific evidence does not show that the treatment is as beneficial as any established alternatives.
- The treatment has not been demonstrated to improve net health outcomes.
- The treatment has scientific evidence to support its use, but not for the specific indication for which it is being requested.
- The treatment is a drug or device that is prescribed for other than its FDA-approved use(s) and is not recognized as "effective" for the use for which it is being prescribed. To be considered "effective" for

other than its FDA-approved use, a prescription drug or device must be so recognized in one of the standard reference compendia or, if not, then in a majority of relevant peer-reviewed medical literature; or by the U.S. Secretary of Health and Human Services.

- The treatment is considered to be experimental or investigational by U.S. standards.
- The treatment is drug combination therapy, when the scientific literature only supports the drug's use as monotherapy and not when utilized in combination with other drugs.
- The treatment is drug monotherapy, when the scientific literature only supports the drug's use when utilized in combination with other drugs.
- The treatment is not provided by a provider that has demonstrated medical proficiency in the provision of the treatment.
- The treatment is only available in the U.S. as part of a clinical trial or research program for the illness or condition being treated.
    - Although the plan does not pay for items, drugs, devices, or services (including items, drugs, devices, or services provided in a clinical trial) for investigational use, the plan does not deny qualified individuals from participating in approved clinical trials. The terms "qualified individual" and "approved clinical trial" are defined in 42 U.S.C. §300gg-8. If a qualified individual is participating in an approved clinical trial, the plan will not deny, limit, or impose additional conditions on the coverage for routine patient costs for items and services furnished in connection with participation in the trial and will not discriminate against the individual on the basis of the individual's participation in such trial. The plan will apply its standard terms and conditions for routine patient costs for items and services furnished in connection with participation in the trial.
- The treatment is the subject of an on-going phase I or phase II clinical trial or is the research, experimental, study, or investigational arm of an on-going phase III clinical trial.

# Explanation of Benefits (EOB)

An **Explanation of Benefits (EOB)** is a detailed account of each medical claim processed by the plan, which the plan sends to you to notify you of claim payment or denial. You may also sign in to your Regence account or contact UMP Customer Service to request a copy of an EOB (see Directory for link and contact information). You will need to provide identifying information over the phone.

# Family

**Family** is defined as all eligible family members (subscriber and dependents) who are enrolled on a single account.

# Fee schedule

A **fee schedule** is a list of the plan's maximum payment amounts for specific services or supplies. Preferred providers have agreed to accept these fees as payment in full for services to members. See the definition of "Allowed amount, medical services" for more details.

# Formulary

A **formulary** is a list available online that specifies how prescription drugs are covered by the plan. By using this list, you may find out if a prescription drug is covered, if the drug must be ordered through the plan's specialty drug pharmacy, and whether the drug has any limitations (such as needing prior authorization or quantity limits; see the "Limits on your prescription drug coverage" section).

The UMP Preferred Drug List is sometimes called a formulary (see "The UMP Preferred Drug List" section for more information).

## Generic drug

A **generic drug** is a prescription drug with the same active ingredient(s), but not necessarily the same inactive ingredients, as a brand-name drug that is no longer protected by a commercial patent. A generic drug is therapeutically equivalent to the brand-name prescription drug, which means it works like the brand-name drug in dosage, strength, performance, and use. All generic drugs sold in the U.S. must be reviewed and approved by the FDA, and meet the same quality and safety standards as brand-name drugs.

## Generic equivalent

A **generic equivalent** is a generic prescription drug that has the same active ingredients as its brand-name counterpart. For a generic drug to be considered "equivalent," it has to be approved by the FDA as being interchangeable with that brand-name drug. Under Washington State law, the pharmacist is required to dispense a generic equivalent in place of a brand-name drug, unless your provider objects. See "Substitution under Washington State Law" on page 85 for how this works.

## Gestational Parent

The individual who carries the pregnancy and gives birth.

## Grievance

A **grievance** is also called a complaint. See the "Complaint and appeal procedures" section for details on how these are handled.

## Habilitative (neurodevelopmental) services

**Habilitative (neurodevelopmental) services** are health care services that help you keep, learn, or improve skills and functioning for daily living. Examples include therapy for a child who isn't walking or talking at the expected age. These services may include physical and occupational therapy, speech-language pathology, and other services for people with disabilities in a variety of inpatient and/or outpatient settings.

## Health Care Authority (HCA)

The **Health Care Authority (HCA)** is the Washington State agency that administers the Uniform Medical Plan (UMP Classic, UMP Classic Medicare with Part D (PDP), UMP Select, and UMP CDHP), in addition to the following health care programs: Washington Prescription Drug Program, PEBB Program, SEBB Program, Behavioral Health and Recovery, and Apple Health (also known as Medicaid).

## Health intervention

**Health intervention** is a prescription drug, service, or supply provided to prevent, diagnose, detect, treat, or palliate the following: disease, illness, injury, genetic or congenital anomaly, pregnancy, or biological or psychological condition that lies outside the range of normal, age-appropriate human variation. A health intervention may also maintain or restore functional ability. A health intervention is defined not only by the intervention itself, but also by the medical condition and patient indications for which it is being applied. A health intervention is new if it is not yet in widespread use for the medical condition and the patient indications being considered.

# Health outcomes

**Health outcomes** are results that affect health status as measured by the length or quality (primarily as perceived by the member) of a person's life.

# Health savings account (HSA)

A **health savings account (HSA)** is a tax-advantaged spending and savings account that may be used to pay for qualified medical expenses. IRS rules govern who may have an HSA and how the funds may be spent. The HSA is funded by pre-tax contributions from you, the State of Washington, or both. See the "Deductibles and limits" section for information on contribution limits. You may contribute up to the annual limit even if your health plan deductible is lower. The contribution limits are subject to change every year, adjusted for inflation.

# High-deductible health plan (HDHP)

A **high-deductible health plan (HDHP)** is any plan with an annual deductible of at least $1,750 for an individual or $3,500 for a family. UMP CDHP is an HDHP with an HSA.

# Home health agency

A home health agency is an agency or organization that:

- Provides a program of home health care;
- Practices within the scope of its license as a provider of home health services; and
- Is Medicare-certified, accredited by the Joint Commission on Accreditation of Healthcare Organizations, or a preferred provider.

# Hospice

**Hospice** is services provided by a state-licensed hospice program in the member's home or in a hospice facility to terminally ill members. Hospice care includes services such as pain care relief for terminally ill members without the intent to cure, and support services for their families.

# Hospital

A **hospital** is an institution accredited under the Hospital Accreditation Program of the Joint Commission and licensed by the state where it is located. A hospital has a defined course of therapeutic intervention and special programming in a controlled environment. A hospital also offers a degree of security, supervision, and structure. Hospital patients must be medically monitored with 24-hour medical availability and 24-hour onsite services as defined in federal guidelines outlining *Conditions of Participation for Hospitals*.

The term hospital does not include a convalescent nursing home or institution (or a part of one) that:

- Furnishes primarily domiciliary or custodial care.
- Is operated as a school.
- Is used principally as a convalescent facility, rest facility, nursing facility, or facility for the aged.

# Inpatient rate

The **inpatient rate** depends on the provider's or facility's network status during the inpatient stay.

The plan pays for professional services, such as provider visits or lab tests, based on the provider's network status during an inpatient stay:

- Preferred providers and facilities: You pay 15 percent of the allowed amount after you meet your deductible. The plan pays 85 percent of the allowed amount.
- Participating providers and facilities: You pay 40 percent of the allowed amount after you meet your deductible. The plan pays 60 percent of the allowed amount.
- Out-of-network providers and facilities: You pay 40 percent of the allowed amount after you meet your deductible. You pay all charges billed to you above the allowed amount (known as balance billing). The plan pays 60 percent of the allowed amount.

## Inpatient stay

An **inpatient stay** begins when you are admitted to a hospital or other medical facility, and ends when you are discharged from that facility.

## Independent review organization (IRO)

An **independent review organization (IRO)** conducts the independent (or external) review of an appeal. An IRO is a group of medical and benefit experts certified by the Washington State Office of the Insurance Commissioner and not related to the plan, Regence BlueShield, ArrayRx, or HCA. An IRO is intended to provide unbiased, independent clinical and benefit expertise, as well as evidence-based decision making while ensuring confidentiality. The IRO reviews your appeal to determine if the plan's decision is consistent with state law and the applicable COC. The plan pays the IRO's charges. See "External review (independent review)" on page 114.

## Intensive Outpatient Program

**Intensive Outpatient Program** (IOP) is an outpatient program that is licensed as a facility or agency by the appropriate state agency and is provided under the supervision of a psychiatrist or psychiatric extender. IOP is intended to provide treatment on an outpatient basis, does not include boarding or housing, and is intended to provide treatment interventions in a structured setting, with patients returning to their home environments each day. IOP is a minimum of three hours per day, three days per week.

## Limited benefit

> **TIP:** This definition applies only to those benefits in which it is used in this COC. Other benefits have additional limits related to medical necessity or prior authorization of services (see the "Limits on plan coverage" section).

A **limited benefit** is a benefit that is limited to a certain number of visits or a maximum dollar amount. The limit applies to these benefits even if the provider prescribes additional visits and even if the visits are medically necessary.

For benefits limited to a certain number of visits, any visits that are applied to your deductible also count against your annual visit limit. In addition, visits that are paid by another health plan that is primary apply to the plan limit. For example, if your primary plan applies your first 12 chiropractor sessions to your deductible, you may receive coverage for 12 more sessions in that calendar year, for a total of 24 visits (the visit maximum for chiropractic services). For benefits limited to a certain dollar amount (e.g., wigs),

you pay all charges billed to you above the maximum dollar limit. Any amounts billed above the maximum dollar limit do not apply to your out-of-pocket limit.

These limits apply **per member**.

Services are counted against a limited benefit according to the type of service, not the provider type. When a provider practicing within the scope of their license provides services coded under a limited benefit (e.g., spinal manipulation or physical therapy), those services will be counted against the benefit regardless of the provider type. In addition, if more than one type of limited benefit service is provided during a single visit, the services will count against all the limited benefits. For example, if both manipulation and physical therapy codes are billed for a visit, that visit will count against both the spinal and extremity manipulation and physical therapy benefit limits.

# Maintenance care

**Maintenance care** is a health intervention after the member has reached maximum rehabilitation potential or functional level and has shown no significant improvement for one to two weeks, and instruction in the maintenance program has been completed.

Maintenance care may apply to several different services, including, but not limited to, physical therapy, speech therapy, neurodevelopmental therapy, home health care, and skilled nursing care.

# Medical

**Medical** generally refers to all plan benefits and services other than those covered under preventive care and prescription drug benefits (except as the term is used in the eligibility sections of this COC).

# Medical benefit

**Medical benefit** refers to services subject to your deductible, copay and/or coinsurance. See the "What you pay for services" section for a description of how this works.

# Medical emergency

A **medical emergency** means a medical condition that manifests itself by acute symptoms of sufficient severity (including severe pain) so that a prudent layperson who has an average knowledge of medicine and health would reasonably expect that not seeking immediate medical treatment at an emergency room would result in any one of the following:

- Places the member's health, or with respect to pregnancy, the health of an unborn child, in serious jeopardy;
- Causes serious impairment to bodily functions; or
- Causes serious dysfunction of any bodily organ or part.

# Medical food

**Medical food** is food administered under the supervision of a provider, intended for the specific dietary management of a disease or condition for which there are distinctive nutritional requirements.

# Medically necessary or medical necessity

**ALERT!** The provider or member must provide documentation demonstrating medical necessity when requested by the plan, or the plan may deny services as not medically necessary. The plan may not cover some medically necessary services. All benefits or services that are medically necessary are subject to the plan's coverage limitations, exclusions, and provisions of the plan. It is important to review this COC or verify coverage with UMP Customer Service before receiving services.

**Medically necessary or medical necessity** means health care services, supplies, prescription drugs, or interventions that a licensed health care provider recommends and all the following conditions are met:

- The purpose of the service, supply, intervention, or prescription drug is to prevent, evaluate, treat, or diagnose an illness, injury, disease, or its symptoms.
- The level of service, supply, intervention, prescription drug, or prescription drug dose is appropriate considering the potential benefits and harm to the member.
- The level of service, supply, intervention, prescription drug, or prescription drug dose is known to be effective in improving health outcomes.
- The level of service, supply, intervention, prescription drug, or prescription drug dose recommended for this condition is cost-effective compared to alternative interventions, including no intervention.
- The service, supply, or intervention is not being recommended for reasons of convenience to the patient or health care provider.
- For services that the HTCC has reviewed, and that UMP has implemented, medical necessity is established only when HTCC's coverage conditions are met.

The fact that a physician or other provider prescribes, orders, recommends, or approves a service or supply, prescription drug, or prescription drug dose does not make it medically necessary.

The plan may require proof that services, interventions, supplies, or prescription drugs (including court-ordered care) are medically necessary. Depending on the circumstances, such proof may be documentation about the member's condition or scientific evidence about the effectiveness of the treatment.

The plan will not provide benefits if the required proof is not received, or does not adequately justify the medical necessity of the service, supply, prescription drug, or prescription drug dose. Claims processing may be delayed if proof of medical necessity is required but not adequately provided by the health service provider.

The plan uses scientific evidence from peer-reviewed medical literature to determine effectiveness for services and interventions not yet in widespread use for the medical condition and member indications being considered.

For services that the HTCC has reviewed, and that UMP has implemented, state law requires that UMP use the HTCC's coverage criteria in determining whether the service is medically necessary. When the HTCC determines that a service is not covered, then the service is not covered by the plan. If the HTCC determines that a service is covered, then the HTCC's criteria (if any) determine medical necessity. The HTCC's decisions and related documentation are available on the HCA website at **hca.wa.gov/hta**.

For services, interventions, supplies, prescription drugs, or prescription drug doses not related to an HTCC review, the plan first uses scientific evidence, then professional standards, then expert opinion to determine coverage.

Scientific evidence consists primarily of controlled clinical trials that either directly or indirectly demonstrate the effect of the intervention on health outcomes. If controlled clinical trials are not available, observational studies that demonstrate a causal relationship between the intervention and health outcomes may be used. Partially controlled observational studies and uncontrolled clinical series may be suggestive, but do not by themselves demonstrate a causal relationship unless the magnitude of the effect observed exceeds anything that could be explained either by the natural history of the medical condition or potential experimental biases.

Interventions for which clinical trials have not been conducted because of epidemiological reasons (that is, rare or new diseases or orphan populations) shall be evaluated based on professional standards of care or expert opinion.

A level of service, supply, prescription drug, prescription drug dose, or intervention is considered cost effective if the benefits and harms relative to the costs represent an economically efficient use of resources for members with this condition. The plan applies this criterion based on the individual member's medical situation and characteristics. Cost-effective does not always mean the lowest price.

Preventive services not covered by the plan's preventive care benefit will still be covered under the medical benefit or prescription drug benefit if the outlined criteria are met for medical necessity.

# Member

A **member** is an eligible employee, retiree, former employee or former dependent in Continuation Coverage, survivor, retired employee of a former employer group, or dependent enrolled in the plan (see also the "Enrollee" definition).

# Network

**Network** is the preferred and participating facilities, providers, and suppliers your health plan contracts with to provide health care services.

# Network pharmacy

A **network pharmacy** contracts with ArrayRx to provide prescription drug coverage to UMP members at the contracted rate (allowed amount). See the "Where to buy your prescription drugs" section for details about the advantages of using network pharmacies.

# Network provider

A **network provider** is a preferred or participating provider. See the "Participating provider" definition and the "Preferred provider" definition for specific details.

# Network rate

The **network rate** means the plan pays 85 percent of the allowed amount for preferred providers after you meet your medical deductible.

# Network status

**Network status** refers to whether a provider is preferred, participating, or out-of-network with the plan. You may find out the network status of your provider by visiting the UMP provider search or by contacting UMP Customer Service (see Directory for link and contact information).

## Network vaccination pharmacy

A **network vaccination pharmacy** is a network pharmacy that can administer covered immunizations to plan members at the preventive rate. Locate a network pharmacy on the UMP Prescription drug coverage webpage or by contacting ArrayRx Customer Service (see Directory for link and contact information) and contact the pharmacy directly to verify if they administer vaccinations.

## Noncovered prescription drugs

**Noncovered prescription drugs** refer to any drug that is only covered if the member receives an exception from the plan. Some drugs may be medically necessary, yet still are not covered. See the "Prescription drugs and products UMP does not cover" section.

## Noncovered services

**Noncovered services** refers to any medical service that is not covered by the plan. Some services may be medically necessary, yet still are not covered. See the "What the plan does not cover" section. When the HTCC determines that a service is not covered, then the service is not covered by the plan.

## Nonduplication of benefits

**Nonduplication of benefits** is how UMP coordinates benefits when UMP is your secondary coverage. When another plan is primary (pays first), that plan pays their normal benefit. UMP then pays up to the amount we would have paid if UMP had been the primary plan. If the primary plan pays as much or more than the normal UMP benefit, UMP pays nothing. UMP does not pay the rest of the allowed amount. See examples on page 105.

## Non-network pharmacy

A **non-network pharmacy** does not contract with ArrayRx. See page 81 for what happens if you purchase covered prescription drugs at a non-network pharmacy. Non-network pharmacies do not include excluded pharmacies (see the definition of "Excluded pharmacy"). You will be reimbursed at the Out-of-network rate if you utilize a non-network pharmacy (see the definition of "Out-of-network rate, prescription drugs"). There are no network pharmacies outside of the U.S.

## Nonprescription alternative

A **nonprescription alternative** includes an over-the-counter drug, dietary supplement, herbal supplement, vitamin, mineral, medical food, or medical device that you may buy without a prescription that has similar safety, efficacy, and ingredients as a prescription drug.

## Nonprescription drug

A **nonprescription drug** includes an over-the-counter drug, dietary supplement, herbal supplement, vitamin, mineral, medical food, or medical device that you may buy without a prescription.

## Normal benefit

The plan's **normal benefit** is the dollar amount the plan would normally pay if no other group health plan had the primary responsibility to pay the claim for a benefit.

## Occupational injury or illness

An **occupational injury or illness** is one resulting from work that is for pay or profit.

# Orthognathic and Telegnathic surgery

**Orthognathic surgery** is surgery to correct conditions of the jaw and face related to structure, growth, or TMJ disorders, or to correct orthodontic problems that cannot be easily treated with braces.

**Telegnathic surgery** means skeletal advancement to enlarge and stabilize the pharyngeal airway to treat obstructive sleep apnea.

# Out-of-network provider(s)

An **out-of-network provider** is a health care provider that is:

- In the Regence BlueShield service area, but is not contracted as part of Regence BlueShield's PPO network; or
- Outside the Regence BlueShield service area but is not contracted with another Blue Cross or Blue Shield organization in the BlueCard Program (designated as a provider in the PPO network) to provide services and supplies to plan members.

See page 15 for a description of how services by these providers are covered.

# Out-of-network rate, medical

Out-of-network providers are paid at the **out-of-network rate**. When you receive medical services from out-of-network providers, you pay 40 percent of the allowed amount after you meet your deductible. You pay all charges billed to you above the allowed amount (known as balance billing). The plan pays 60 percent of the allowed amount.

# Out-of-network rate, prescription drugs

**Out-of-network rate, prescription drugs:** If you use a non-network retail pharmacy, you will pay the entire cost of the prescription drug at the time of purchase and must submit a claim for reimbursement. You will be reimbursed according to the out-of-network rate, minus applicable cost share and deductible. You will not be reimbursed for amounts above this rate. Your cost share at non-network pharmacies is shown under the "What you pay for prescription drugs" section plus any amount above the out-of-network rate from the pharmacy (this amount does not apply to your deductible or out-of-pocket limit).

# Out-of-pocket limit

The **out-of-pocket limit** is the most you pay during a calendar year for covered medical services and prescription drugs before the plan pays 100 percent of the allowed amount for preferred providers and network pharmacies. This limit does not include your premium, balance-billed charges, or services the plan does not cover. For more information on how this works, see the "Out-of-pocket limit" section.

For this plan, the limit is $4,200 for one member enrolled in the plan, or $8,400 if there are two or more members enrolled in the plan. However, no one member may exceed $7,000 in covered out-of-pocket expenses annually. Once the out-of-pocket limit has been reached, the plan pays 100 percent of the allowed amount for covered medical services from preferred providers and 100 percent of the allowed amount for covered prescription drugs and products for the rest of the calendar year.

See page 26 for what does and does not apply to this limit. Your deductible does apply to this limit.

# Outpatient rate

The plan's **outpatient rate** depends on the provider's status:

- Preferred providers: You pay 15 percent of the allowed amount after you meet your deductible. The plan pays 85 percent of the allowed amount.
- Participating providers: You pay 40 percent of the allowed amount after you meet your deductible. The plan pays 60 percent of the allowed amount.
- Out-of-network providers: You pay 40 percent of the allowed amount after you meet your deductible. You pay all charges billed to you above the allowed amount (known as balance billing). The plan pays 60 percent of the allowed amount.

## Outpatient surgery center

See the "Ambulatory surgery center (ASC)" definition.

## Outward Bound

An international network of outdoor education organizations whose aim is to foster the personal growth and social skills of participants by using challenging expeditions in the outdoors.

## Over-the-counter alternative

An **over-the-counter alternative** is a drug that you may buy without a prescription that has similar safety, efficacy, and ingredients as a prescription drug.

## Over-the-counter drugs

**Over-the-counter drugs** are medications you may get without a prescription.

## Over-the-counter equivalent

An **over-the-counter equivalent** is a drug you may buy without a prescription that has identical active ingredients and strengths as a prescription drug or product in a comparable dosage form.

## P&T Committee

See the "Pharmacy & Therapeutics (P&T) Committee" definition.

## Partial Hospitalization Program (PHP)

**Partial Hospitalization Program** (PHP) is an inpatient program that is provided under the supervision of an attending psychiatrist or psychiatric extender. PHP is intended to provide treatment on an inpatient basis, does not include boarding or housing, and is intended to provide treatment interventions in a structured setting, with patients returning to their home environments each day. PHP is a minimum of five hours per day, five days per week.

## Participating provider(s)

A **participating provider** is:

- Contracted with Regence BlueShield but is not in your plan's Preferred Network, and they cannot balance bill you.
- Considered out-of-network for your plan, except for the following services:
  - Covered preventive services.
  - Mental health or substance use disorder.
  - Emergency services for a medical emergency (including air ambulance).

## Peer-reviewed medical literature

**Peer-reviewed medical literature** is scientific studies printed in journals or other publications in which original manuscripts are published only after being critically reviewed for scientific accuracy, validity, and reliability by unbiased independent experts. Peer-reviewed medical literature, for example, does not include information from health-related websites or in-house publications of pharmaceutical manufacturers.

## Pharmacy & Therapeutics (P&T) Committee

**Pharmacy & Therapeutics (P&T) Committee** is a group of providers and other health care professionals who review prescription drugs and make recommendations on the status of prescription drugs on the UMP Preferred Drug List (see "The UMP Preferred Drug List" section for more information).

## Physician services

**Physician services** are health care services provided or coordinated by a licensed medical physician, such as a:

- Doctor of osteopathic medicine (D.O.)
- Medical doctor (M.D.)
- Naturopathic physician (N.D.)

Find the complete list of covered provider types on the UMP website at **ump.regence.com/pebb/benefits/providers/covered-providers**.

## Plan

**Plan**, as referred to in this document, means Uniform Medical Plan Consumer-Directed Health Plan (UMP CDHP), a self-insured PPO plan offered by the PEBB Program. In the eligibility sections (see "Eligibility and enrollment" on page 125 and "Eligibility and enrollment for a retiree or survivor" on page 140) "plan" may include other plans not sponsored by the PEBB Program. In the "If you have other HDHP coverage" section, "plan" may mean any health insurance coverage.

## Preauthorization

Preauthorization is plan approval for coverage of specific services, supplies, or prescription drugs before they are provided to the member. Preauthorization is not a guarantee of coverage. If you or your provider do not receive preauthorization for certain medical services or prescription drugs, the plan may deny the claim. See the "Preauthorizing medical services" section for how this works. A list of medical services that require preauthorization is available on the UMP Policies that affect your care web page or by contacting UMP Customer Service (see Directory for link and contact information). See page 83 for information on prescription drugs that must be preauthorized.

## Preferred drug

A **preferred drug** is a prescription drug that is listed on the UMP Preferred Drug List.

## Preferred Drug List

The UMP **Preferred Drug List** is a list available online that specifies how prescription drugs are covered by the plan. By using this list, you may find out if a prescription drug is covered, how much you will pay, if the drug must be ordered through the plan's specialty drug pharmacy, and whether the drug has any

limitations (such as needing prior authorization or quantity limits; see the "Limits on your prescription drug coverage" section).

Drugs are designated by "tiers":

- Preventive Tier drugs (e.g. contraceptives) cost you $0
- Value Tier are cost-effective prescription drugs for treatment of certain chronic conditions
- Tier 1 are primarily generic prescription drugs
- Tier 2 are preferred brand-name prescription drugs and some high-cost generic drugs

The UMP Preferred Drug List is based on the Washington Preferred Drug List and recommendations by one of the P&T Committees (see the "How UMP decides which prescription drugs are preferred" section for more information).

# Preferred provider(s)

A **preferred provider** is a provider:

- In the Regence service area and contracted as part of Regence BlueShield's Preferred Network; or
- Outside the Regence service area and contracted with another Blue Cross or Blue Shield organization in the BlueCard Program (designated as a Provider in the "Preferred Provider Organization (PPO) Network") to provide services and supplies to plan members.

# Preferred Provider Organization (PPO)

A **Preferred Provider Organization (PPO)** is a health plan that has a network of providers who have agreed to provide services at discounted rates. Members may self-refer to most specialists. UMP CDHP is a PPO.

# Prenatal

**Prenatal** means during pregnancy.

# Prescription drug

**Prescription drug** means a drug approved by the FDA that can be dispensed only with an order given by a properly authorized person. The designation of a medication as a prescription drug is made by the FDA.

# Preventive care

**Preventive care** means those services described:

- By the Public Health Services Act, Section 2713:
    - Evidence-informed preventive care and screenings for women as described in HRSA Guidelines in accordance with 45 CFR §147.130 (a)(iv).
    - Evidence-informed preventive care screenings for infants, children, and adolescents supported by the Health Resources and Services Administration (HRSA).
    - Services with an A or B rating by the U.S. Preventive Services Task Force.
- As covered immunizations recommended by the USPSTF, HRSA, and CDC.

## Preventive rate

Covered preventive services are **not** subject to your deductible. The plan's **preventive rate** depends on the provider's status:

- Preferred providers: You pay $0. The plan pays 100 percent of the allowed amount.
- Participating providers: You pay $0. The plan pays 100 percent of the allowed amount.
- Out-of-network providers: You pay 40 percent of the allowed amount. You pay all charges billed to you above the allowed amount (known as balance billing). The plan pays 60 percent of the allowed amount.

## Primary care provider (PCP)

A **primary care provider (PCP)** is a physician (see the "Physician services" definition), nurse practitioner, or physician assistant who provides, coordinates, or helps a member access a range of health care services. See page 17 for a list of specialties that may be a primary care provider.

## Primary payer

The **primary payer** is the insurance plan that processes the claim first when a member has more than one group insurance plan covering the services and the plans must coordinate benefits.

## Prior authorization

**Prior authorization** is plan approval for specific services, supplies, or prescription drugs before they are covered. Prior authorization is not a guarantee of coverage. If you or your provider do not receive prior authorization for certain medical services or prescription drugs, the plan may deny the claim. See the "Prior authorization of medical services" section for how this works. For a list of medical services that require prior authorization visit the Policies that affect your care webpage or contact UMP Customer Service. See the Directory for the webpage link and contact information. See page 83 for information on prescription drugs that require prior authorization.

## Professional services

**Professional services** are non-facility medical services performed by professional providers, including, but not limited to, medical doctors, doctors of osteopathy, naturopathic physicians, and advanced registered nurse practitioners.

## Provider

A **provider** is an individual medical professional (such as a doctor or nurse), hospital, skilled nursing facility, pharmacy, program, equipment and supply vendor, or other facility, organization, or entity that provides care or bills for health care services or products.

## Provider network

A **provider network** is a group of providers who negotiate a contract with Regence BlueShield to provide health care services to plan members. These providers have agreed to see members under certain rules, including billing at contracted rates (see the "Allowed amount, medical services" definition). The Regence BlueShield, including the BlueCard Program, provider network for UMP CDHP members in 2027 consist of preferred and participating providers.

## Public Employees Benefits Board (PEBB)

The **Public Employees Benefits Board (PEBB)**, is a group of representatives, appointed by the governor, who approves insurance benefit plans for employees and their dependents, and establishes eligibility criteria for participation in insurance benefit plans.

## Public Employees Benefits Board (PEBB) plan

A **Public Employees Benefits Board (PEBB) plan** is one of several health benefit plans, including the Uniform Medical Plan (UMP Classic, UMP Classic Medicare with Part D (PDP), UMP Select, and UMP CDHP), offered through the PEBB Program to eligible public employees, former employees and dependents in Continuation Coverage, retirees, survivors, retired employees of a former employer group, and their eligible dependents. The PEB Board designs benefits and eligibility and is administered by HCA as part of a comprehensive benefits package.

## Public Employees Benefits Board (PEBB) Program

The **Public Employees Benefits Board (PEBB) Program** is the HCA program that administers PEBB benefit eligibility and enrollment.

## Qualified medical expense

A **qualified medical expense** is a cost payable through an HSA, without paying income tax or tax penalties. See IRS publication 502 for the complete definition. IRS publication 502 also lists the specific types of services and supplies that qualify. You may also access a list on the HealthEquity website (see Directory for link).

## Quantity limit

A **quantity limit** is a limit on how much of a prescription drug you may get for a specific time period (days' supply).

## Reconstructive surgery

**Reconstructive surgery** is surgery and follow-up treatment needed to correct or improve a part of the body because of birth defects, accidents, injuries, or medical conditions.

## Regence BlueShield service area

The **Regence BlueShield service area** means the Washington counties of Clallam, Columbia, Cowlitz, Grays Harbor, Jefferson, King, Kitsap, Klickitat, Lewis, Mason, Pacific, Pierce, San Juan, Skagit, Skamania, Snohomish, Thurston, Yakima, Wahkiakum, Walla Walla, Whatcom, and any other areas designated by Regence. Check the Regence website at **regence.com** for up-to-date information.

## Rehabilitative services

**Rehabilitative services** are health care services that help you keep, get back, or improve skills and functioning for daily living that have been lost or impaired because you were sick, hurt, or disabled. These services may include physical and occupational therapy, speech-language pathology, and psychiatric rehabilitation services in a variety of inpatient and/or outpatient settings.

## Residential treatment facility

**Residential treatment facility** means a facility that offers a defined course of therapeutic intervention and special programming in a controlled environment; offers a degree of security, supervision, and

structure; and is licensed by the appropriate state and local authority to provide such services. Patients also must be medically monitored with 24-hour medical availability and 24-hour onsite clinician services.

Residential treatment facilities typically do not include halfway houses; supervised living; group homes; wilderness courses or camps; Outward Bound; outdoor youth programs; boarding houses; or settings that primarily either focus on building self-esteem or leadership skills or provide a supportive environment to address long-term social needs. However, services by providers in such settings may be covered if they are billed separately and otherwise would be covered.

## Respite care

**Respite care** is continuous care for a homebound hospice member of more than four hours a day to provide family members temporary relief from caring for the member.

## Retired employee of a former employer group

**A retired employee of a former employer group** includes a retired employee of a PEBB employer group and a retired school employee of a SEBB employer group continuing enrollment in PEBB health plan coverage after losing eligibility for PEBB retiree insurance coverage upon the employer group ending participation in insurance plans and contracts with the Health Care Authority (HCA).

## Routine

**Routine** services are those provided as preventive, not because of an injury or illness. In the case of covered immunizations, routine refers to covered immunizations that are recommended by the USPSTF, HRSA, and CDC (see the "Covered immunizations" in the "Preventive care" benefit).

## Same-day surgery center

See the “Ambulatory surgery center (ASC)” definition.

## School Employees Benefits Board (SEBB)

**The School Employees Benefits Board (SEBB)** is a group of representatives, appointed by the governor, who designs and approves insurance benefit plans for school employees and their dependents, and establishes eligibility criteria for participation in insurance benefit plans.

## School Employees Benefits Board (SEBB) plan

A **School Employees Benefits Board (SEBB) plan** is one of several health benefit plans, including the Uniform Medical Plan (UMP Achieve 1, UMP Achieve 2, and UMP High Deductible), offered through the SEBB Program to eligible school employees as well as their dependents, school board members and their dependents, and former employees of school districts, educational service districts (ESDs), and charter schools and their dependents.

## School Employees Benefits Board (SEBB) Program

**The School Employees Benefits Board (SEBB) Program** is the program within HCA that administers insurance and other benefits for eligible school employees, eligible school board members, and eligible dependents.

## Scientific evidence

**Scientific evidence** means scientific studies published in or accepted for publication by medical journals that meet nationally recognized requirements for scientific manuscripts and that submit most of their published articles for review by experts who are not part of the editorial staff. Scientific evidence also

refers to findings, studies, or research conducted by or under the auspices of federal government agencies and nationally recognized federal research institutes. However, scientific evidence does not include published peer-reviewed literature sponsored to a significant extent by a pharmaceutical manufacturing company or medical device manufacturer or a single study without other supportable studies.

## Scope of practice

**Scope of practice** refers to the services a provider may perform and bill for, based on the provider's professional license as issued by local authorities. For example, some provider types may prescribe prescription drugs, and some may not.

## Screening

**Screening** refers to services performed to prevent or detect illness in the absence of disease or symptoms.

## Secondary coverage

When you are covered by more than one group health plan, you have **secondary coverage** that may pay a part or the rest of a provider's bill after your primary payer has paid. See the "If you have other HDHP coverage" section for more information on how this plan coordinates benefits.

## Skilled nursing care

**Skilled nursing care** is services from licensed nurses in your own home or in a nursing home. Skilled care services are from technicians and therapists in your own home or in a nursing home.

## Skilled nursing facility

A **skilled nursing facility** is an institution, or part of an institution, that provides skilled nursing care 24 hours a day and is classified as a skilled nursing facility by Medicare. Medicaid-eligible, long-term care facilities are not necessarily skilled nursing facilities.

## SmartHealth

**SmartHealth** is a voluntary wellness program offered by the PEBB Program that allows eligible subscribers to earn wellness incentives. Employees who waive medical and eligible retirees who defer PEBB retiree insurance coverage are not eligible to qualify for the incentive.

If the subscriber qualified and earned the wellness incentive during the 2026 plan year and the subscriber is still eligible to participate in the wellness incentive program in 2027, $125 is deposited into the subscriber's HSA in January 2027. If the subscriber qualifies and earns the wellness incentive during the 2027 plan year, $125 is deposited into the subscriber's HSA in January 2028 if the subscriber is still enrolled in a Consumer-Directed Health Plan in 2028. However, the SmartHealth wellness incentive will be ending at the end of 2027 and will no longer be available to earn in 2028. The wellness incentive applies to your maximum annual contribution limit. More details on eligibility and program requirements are on HCA's SmartHealth webpage at **hca.wa.gov/pebb-smarthealth**.

## Special rate

The plan's **special rate** is for services that have unique payment rules. These rules are described in the table (see the "How much you pay for covered services" column) located in the "Types of services" section.

# Specialty drugs

**Specialty drugs** are typically used to treat complex, rare conditions or chronic diseases. These drugs often require ongoing close monitoring, special handling, storage, administration (injectable, infused, inhaled), and are subject to additional requirements. Specialty drugs are identified on the UMP Preferred Drug List. See the "Where to buy your prescription drugs" and "What you pay for prescription drugs" sections for information on how specialty drug prescriptions are handled.

# Standard rate

The plan's **standard rate** depends on the provider's status:

- Preferred providers: You pay 15 percent of the allowed amount after you meet your deductible. The plan pays 85 percent of the allowed amount.
- Participating providers: You pay 40 percent of the allowed amount after you meet your deductible. The plan pays 60 percent of the allowed amount.
- Out-of-network providers: You pay 40 percent of the allowed amount after you meet your deductible. You pay all charges billed to you above the allowed amount (known as balance billing). The plan pays 60 percent of the allowed amount.

# Standard reference compendium

**Standard reference compendium** refers to any of these sources:

- *The American Hospital Formulary Service Drug Information*
- *The American Medical Association Drug Evaluation*
- *The United States Pharmacopoeia Drug Information*
- Other authoritative compendia as identified from time to time by the U.S. Secretary of Health and Human Services

# State Agency

**State agency** means an office, department, board, commission, institution, or other separate unit or division, however designated, of the Washington state government. It includes the legislature, executive branch, and agencies or courts within the judicial branch, as well as institutions of higher education and any unit of state government established by law.

# State-registered domestic partner

**State-registered domestic partner** means an adult who meets the requirements for a valid state-registered domestic partnership and has been issued a certificate of state-registered domestic partnership by the Washington State Secretary of State, or an adult whose legal union (other than a marriage) was validly formed in another jurisdiction and is substantially equivalent to a domestic partnership under Washington law.

# Step Therapy

The **step therapy** program is a type of prior authorization requirement directing patients and prescribers to lower risk and more cost-effective prescription drugs before approving other covered prescription drugs.

## Subscriber

A **subscriber** is an eligible employee, retiree, former employee or former dependent in Continuation Coverage, retired employee of a former employer group, or survivor who is the primary certificate holder and plan member.

## Substance use disorder

**Substance use disorder** is a treatable mental disorder that affects a person's brain and behavior, leading to their inability to control their use of substances like legal or illegal drugs, alcohol, or medications.

## Substance use disorder facility

A **substance use disorder facility** is an institution, or part of an institution, that specifically treats dependency on a substance or alcohol and meets all these criteria:

- Is certified by the Washington State Division of Behavioral Health and Recovery (DBHR), or for facilities outside of the Regence BlueShield service area, is contracted with the local BlueCard network
- Is licensed by the state
- Keeps adequate patient records that contain course of treatment, progress, discharge summary, and follow-up programs
- Performs services under full-time supervision of a physician or registered nurse
- Provides services, for a fee, to persons receiving substance use disorder treatment, including room and board, as well as 24-hour nursing

## Therapeutic alternative

A **therapeutic alternative** is a drug that is not chemically identical to a nonpreferred drug but has similar effects when given in therapeutically equivalent doses.

## Therapeutic equivalent

A **therapeutic equivalent** is a drug that is chemically identical to a nonpreferred drug and is expected to have the same effectiveness and toxicity when given in the same doses.

## Therapeutic interchange

**Therapeutic interchange** is when a pharmacist, with the endorsing provider's permission (see page 86), substitutes a nonpreferred prescription drug with a preferred drug that is a therapeutic alternative or equivalent.

## Tobacco cessation services

**Tobacco cessation services** are provided for quitting tobacco use through counseling and nicotine replacement therapy products.

## Unicompartmental

**Unicompartmental** refers to a diagnosis or procedure affecting only one part, or "component," of a joint (e.g. knee) as opposed to more than one part of a joint.

# Uniform Medical Plan Consumer-Directed Health Plan (UMP CDHP)

**Uniform Medical Plan Consumer-Directed Health Plan (UMP CDHP)** is a self-insured PPO health plan offered through the PEBB Program and managed by HCA.

# Disclosures

If you need this document in another format or if you need information about how to file a discrimination complaint, read the UMP nondiscrimination and language assistance notice by visiting **hca.wa.gov/ump-nondiscrimination**.

Ardon Health, CoverMyMeds, Moda Health, Postal Prescription Services (PPS), Costco Mail-Order Pharmacy, and ArrayRx are separate companies that provide pharmacy services.

Doctor On Demand is a separate company that provides telehealth services.

HealthEquity is a separate company that provides health savings account administration services.

Omada Health is a separate company that provides care and disease management services.

Pelago is a separate company that provides tobacco cessation/addiction services.

SmartHealth is provided through WebMD which is a separate company that provides health education services and materials.

Teladoc Health is a separate company that provides care and disease management services.

2nd.MD is a separate company that provides treatment decision support. The services provided are not medical care nor a substitute for medical care.